

Amendment No. 1 to SB2227

Bailey
Signature of Sponsor

AMEND Senate Bill No. 2227

House Bill No. 2539

by deleting all language after the enacting clause and substituting:

SECTION 1.

(a) Tennessee Code Annotated, Section 4-29-247(a), is amended by deleting subdivisions (6), (10), (14), and (42).

(b) Tennessee Code Annotated, Section 4-29-247(a)(8), is amended by deleting "§ 63-11-101" and substituting "§ 63-11-102".

(c) Tennessee Code Annotated, Section 4-29-247(a)(38), is amended by deleting "§ 63-13-502" and substituting "§ 63-13-202".

SECTION 2. Tennessee Code Annotated, Section 4-29-248(a), is amended by deleting subdivisions (5), (7), (34), and (35).

SECTION 3.

(a) Tennessee Code Annotated, Section 4-29-249(a), is amended by deleting subdivisions (2), (5), (7), (9), (12), (33), and (39).

(b) Tennessee Code Annotated, Section 4-29-249(a)(21), is amended by deleting "§ 63-13-401" and substituting "§ 63-13-101".

SECTION 4. Tennessee Code Annotated, Section 4-29-250(a), is amended by deleting subdivisions (3), (5), and (8).

SECTION 5. Tennessee Code Annotated, Section 8-8-201(b)(1), is amended by deleting "63-3-126,", "63-16-115,", and "63-25-110,", and deleting "63-17-219" and substituting "63-17-217".

SECTION 6. Tennessee Code Annotated, Section 8-50-117, is amended by deleting "chapter 22, part 1," and substituting "chapter 33, part 18,".

SECTION 7.

(a) Tennessee Code Annotated, Section 8-50-119(b)(1)(A)(i)(a), is amended by deleting "chapter 22, part 1" and substituting "chapter 33, part 18".

(b) Tennessee Code Annotated, Section 8-50-119(b)(1)(A)(i)(d), is amended by deleting "chapter 22" and substituting "chapter 33, part 18".

(c) Tennessee Code Annotated, Section 8-50-119(b)(1)(A)(i)(e), is amended by deleting "title 63, chapter 13, part 2" and substituting "the Occupational Therapy Practice Act, compiled in title 63, chapter 33, part 17".

(d) Tennessee Code Annotated, Section 8-50-119(b)(1)(A)(ii)(a), is amended by deleting "board for professional counselors, marital and family therapists, and clinical pastoral therapists" and substituting "commissioner of health with regard to licensure under chapter 33, part 18".

(e) Tennessee Code Annotated, Section 8-50-119(b)(1)(A)(ii)(d), is amended by deleting "board of occupational therapy" and substituting "commissioner of health with regard to licensure under the Occupational Therapy Practice Act, compiled in title 63, chapter 33, part 17".

SECTION 8.

(a) Tennessee Code Annotated, Section 33-3-114, is amended by deleting "63-11-213" and substituting "63-11-211".

(b) Tennessee Code Annotated, Section 33-3-114, is amended by deleting "63-22-114" and substituting "63-33-1807".

SECTION 9.

(a) Tennessee Code Annotated, Section 36-6-413(b)(5)(C)(iii), is amended by deleting "§ 63-22-115" and substituting "§ 63-33-1801".

(b) Tennessee Code Annotated, Section 36-6-413(b)(5)(C)(iv), is amended by deleting "chapter 22, part 2" and substituting "chapter 33, part 19".

(c) Tennessee Code Annotated, Section 36-6-413(b)(5)(C)(v), is amended by deleting "professional counselor as defined in § 63-22-104" and substituting "licensed professional counselor as defined in § 63-33-1801".

(d) Tennessee Code Annotated, Section 36-6-413(b)(5)(C)(vii), is amended by deleting the subdivision and substituting:

(vii) A certified or licensed clinical pastoral therapist as defined in § 63-33-1901; or

SECTION 10. Tennessee Code Annotated, Section 37-1-411, is amended by deleting "§ 63-11-213" and substituting "§ 63-11-211".

SECTION 11. Tennessee Code Annotated, Section 37-3-703(d), is amended by deleting the subsection and substituting:

(d) The state of Tennessee shall conduct ongoing evaluations of the healthy start pilot project and publish data on or before December 31 of each year. Each state agency that provides services to children shall make available nonidentifying information about healthy start participants for the purpose of conducting the evaluation. The report must include the following information for the preceding fiscal year:

- (1) The number of families receiving services through the project;
- (2) The average cost of services provided under the project;
- (3) The performance measures achieved by program participants; and
- (4) An overall statement of the achievements and progress of the project during the preceding fiscal year, along with recommendations for improvement or expansion.

SECTION 12. Tennessee Code Annotated, Section 48-101-610(d)(1)(B), is amended by deleting "chapter 3" and substituting "chapter 33, part 14".

SECTION 13. Tennessee Code Annotated, Section 48-248-401(d)(1)(B), is amended by deleting "chapter 3" and substituting "chapter 33, part 14".

SECTION 14. Tennessee Code Annotated, Section 48-249-1109(e)(1)(B), is amended by deleting "chapter 3" and substituting "chapter 33, part 14".

SECTION 15. Tennessee Code Annotated, Section 52-3-114, is amended by deleting "63-11-213" and substituting "63-11-211", and deleting "63-22-114" and substituting "63-33-1807".

SECTION 16. Tennessee Code Annotated, Section 53-10-303(a), is amended by deleting subdivision (1)(D), deleting the final period in subdivision (2) and substituting "; and", and adding the following as a new subdivision:

(3) A licensed podiatrist selected by the commissioner of health.

SECTION 17. Tennessee Code Annotated, Section 53-10-309, is amended by deleting the first two sentences.

SECTION 18. Tennessee Code Annotated, Section 55-21-113(b), is amended by deleting "title 63, chapter 13" and substituting "the Physical Therapy Practice Act, compiled in title 63, chapter 33, part 16".

SECTION 19. Tennessee Code Annotated, Section 56-7-1003(a)(3), is amended by deleting "or title 68, chapter 24, part 6,".

SECTION 20. Tennessee Code Annotated, Section 56-7-1012(f)(3), is amended by deleting "or title 68, chapter 24, part 6,".

SECTION 21. Tennessee Code Annotated, Section 56-7-2401(a), is amended by:

- (1) Deleting "§ 63-3-101" and substituting "§ 63-33-1401";
- (2) Deleting "§ 63-22-115" and substituting "§ 63-33-1801"; and
- (3) Deleting "§ 63-22-122" and substituting "§ 63-33-1801".

SECTION 22. Tennessee Code Annotated, Section 56-7-2402(a), is amended by deleting "§ 63-3-101" and substituting "§ 63-33-1401".

SECTION 23. Tennessee Code Annotated, Section 56-7-2405(a), is amended by deleting "§ 63-3-101" and substituting "§ 63-33-1401".

SECTION 24. Tennessee Code Annotated, Section 56-7-2409(b), is amended by deleting "a chiropractic physician licensed under title 63, chapter 4, or to a physical therapist or occupational therapist licensed under title 63, chapter 13" and substituting "a chiropractic physician licensed under title 63, chapter 4; a physical therapist licensed under the Physical Therapy Practice Act, compiled in title 63, chapter 33, part 16; or an occupational therapist licensed under the Occupational Therapy Practice Act, compiled in title 63, chapter 33, part 17".

SECTION 25. Tennessee Code Annotated, Section 62-4-109(a)(1), is amended by deleting "§ 63-18-102" and substituting "§ 63-33-201".

SECTION 26. Tennessee Code Annotated, Section 63-1-102(2)(D), is amended by deleting "chapter 3" and substituting "chapter 33, part 14".

SECTION 27. Tennessee Code Annotated, Section 63-1-104(b), is amended by deleting "and shall be signed by the executive officer and by the members of the state board charged with the duty by law of issuing the preliminary certificates".

SECTION 28. Tennessee Code Annotated, Section 63-1-120, is amended by deleting the section and substituting the following:

(a)

(1) With respect to a person required to be licensed, permitted, certified, or authorized by a board, council, committee, agency, or regulatory program created pursuant to chapters 4, 5, 6, 7, 8, 9, 10, 11, 12, 17, 19, 23, 29, and 33 of this title and to title 68, chapter 140, attached to the division of health related boards, such board, council, committee, agency, or regulatory program may:

(A) Deny an application for licensure, certification, permit, or authorization;

(B) Permanently withhold issuance of licensure, certification, permit, or authorization;

(C) Suspend, limit, or restrict previously issued licensure, certification, permit, or authorization; or

(D) Otherwise discipline a holder of a license, certificate, permit, or authorization, if the applicant, licensee, or certificate or permit holder has been disciplined by another state of the United States for any acts or omissions that would constitute grounds for discipline of a person licensed, certified, permitted, or authorized in this state.

(2) A certified copy of the initial order, final order, or other equivalent document memorializing the disciplinary action from the disciplining state constitutes prima facie evidence of a violation of this section and is sufficient grounds upon which to deny, restrict, or condition the license, permit, or certificate renewal or application, or to otherwise discipline a licensee licensed in this state.

(b) In disciplinary actions against individuals holding a license, certificate, permit, or authorization in this state at the time of a disciplinary action in another reporting state, in the absence of justifying evidence to the contrary, there is a rebuttable presumption that the sanction proposed in the proceeding is comparable to that in the reporting state; however, such presumption does not exist for those who are applying for licensure, certification, permit, or authorization in this state during or after the time the disciplinary action in the other state is pending or has become final. If a board, council, committee, agency, or regulatory program created pursuant to chapters 4, 5, 6, 7, 8, 9, 10, 11, 12, 17, 19, 23, 29, and 33 of this title or title 68, chapter 140 denies, restricts, or conditions a licensure, certification, permit, or authorization based on a disciplinary action in another state, then the applicant, upon written request, filed within thirty (30) days of the date of the action on the application, is entitled to a contested case hearing.

SECTION 29. Tennessee Code Annotated, Section 63-1-125(b)(1), is amended by deleting the subdivision and substituting:

(1) "Board" means a health-related board or agency created in this title or title 68 and includes the following:

(A) The commissioner of health with regard to licensure of professional counselors and marital family therapists under chapter 33, part 18 of this title;

(B) The commissioner of health with regard to licensure of clinical pastoral therapists under chapter 33, part 19 of this title;

(C) Board of social work licensure, created by § 63-23-101;

(D) Board of examiners in psychology, created by § 63-11-102; and

(E) The commissioner of health with regard to licensure under the Occupational Therapy Practice Act, compiled under chapter 33, part 17 of this title; and

SECTION 30.

(a) Tennessee Code Annotated, Section 63-1-125(e)(2), is amended by deleting "chapter 22" and substituting "chapter 33, part 18".

(b) Tennessee Code Annotated, Section 63-1-125(e)(3), is amended by deleting the subdivision and substituting:

(3) An alcohol and drug abuse counselor licensed under chapter 11, part 4 of this title; and

(c) Tennessee Code Annotated, Section 63-1-125(e)(4), is amended by deleting "chapter 13" and substituting "chapter 33, part 17".

(d) Tennessee Code Annotated, Section 63-1-125(e), is amended by deleting "or title 68".

SECTION 31.

(a) Tennessee Code Annotated, Section 63-1-126(a)(4), is amended by adding the following as a new subdivision:

() Under this title by the commissioner of health.

(b) Tennessee Code Annotated, Section 63-1-126(a)(4)(B), is amended by deleting the subdivision.

SECTION 32. Tennessee Code Annotated, Section 63-1-139, is amended by deleting ", title 68, chapter 24," wherever it appears.

SECTION 33.

(a) Tennessee Code Annotated, Section 63-1-144(a), is amended by deleting "council" wherever it appears and substituting "council, agency,".

(b) Tennessee Code Annotated, Section 63-1-144(b), is amended by deleting the subsection and substituting:

(b) The agency, board, committee, council, or division shall include in any order in which the payment of costs has been assessed an amount that is the maximum amount owed by the license or certificate holder at the time the order is entered. Prior to the expiration of sixty (60) days from the effective date of the order, the agency, board, committee, council, or division shall send to the license or certificate holder, by certified mail, return receipt requested, and by regular United States mail, a final costs assessment that does not exceed the maximum amount in the order.

(c) Tennessee Code Annotated, Section 63-1-144(c)(1), is amended by deleting "board, committee, or council" and substituting "agency, board, committee, council, or division".

SECTION 34. Tennessee Code Annotated, Section 63-1-150(a), is amended by deleting "63-11-220" and substituting "63-11-213".

SECTION 35. Tennessee Code Annotated, Section 63-1-151(a), is amended by deleting "chapter 3, chapters 5-9 or chapter 19 of this title" and substituting "chapters 5-9, chapter 19, or chapter 33, part 14 of this title".

SECTION 36. Tennessee Code Annotated, Section 63-1-153, is amended by deleting the section.

SECTION 37.

(a) Tennessee Code Annotated, Section 63-1-155(a)(1)(C), is amended by deleting the subdivision.

(b) Tennessee Code Annotated, Section 63-1-155(a)(1)(D), is amended by deleting "or title 68, chapter 24, part 6," wherever it appears.

SECTION 38. Tennessee Code Annotated, Section 63-1-164(f), is amended by deleting the subsection.

SECTION 39.

(a) Tennessee Code Annotated, Section 63-1-172(a)(1), is amended by deleting "chapter 13" and substituting "chapter 33, part 16".

(b) Tennessee Code Annotated, Section 63-1-172(a)(2), is amended by deleting "chapter 13" and substituting "chapter 33, part 17".

SECTION 40. Tennessee Code Annotated, Section 63-1-176(b)(4), is amended by deleting the subdivision and substituting:

(4) Render counseling as defined in § 63-33-1801.

SECTION 41. Tennessee Code Annotated, Section 63-1-177, is amended by deleting "24,".

SECTION 42. Tennessee Code Annotated, Section 63-1-201(2), is amended by deleting subdivisions (H), (I), and (J) and substituting:

(H) Licensed professional counselor licensed under chapter 33, part 18 of this title;

(I) Marital and family therapist licensed under chapter 33, part 18 of this title; and

(J) Clinical pastoral therapist licensed under chapter 33, part 19 of this title; and

SECTION 43. Tennessee Code Annotated, Section 63-1-314, is amended by deleting the section.

SECTION 44. Tennessee Code Annotated, Title 63, Chapter 3, is amended by deleting the chapter.

SECTION 45. Tennessee Code Annotated, Section 63-5-107(c)(1)(A), is amended by deleting the subdivision and substituting:

(A)

(i) Each person licensed as a dentist or dental hygienist, or registered as a dental assistant, is required to attend and complete during each two-year licensure cycle, except for the initial licensure cycle and the first licensure renewal cycle, the number of hours of board-approved continuing education courses that are prescribed by the board, subject to a minimum of twenty-four (24), and a maximum of sixty (60), clock hours. However, for the purposes of the biennial continuing education requirement for dental hygienists, no more than fifty percent (50%) of such continuing education requirement may be earned from online or web-based courses. Sponsors of such courses shall submit a sufficient description of course content to the board in order to obtain approval.

(ii) The board may approve all, part, or none of the course at its discretion. Additional procedures for implementing this requirement may be adopted by the board in its rules. The board may waive all or part of the continuing education requirements in unusual situations in which an applicant for renewal has demonstrated inability to attend such courses not attributable to fault of the applicant. The board has discretion to require completion of continuing education courses of a type and number of hours satisfactory to the board in cases involving license renewal of dentists or dental hygienists who have not actively practiced or have been retired from practice for more than two (2) years.

(iii) In addition to subdivisions (c)(1)(A)(i) and (ii), all applicants shall provide proof of current CPR certification by an organization approved by the board and maintain such certification unless such requirement is waived by the board in its discretion; provided, that a dentist who obtains a waiver from the

board for CPR requirements shall not practice dentistry unless a dentist, dental hygienist, or dental assistant who is currently certified in CPR is present.

SECTION 46. Tennessee Code Annotated, Section 63-6-105, is amended by deleting the section.

SECTION 47.

(a) Tennessee Code Annotated, Section 63-6-221(h), is amended by deleting "The board shall enter into a memorandum of understanding, contract or other written arrangement with the health facilities commission such that the commission:" and substituting:

The department's investigators are authorized on behalf of the board to do the following:

(b) Tennessee Code Annotated, Section 63-6-221(h)(1), is amended by deleting "Provides" and substituting "Provide".

(c) Tennessee Code Annotated, Section 63-6-221(h)(2), is amended by deleting "Is authorized to require" and substituting "Require".

(d) Tennessee Code Annotated, Section 63-6-221(h)(3), is amended by deleting "Is authorized to initiate" and substituting "Initiate".

(e) Tennessee Code Annotated, Section 63-6-221(h)(4), is amended by deleting "Is authorized to respond" and substituting "Respond".

SECTION 48. Tennessee Code Annotated, Section 63-6-221(i), is amended by deleting the subsection and substituting:

(i) The results of all site surveys must be transmitted to the board. The results must include any requirement for plans of correction, the determination of the acceptability of the submitted plans of correction, and the verification that the plans of correction have been implemented. The board shall make a final determination on certifying the surgical suite for performance of office-based surgeries. The results of site

surveys and board determinations must be shared on a routine basis with the board for licensing health care facilities.

SECTION 49. Tennessee Code Annotated, Title 63, Chapter 6, Part 10, is amended by deleting the part.

SECTION 50. Tennessee Code Annotated, Section 63-7-127(e)(3), is amended by deleting "Occupational and Physical Therapy Practice Act, compiled in chapter 13" and substituting "Occupational Therapy Practice Act, compiled in chapter 33, part 17".

SECTION 51. Tennessee Code Annotated, Title 63, Chapter 11, Part 1, is amended deleting the part and substituting:

63-11-101. Chapter definitions.

As used in this chapter:

- (1) "Board" means the board of examiners in psychology, created by § 63-11-102;
- (2) "Commissioner" means the commissioner of health;
- (3) "Department" means the department of health; and
- (4) "Division" means the division of health related boards in the department of health, created by § 63-1-101.

63-11-102. Creation – Membership – Immunity.

(a) There is created a board of examiners in psychology to consist of nine (9) members who are appointed by the governor under conditions set forth in this part.

(b)

(1) Two (2) members must be chosen from, and be members of the faculty with the rank of assistant professor or above of, the accredited colleges and universities in the state and be engaged in teaching, research, or administration of psychology.

(2) Four (4) members must be licensed psychologists.

(3) One (1) member must be a private citizen who is neither a licensed psychologist nor a licensed psychological examiner or licensed senior psychological examiner and who has no professional or commercial interest in the practice of psychology.

(4) One (1) member must be a licensed applied behavioral analyst.

(5) One (1) member must be a licensed alcohol and drug counselor.

(c) The board shall perform such duties and exercise such powers as this chapter prescribes and confers upon it.

(d) Members of the board are not liable to civil actions for any act performed in good faith in the performance of their duties as set forth in this chapter.

63-11-103. Appointments – Vacancies.

(a) All appointments are for terms of five (5) years.

(b) Vacancies must be filled for any unexpired terms, and members shall serve until their successors are appointed and have been qualified. Only practicing psychologists must be appointed to succeed practicing psychologists, and only academic psychologists must be appointed to succeed academic psychologists. Board members are ineligible for reappointments for a period of five (5) years following completion of their terms.

(c)

(1) Appointments to the board for the psychologist members may be made from lists of qualified candidates to be furnished to the governor by interested psychology groups, including, but not limited to, the Tennessee Psychological Association. Each list must contain the names of academic psychologists and practicing psychologists. All vacancies occurring in the board may be filled by the governor for the unexpired term from the lists of qualified candidates within thirty (30) days after the vacancy occurs. The governor shall

consult with such interested groups to determine qualified persons to fill positions on the board.

(2) In making appointments to the board, the governor shall strive to ensure that at least one (1) person serving on the board is sixty (60) years of age or older.

63-11-104. Oath of office – Records.

(a) Immediately and before entering upon the duties of the office, the members of the board shall take the constitutional oath of office and file the same in the office of the governor.

(b) The board shall have available for the governor adequate records of its proceedings and of all licenses and certificates.

63-11-105. Organization – Administration.

(a) The board shall elect annually a chair and vice chair.

(b) Each member must receive all necessary expenses incident to conducting the business of the board and, in addition thereto, be entitled to a per diem of one hundred dollars (\$100) for each day's service in conducting the business of the board. All reimbursements for travel expenses must be in accordance with the comprehensive travel regulations as promulgated by the department of finance and administration and approved by the attorney general and reporter.

(c) The board shall hold at least one (1) regular meeting each year. Called meetings may be held at the discretion of the chair or at the written request of two (2) members of the board.

(d) The board shall adopt a seal, which must be affixed to all certificates issued by the board.

(e) Six (6) members of the board at all times constitute a quorum. An action taken by the board requires the majority of the quorum present.

(f) The board is empowered to hire such assistance as is necessary to carry on its activities, within the limits of funds available to the board, and is empowered to accept grants from foundations or institutions to develop tests and carry out its functions.

(g) The board shall receive administrative support from the division.

(h) The board shall establish continuing educational requirements for all licenses created in this chapter.

63-11-106. Powers and duties.

(a) The board has the following powers and duties in addition to the powers and duties granted to or imposed upon it by this chapter:

(1) To prescribe rules for examination of candidates for a license created by this chapter;

(2) To ascertain the qualifications and fitness of applicants for all licenses created by this chapter;

(3) To issue licenses to applicants who successfully meet the qualifications for licensure under this chapter and the board's rules;

(4) To receive complaints and review issues related to complaints and disciplinary actions involving a person licensed under this chapter;

(5) To conduct investigations, as appropriate, into allegations against licensees of the board;

(6) To issue private letter rulings in accordance with § 63-1-174;

(7) To make rules and establish fees as are necessary to carry out and make effective this chapter; and

(8) To administer oaths, summon witnesses, and take testimony in all matters relating to the board's duties.

(b) All rules promulgated by the board must comply with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

(c) The board shall use the division-employed investigators, inspectors, and agents, or use any other means necessary, to bring about and maintain the rigid administration and enforcement of this chapter and all laws regulating the practice of a profession created by this chapter.

(d) The board may maintain the existing separate accounts established for the boards and committee that formerly licensed the professions now listed in this chapter; provided, the division combines the revenue collected such that the division may use the revenue of each account to offset expenditures across the accounts for this board. Funds remaining in these accounts at the end of any fiscal year do not revert to the general fund but must remain available for expenditure in accordance with law.

63-11-107. License – Application.

(a) Applicants for a license created by this chapter shall submit an application on the form created by the board and any required fees to the division.

(b) In addition to the specific requirements for licensure in this chapter, the application must include evidence that the applicant meets the following requirements:

(1) The applicant is of good moral character and conducts the applicant's professional activities in accordance with the accepted professional and ethical standards;

(2) The applicant has not had a professional credential refused, revoked, suspended, or restricted and does not have a complaint, allegation, or investigation pending in any regulatory jurisdiction in the United States or in another country for unprofessional conduct, and has not committed an act that would constitute grounds for discipline of a license under § 63-11-109; and

(3) The applicant is either a United States citizen or is lawfully present in the United States in accordance with § 4-58-103(a)(1).

(c) The division and applicant shall comply with the criminal background requirements of § 63-1-116.

(d) Each applicant seeking licensure must pay all fees as set by the board, including, but not limited to, nonrefundable application fees and examination fees.

(e) All licensees must comply with § 63-1-109.

(f) Portions of an application involving an applicant's health condition, including mental health conditions and substance use disorders, and records involving an applicant's health condition, are confidential, privileged, and not public records subject to inspection by citizens of this state, as described in § 10-7-503.

(g)

(1) When the board receives a completed application for initial licensure, or a completed application for licensure, from an applicant who is licensed in another state or territory of the United States or in the District of Columbia, then the board shall, within sixty (60) days from the date the board receives the completed application:

(A) Render a decision on the application; or

(B) Inform the applicant of the need to appear before the board.

(2) As used in this subsection (g), "completed application" means an application that satisfies all statutory and board rule requirements.

63-11-108. License renewal – Retirement.

(a) A license issued under this chapter is valid for twenty-four (24) months from the date of issuance and is renewable biennially at the end of the birth month of the licensee. The division, with the approval of the commissioner, shall establish a system of license or certification renewals at alternative intervals that allow for the distribution of the license or certification workload as uniformly as is practicable throughout the calendar year. Licenses or certificates issued under the alternative method are valid for twenty-four (24) months and expire on the last day of the last month of the license or certification period.

(b) In addition to any requirements of this chapter creating the license, a licensee requesting renewal of a certificate must provide the following to the division in advance of the expiration date of the license:

(1) A completed application on a form provided by the division and payment of the fees set by the board; and

(2) A certified or sworn statement that the licensee:

(A) Has completed all the continuing education hours required for the licensee, whether required by statute or rule, prior to the expiration date of the license; and

(B) Has not had a professional credential revoked, suspended, or restricted by a certification board, and does not have a complaint, allegation, or investigation pending in this state or in any regulatory jurisdiction for unethical or unprofessional conduct.

(c) If a licensee fails to pay the license renewal fee within sixty (60) days after it becomes due, as provided in this section, the licensee must apply for a new license rather than renew the existing license.

(d) A licensee who has lost the ability to renew pursuant to subsection (c) may make application in writing to the board for the reinstatement of the license, and, upon good cause shown, the board, in its discretion, may reinstate the license upon payment of all past due renewal fees and upon further payment of a sum set by the board.

(e) Applicants and licensees shall notify the board of the following within thirty (30) days of their occurrence:

(1) A change of name, address, or other vital information;

(2) The filing of a criminal charge or civil action against the applicant or licensee;

(3) The initiation of disciplinary charges, investigations, findings, or sanctions by a healthcare organization, federal or state agency, or other professional association against the applicant or licensee; and

(4) Any other change in information provided by the applicant or licensee to the division.

(f) A person licensed or certified to practice by this chapter who has retired or may retire on or after the effective date of this act from such practice in this state must not be made to register as required by this chapter if the person files with this board an affidavit on a form to be furnished by the board, which affidavit must state the date on which the person retired from such practice and such other facts as tend to verify the retirement as the board deems necessary. If the person thereafter reengages in such practice in this state, then the person must apply for registration with the board as provided by this chapter and meet other requirements as may be set by the board.

(g) For the purpose of effecting service of process upon a licensee, the board may notify the licensee by certified mail, return receipt requested, at the address on file with the division.

63-11-109. License denial, suspension, or revocation – Disciplinary grounds – Hearing.

(a) The board has the power:

(1) To deny an application for a license to an applicant who applies for the same through reciprocity or otherwise;

(2) To permanently or temporarily withhold issuance of a license;

(3) To issue a license contingent upon such circumstances or requirements as the board may determine;

(4) To issue a conditional license that is limited or restricted for such time and in such manner as the board may determine, including a limited license of temporary duration for applicants out of practice for an extended period of time;

(5) To issue discipline to an applicant or licensee, or on a previously issued license, including, but not limited to, private censures and warnings, public reprimands, limits, restrictions, conditions, or suspension periods, for such time and in such manner as the board may deem proper in its discretion;

(6) To discipline a licensee who has been disciplined by another state of the United States for any acts or omissions that would constitute grounds for discipline if licensed in this state, following the procedure in § 63-1-120; or

(7) To revoke, or permanently revoke, a license.

(b) The grounds upon which the board may exercise the powers enumerated in subsection (a) include, but are not limited to:

(1) Engaging in unprofessional, dishonorable, or unethical conduct;

(2) Making false statements or representations, being guilty of fraud or deceit in obtaining admission to practice, or being guilty of fraud or deceit in the course of professional practice;

(3) Violating or attempting to violate, directly or indirectly, or assisting in or abetting the violation of, or conspiring to violate, a criminal statute of this state, this chapter, a rule promulgated by the board, or a lawful order of the board issued pursuant to rules;

(4) Engaging in gross professional liability, or a pattern of continued or repeated professional liability, ignorance, negligence, or incompetence, in the course of practice;

(5) Being habitually intoxicated or personally misusing drugs, or using intoxicating liquors, narcotics, controlled substances, controlled substance analogues, or other drugs or stimulants, in such manner as to adversely affect the person's ability to safely practice in the profession;

(6) Willfully betraying a professional secret;

(7) Advertising the professional practice with untrue or misleading statements or causing the publication or circulation of fraudulent advertising relative to any disease, human ailment, or condition;

(8) Willfully violating the rules that may be promulgated by the board to regulate advertising by practitioners who are under the jurisdiction of the board;

(9) Being convicted of a felony, an offense under state or federal laws relative to drugs or the professional practice, an offense involving moral turpitude, or an offense for which the licensee or applicant is required to register as a sexual offender or violent sexual offender pursuant to the Tennessee Sexual Offender and Violent Sexual Offender Registration, Verification and Tracking Act of 2004, compiled in title 40, chapter 39, part 2;

(10) Engaging in professional practice when mentally or physically unable to safely do so;

(11) Making or signing in one's professional capacity a certificate that is known to be false at the time one makes or signs the certificate;

(12) Offering, undertaking, or agreeing to cure or treat a disease, injury, ailment, or infirmity by a secret means, method, device, or instrumentality;

(13) Giving, receiving, or aiding or abetting in the giving or receiving of rebates, either directly or indirectly;

(14) Engaging in practice under a false or assumed name or the impersonation of another practitioner of a like, similar, or different name;

(15) Practicing a level of psychology inappropriate or beyond the scope of the particular license or licenses or certification held by the licensee or certified person; and

(16) Engaging in any other action deemed to be grounds for disciplinary action under rules promulgated by the board.

(c) All proceedings for disciplinary action against a licensee under this chapter must be conducted in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

(d) An action taken under this section is subject to the applicable provisions of the Fresh Start Act, compiled in chapter 1, part 1 of this title.

SECTION 52. Tennessee Code Annotated, Title 63, Chapter 11, Part 2, is amended by deleting the part and substituting:

63-11-201. Levels of psychological practice.

(a) Five (5) levels of psychological practice are defined for the purposes of this chapter. Such levels are to be known and referred to as:

- (1) Psychological examiner;
- (2) Senior psychological examiner;
- (3) Psychologist;
- (4) Certified psychological assistant; and
- (5) Certified psychological testing technician.

(b) An individual who engages in the practice of psychology as a psychological examiner, senior psychological examiner, or psychologist must possess a valid license under this chapter.

(c) A person engaged in the practice of psychology as a certified psychological assistant or certified psychological testing technician must possess a valid certificate to practice under this chapter.

(d)

(1) This section does not prohibit a charitable clinic from employing or contracting with a psychologist if the contractual relationship between the psychologist and the charitable clinic is evidenced by a written contract, job description, or documentation, containing language that does not restrict the

psychologist from exercising independent professional judgment in diagnosing and treating patients.

(2) As used in this subsection (d), "charitable clinic" means an entity that meets the following standards:

(A) Has received a determination of exemption from the internal revenue service under 26 U.S.C. § 501(c)(3) or is a distinct part of an entity that has received such a determination of exemption;

(B) Has clinical facilities located in this state;

(C) Has a primary mission to provide health care or dental care services to low-income, uninsured, or underserved individuals;

(D) Provides one (1) or more of the following services for free or at a discounted rate:

(i) Medical care;

(ii) Dental care;

(iii) Mental health care; or

(iv) Prescription medications;

(E) Utilizes volunteer healthcare professionals and nonclinical volunteers; and

(F) Is not required to be licensed under § 68-11-202(a)(1).

63-11-202. Practice of a psychological examiner.

(a) A person practices as a psychological examiner within the meaning of this chapter when the person claims to be a psychological examiner or renders to individuals or to the public health-related clinical activities or services involving the application of recognized principles, methods, and procedures of the science and profession of psychology, such as interviewing or administering and interpreting tests of mental abilities, aptitudes, interests, and personality characteristics, for such purposes as psychological evaluation or for educational or vocational selection, guidance, or

placement. The psychological examiner shall practice the following health-related clinical activities or services only under qualified supervision provided by a psychologist or a senior psychological examiner:

- (1) Overall personality appraisal or classification;
- (2) Personality counseling;
- (3) Psychotherapy;
- (4) Behavior analysis; or
- (5) Personality readjustment techniques.

(b) This section does not expand or restrict the scope of practice for a psychological examiner.

(c) Those psychological examiners rendering health-related clinical activities or services who have been duly licensed prior to July 1, 1991, who request to the board in writing or via online application, are senior psychological examiners. Those duly licensed psychological examiners rendering health-related clinical activities or services licensed after July 1, 1991, qualify for senior psychological examiner licensure upon completion of at least five (5) years of applied experience from the date their temporary permit was first issued by the board and upon completion of two hundred (200) hours of post-licensure continuing education units distributed by categories, according to the November 1997 rules governing continuing education units. A person practices as a senior psychological examiner within the meaning of this chapter when the person claims to be a senior psychological examiner or renders to individuals or to the public for remuneration a service involving the application of recognized principles, methods, and procedures of the science and profession of psychology. A senior psychological examiner, while possessing a different scope of practice than a psychologist, is considered a health service provider and able to engage in the following designated areas of health-related psychological practice without supervision:

(1) Interviewing or administering and interpreting tests of mental abilities, aptitudes, interests, and personality characteristics for such purposes as psychological evaluation or for educational or vocational selection, guidance, or placement;

(2) Overall personality appraisal or classification, psychological testing, projective testing, and evaluations for disability or vocational purposes;

(3) Diagnosis of nervous and mental disorders;

(4) Personality counseling;

(5) Psychotherapy;

(6) Behavior analysis;

(7) Personality readjustment techniques; or

(8) Supervision of a psychological examiner.

63-11-203. Practice of a psychologist.

(a) As used in this part, "practice of psychology":

(1) Means, without regard to whether payment is received for services rendered, the observation, description, evaluation, interpretation, and modification of human behavior by the application of psychological principles, methods, and procedures for the purpose of assessing, preventing, or eliminating symptomatic, maladaptive, or undesired behavior and of enhancing interpersonal relationships, work and life adjustment, personal effectiveness, behavioral health, and mental health of individuals, families, groups, and the public; and

(2) Includes:

(A) Psychological testing and the evaluation or assessment of personal characteristics, such as intelligence, personality, abilities, interests, aptitudes, and neuropsychological functioning;

(B) Counseling, psychoanalysis, psychotherapy, hypnosis, biofeedback, and behavior analysis and therapy;

(C) Psychological diagnosis and treatment of mental, emotional, and nervous disorders or disabilities; alcoholism and substance abuse; disorders of habit or conduct; and psychological aspects of physical illness, accident, injury, or disability;

(D) Case management and utilization review of psychological services;

(E) Psychoeducational evaluation, therapy, remediation, and consultation;

(F) Forensic evaluation; provided, that a person licensed under this chapter performing forensic evaluation is considered to be acting within their lawful scope of practice and the practice of forensic evaluation is not exclusive to individuals licensed under this chapter;

(G) Parent coordination; provided, that a person licensed under this chapter performing parent coordination is considered to be acting within their lawful scope of practice and the practice of parent coordination is not exclusive to individuals licensed under this chapter; and

(H) Telepsychology, which means the practice of psychology via electronic communications technology by a person licensed under this chapter.

(b) A person represents to be a psychologist if that person uses a title or description of services incorporating the words "psychology," "psychological," or "psychologist," if the person possesses expert qualification in any area of psychology or if that person offers to the public or renders to individuals or to groups of individuals services pursuant to the practice of psychology.

63-11-204. Psychotherapy limitations – Conduct outside psychological practice prohibited.

(a) Sections 63-11-201 — 63-11-203 do not permit the use of those forms of psychotherapy that involve the administration or prescription of drugs or electroshock or in any way infringe upon the practice of medicine as defined in the laws of this state.

(b) A psychologist, psychological examiner, senior psychological examiner, certified psychological assistant, or certified psychological testing technician who engages in psychotherapy shall establish and maintain effective intercommunication with a psychologically oriented physician, usually a psychiatrist, to make provision for the diagnosis and treatment of medical problems by a physician with an unlimited license to practice the healing arts in this state.

(c) A psychologist, psychological examiner, senior psychological examiner, certified psychological assistant, or certified psychological testing technician shall not attempt to diagnose, prescribe for, treat, or advise a client with reference to problems or complaints falling outside the boundaries of psychological practice.

(d) Sections 63-11-203 and 63-11-208(c) and (d) are subject to the limitations of this section.

63-11-205. Use of titles.

(a)

(1) Individuals and organizations shall not present themselves, or be presented, to the public by a title incorporating the name "psychological," "psychologist," or "psychology" other than those licensed as psychologists, psychological examiners, senior psychological examiners, certified psychological assistants, or certified psychological testing technicians.

(2) A psychological scientist employed by a recognized research laboratory, college, or university may use the academic or research title conferred upon the psychological scientist by the administration of the laboratory, college, or university.

(3) Persons licensed by the state board of education in areas of school psychology, school psychologist, or school psychological services worker and employed as such in the department of education or in a public or nonpublic elementary or secondary school are not required to be licensed under this part while performing in the course of their employment. This chapter does not apply to persons licensed by the state board of education in areas of school psychology, school psychologist, or school psychological services worker who are employed as such in the department of education or in a public or nonpublic elementary or secondary school while performing in the course of their employment.

(b) This section does not permit such persons to offer their services to any other persons or organizations as consultants or to accept remuneration for psychological services other than that of their institutional salaries unless they have been licensed or certified under this chapter.

(c) Visiting lecturers from recognized laboratories, colleges, or universities are exempt from this section and may utilize their academic or research title when presenting lectures to similar institutions or organizations.

(d) Students of psychology, psychological interns, and other persons preparing for the profession of psychological examiner, senior psychological examiner, certified psychological assistant, certified psychological testing technician, or psychologist under qualified supervision in recognized training institutions or facilities may be designated by such titles as "psychological intern," "psychological trainee," or others clearly indicating such training status.

(e) Psychologists with competency in areas other than the provision of health services retain the right to use the title of "licensed psychologist" with competence in areas other than health services and to practice within their area of competency.

63-11-206. Unauthorized practice – Exemptions – Temporary licenses.

(a) A person who engages in the practice of psychology and does not then possess in full force and virtue a valid license to practice as a psychological examiner, psychologist, or senior psychological examiner, or a valid certificate to practice as a psychological assistant or psychological testing technician, under this chapter commits a Class B misdemeanor.

(b) This chapter does not prevent members of other recognized professions who are licensed, certified, or regulated under the laws of this state from rendering services consistent with their professional training and code of ethics as long as they do not represent themselves to be psychologists, psychological examiners, senior psychological examiners, certified psychological assistants, or certified psychological testing technicians. Duly recognized members of the clergy are not restricted from functioning in their ministerial capacity as long as they do not represent themselves to be psychologists, psychological examiners, senior psychological examiners, certified psychological assistants, or certified psychological testing technicians.

(c)

(1) Students of psychology, psychological interns, and other persons preparing for the profession of psychological examiner, psychologist, senior psychological examiner, certified psychological assistant, or certified psychological testing technician may perform as a part of their training the functions in §§ 63-11-202 and 63-11-203, but only under qualified supervision.

(2) A person practices as a certified psychological assistant within the meaning of this chapter when the person claims to be a certified psychological assistant or renders to individuals or to the public for remuneration a service involving the application of recognized principles, methods, and procedures of the science and profession of psychology and only under the direct employment and supervision of either the psychologist or senior psychological examiner employing the certified psychological assistant or the employing community

mental health center or state governmental agency. All work performed by a certified psychological assistant must be supervised by a psychologist or senior psychological examiner as determined by the board.

(3) A person practices as a certified psychological testing technician within the meaning of this chapter when the person claims to be a certified psychological testing technician or renders to individuals or the public for remuneration a service involving the application of recognized principles, methods, and procedures of the science and profession of psychology and only under the supervision of either the psychologist or senior psychological examiner employing the certified psychological testing technician or the employing community mental health center or state governmental agency. All work performed by a certified psychological testing technician must be supervised by a psychologist or senior psychological examiner as determined by the board.

(d) Use of psychological techniques by governmental institutions and by business and industrial organizations for employment placement, evaluation, promotion, or job adjustment of their own officers or employees or by employment agencies for the evaluation of their own clients prior to recommendation for employment is also specifically allowed; however, an industrial or business firm or corporation shall not sell or offer to the public or to other firms or corporations for remuneration psychological services as specified in §§ 63-11-202 and 63-11-203, unless the services are performed or supervised by individuals duly and appropriately licensed under this chapter.

(e)

(1)

(A)

(i) The board may issue a temporary license to an applicant who has successfully completed the academic course

work and training for the license sought, and who has not yet taken the required written examinations.

(ii) A temporary license is valid for a period not to exceed one (1) year. The board may extend the expiration date of a temporary license following a petition from the licensee and finding that extraordinary circumstances exist that necessitate a delay in taking a required written examination prior to the expiration of the initial temporary license period.

(iii) A temporary license is revoked thirty (30) days after the date a temporary licensee receives notice of the licensee's second failure of the examination for professional practice in psychology (EPPP) or jurisprudence examination. During the thirty-day period prior to revocation of the temporary license, the licensee and the licensee's supervisor shall ensure the notification and transfer of care for the temporary licensee's clients.

(B) The temporary license allows the applicant to perform the functions specified in §§ 63-11-202 and 63-11-203, for which the applicant is seeking licensure under qualified supervision.

(C) In order to receive a temporary license, the applicant must submit a completed application for a temporary license with the applicant's completed application for the desired license and all appropriate fees to the board.

(2)

(A)

(i) A temporary license may be issued by the board for an applicant who has a current license from another state or territory. The temporary license allows the applicant to perform under

qualified supervision the functions specified in §§ 63-11-202 and 63-11-203.

(ii) In order to receive a temporary license, the applicant must submit a completed application for a temporary license with the applicant's completed application for the desired license and all appropriate fees to the board.

(iii) Applications for temporary licenses for applicants currently licensed in another state or territory must be reviewed and approved or denied by a current member of the board designated by the board to perform this function.

(iv) If an applicant is issued a temporary license, then the license remains valid until the board grants or denies the license application.

(v) However, a person must not be issued more than one (1) temporary license, and a temporary license must not be valid for a period of more than one (1) year.

(B) Such a temporary license may be issued to allow an applicant to perform the functions specified in § 63-11-203 if the applicant has:

(i) Successfully completed an approved American Psychological Association academic course of study;

(ii) Previously taken and passed, according to this state's standards, the national licensing examination; and

(iii) A current license in another state or territory in good standing.

(C) Such a temporary license with designation as a health service provider may be issued to an applicant who meets the requirements of subdivision (e)(2)(B) and has:

(i) Successfully completed an American Psychological Association-approved internship; and

(ii) Had at least one (1) year of full-time postdoctoral experience delivering health services.

(D) Such a temporary license may be issued to allow an applicant to perform the functions specified in § 63-11-202 if the applicant has:

(i) Previously taken and passed, according to Tennessee standards, the national licensing examination; and

(ii) A current license in another state or territory in good standing.

(3) The board shall set the fee for all temporary licenses in an amount not to exceed one hundred dollars (\$100). Until such time as the board establishes a fee by rule, the fee is fifty dollars (\$50.00).

(f) This chapter does not expand or limit, in any way, the practice of psychological examiners as provided in § 63-11-202, senior psychological examiners as provided in § 63-11-202, or certified psychological assistants as provided in subdivision (c)(2) and § 63-11-207(b), and does not limit the use of the title "psychological examiner" for persons duly licensed as psychological examiners.

(g) Notwithstanding subsection (a), persons providing services in programs of the department of mental health and substance abuse services or the department of disability and aging either as employees or through contract agencies are not engaged in the practice of psychology and need not be licensed or certified under this chapter unless such persons provide services as a psychological examiner, psychologist, senior psychological examiner, certified psychological assistant, or certified psychological testing technician.

(h)

(1) A provisional license to practice as a psychologist with the designation as a health service provider may be issued by the board of examiners in psychology to an applicant who has successfully completed the academic and internship requirements of § 63-11-208(c)(1) and (d)(1) and who is to be engaged in a one-year postdoctoral experience required by § 63-11-208(d)(2)(A). The provisional licensure allows the applicant to perform the functions specified in §§ 63-11-203 and 63-11-208(d)(2)(B) under the qualified supervision of a psychologist acceptable to the board and who is licensed in this state with a designation as a health service provider in good standing.

(2) In order for a provisional license to remain valid, the licensee must take and pass the examination for professional practice in psychology (EPPP) at the level set for licensure as a psychologist in this state within one (1) year of the issuance of the provisional license unless the applicant previously passed the EPPP at that level. A provisional license is revoked thirty (30) days after the date a provisional licensee receives notice of the licensee's second failure of the EPPP following issuance of the provisional license. During the thirty-day period prior to revocation of the provisional license, the licensee and the licensee's supervisor shall ensure the notification and transfer of care for the provisional licensee's clients.

(3) In order for the provisional license to remain valid, the licensee must take and pass the oral or jurisprudence examination, or both, developed by the board within two (2) years of the issuance of the provisional license. A provisional license is revoked thirty (30) days after the date a provisional licensee receives notice of the licensee's second failure of the oral or jurisprudence examination following issuance of the provisional license. During the thirty-day period prior to revocation of the provisional license, the licensee and the

licensee's supervisor shall ensure the notification and transfer of care for the provisional licensee's clients.

(4) The board may consider petitions of the licensee regarding extraordinary circumstances that would necessitate delay in taking the written or oral examination.

63-11-207. License for psychological examiner, certified psychological assistant, and certified psychological testing technician.

(a) The board shall not issue new psychological examiner licenses. The discontinued issuance of psychological examiner licenses or the designation of senior psychological examiners in no way impairs or abridges the right of duly licensed psychological examiners to practice within the scope of their license or to renew their licensure status as required by law. Unless the person has obtained a license to practice as a psychological examiner or senior psychological examiner, as issued by the board, it is unlawful for the person to practice. A person violates this chapter if the person practices as a certified psychological testing technician without first having obtained a proper certificate.

(b)

(1) A person seeking to practice as a certified psychological assistant in this state shall, before it is lawful for the person to practice as a certified psychological assistant, make application to the board, upon such form and in such manner as is adopted and prescribed by the board, including, but not limited to, the proper registration with the board of the supervisor of record for the certified psychological assistant. Unless the person obtains a proper certificate as issued by the board, it is unlawful for a person to practice, and if the person practices as a certified psychological assistant without first having obtained a proper certificate, then the person has violated this chapter.

(2) In addition to the requirements of § 63-11-107, a candidate for certification shall furnish the board with satisfactory evidence that the candidate:

(A) Is not considered by the board to be engaged in unethical practice; and

(B) Has had two (2) academic years of graduate training in psychology, including a master's degree, from an accredited educational institution recognized by the board as maintaining satisfactory standards, or in lieu thereof, such training and experience as the board considers equivalent thereto. Such graduate training must include specialized training in psychological assessment, test construction and measurement, the administration and interpretation of psychological testing, interviewing skills, and social history methods. The board may, at its discretion, consider equivalent training and experience when presented by qualified applicants with a master's degree in psychology whose credentials differ slightly than those prescribed in this subdivision (b)(2)(B). However, all applicants must in their graduate training show evidence of supervised practicum experience in the administration and interpretation of psychological testing, assessment procedures, and social history and feedback interviewing techniques as determined by the board in order to fulfill educational requirements for certification.

(c)

(1) A person seeking to practice as a certified psychological testing technician in this state shall, before it is lawful for the person to practice as a certified psychological testing technician, make application to the board, upon such form and in such manner as is adopted and prescribed by the board, including, but not limited to, the proper registration with the board of the supervisor or supervisors of record for the certified psychological testing

technician. Unless the person obtains a proper certificate as issued by the board, it is unlawful for the person to practice. A person violates this chapter if the person practices as a certified psychological testing technician without first having obtained a proper certificate.

(2) As used in subdivision (c)(1), "practice as a certified psychological testing technician" means the individual is duly authorized by the board to administer and score standardized psychological and neuropsychological tests and to observe and describe a client's test behavior and test responses. A certified psychological testing technician shall not select psychological or neuropsychological tests or versions of tests to be given, interpret test results, write test results, or give test feedback to clients.

(3) The services provided by a certified psychological testing technician must be provided under the supervision of a psychologist or senior psychological examiner who is registered with the board as a supervisor of record to provide such supervision for the certified psychological testing technician. The certified psychological testing technician must be employed by, or contracted with, the same work setting as the supervising psychologist or senior psychological examiner who is registered with the board.

(4) In addition to the requirements of § 63-11-107, a candidate for certification shall furnish the board with satisfactory evidence that the candidate:

(A) Is not considered by the board to be engaged in unethical practice; and

(B) Proof of completion of a bachelor's degree or graduate degree, or proof of current enrollment in a graduate degree program, from a regionally accredited university, college, or professional school, in either psychology or education with a specialization in educational psychology, counseling psychology, or school psychology. The candidate's

educational training must consist of a minimum of seventy-two (72) hours of total education and training relating to psychological or neuropsychological test administration and scoring that includes the following:

(i) At least twenty (20) hours of direct observation, including at least ten (10) hours of direct observation of a licensed psychologist or senior psychological examiner administering and scoring tests, and at least ten (10) hours of direct observation of either a licensed psychologist, senior psychological examiner, or certified psychological testing technician duly registered by the board administering and scoring tests;

(ii) At least forty (40) hours of administering and scoring tests in the presence of a licensed psychologist or senior psychological examiner; and

(iii) At least twelve (12) hours of additional education on topics related to best practices for the functions identified in subdivision (c)(2). Of those twelve (12) hours, at least three (3) hours must be specifically dedicated to law and ethics, and at least three (3) hours must be dedicated to multicultural competency. Additional topics must relate to growth, knowledge, and skill-based training that would enhance the candidate's professional skill in providing care for persons with medical, psychiatric, neurocognitive, and developmental conditions commonly assessed by psychologists and neuropsychologists.

(5) The education and training specified in subdivisions (c)(4)(B)(i)-(iii) may be obtained through any combination of the following:

(A) Participating in individual or group instruction by a licensed psychologist or senior psychological examiner;

(B) Engaging in independent learning directed by a licensed psychologist or senior psychological examiner;

(C) Completing graduate-level coursework at a regionally accredited university, college, or professional school; or

(D) Taking continuing education courses from organizations offering continuing education sponsorships.

(6) This chapter does not prevent a person engaged in gaining the experience, training, or education required by subdivision (c)(4)(B) from administering and scoring psychological and neuropsychological tests.

63-11-208. License for psychologist.

(a) A person wishing to obtain the right to practice as a psychologist in this state, who has not been licensed on or before the effective date of this act, shall, before it is lawful for the person to practice psychology in this state, make application to the board through the chair upon such form and in such manner as is adopted and prescribed by the board.

(b) Unless the person has obtained a license as prescribed in subsection (a), it is unlawful for the person to practice, and if the person practices psychology without first having obtained such a license, then the person has violated this chapter.

(c)

(1) In addition to the requirements of § 63-11-107, a candidate for such license shall furnish the board with satisfactory evidence that the candidate:

(A) Has received a doctorate in psychology from an accredited educational institution recognized by the board as maintaining satisfactory standards. After December 31, 1995, all such doctoral programs must be approved by the American Psychological Association and listed by the

American Association of State Psychology Boards, or meet equivalent standards as are determined by the board;

(B) Is competent in psychology, as shown by passing such examinations, written or oral, or both, as the board deems necessary;

(C) Is not considered by the board to be engaged in unethical practice; and

(D) Has not within the preceding six (6) months failed an examination given by the board.

(2) The board may, at its discretion, consider equivalent training and experience when presented by qualified applicants with a doctoral degree in psychology whose credentials differ slightly than those prescribed in subdivisions (c)(1)(A) and (d)(2)(A). In such cases, the applicant must petition the board for a waiver and specifically request the board utilize its discretionary authority.

(d) A licensed psychologist who provides or offers health services to the public must be designated by the board as a health service provider. In addition to the requirements for licensure as a psychologist prescribed in subsection (c), an applicant for designation as a health service provider must demonstrate to the board that the applicant:

(1) Has had at least a one-year internship at a site where health services are normally provided and that is part of an organized integrated training program. After December 30, 1995, all such internships must be approved by the American Psychological Association (APA) or listed by the Association of Psychological Post-Doctoral and Internship Centers (APPIC), or meet equivalent standards as are determined by the board; and

(2)

(A) Has successfully completed at least one (1) year postdoctoral experience delivering health services, supervised by a licensed

psychologist designated by the board as a health service provider. If the one (1) year of postdoctoral experience was not in this state, then equivalent supervision must be documented to the satisfaction of the board; and

(B) For purposes of this chapter, "health service," in any variation, means the delivery of direct, preventive, assessment, and therapeutic intervention services to individuals whose growth, adjustment, or functioning is actually impaired or may be at risk of impairment.

(e) Notwithstanding subsection (d), the board may designate as a health service provider, with practice restriction to recipients in an intellectual disabilities facility under the department of disability and aging, a person who meets all of the following criteria:

(1) The person holds a doctorate in psychology from a regionally accredited institution;

(2) The person is currently licensed as a psychologist in at least one (1) state;

(3) The person has academic training in intellectual disability and postdoctoral clinical experience in the field of intellectual disability of at least five (5) years' duration;

(4) The department submits to the board a description of the applicant's proposed functioning at the intellectual disability facility;

(5) The department certifies that:

(A) The applicant has training and experience adequate to perform the applicant's proposed functioning within the department;

(B) The department is unable to recruit otherwise qualified psychologists to provide psychological services; and

(C) This inability threatens the integrity of the department's services and the ability of the department to meet the needs of its clients; and

(6) A psychologist granted designation as a health service provider by the board under this subsection (e) shall surrender this designation upon termination of the person's employment with the department, which designation is, thereafter, null and void.

(f)

(1) Notwithstanding subsection (d), the board may designate as a health service provider, a person who has held a valid license or certificate in another state to practice psychology for at least ten (10) years within the last fifteen (15) years preceding the date of application and whose previous license or certification:

(A) Required training that is generally equivalent to the following licensing standards in this chapter:

(i) A doctoral degree in psychology;

(ii) One (1) year of internship in an organized integrated training program at a site delivering health services; and

(iii) One (1) year of postdoctoral training delivering health services;

(B) Has never been the subject of disciplinary action; and

(C) Allowed the licensee to engage in practice as a health service provider in psychology in the other state.

(2) In order to effectuate the purposes of subdivision (f)(1), the board may promulgate emergency rules pursuant to § 4-5-208(a)(5). The rules must be promulgated in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

63-11-209. Examination of applicants for a license or certificate to practice psychology.

(a) Examination of applicants for a license or certificate to practice psychology may be made by the board at least once a year according to methods and in such subject fields as may be deemed by the board to be the most practical and expeditious to test the applicant's qualifications. Alternatively, the board may accept a nationally recognized test.

(b)

(1) Such examinations must include the basic psychological sciences.

(2) The board shall require the examinations to be written or oral, or both; provided, that in any written examination, the applicant is designated by a number instead of the applicant's name so that the applicant's identity is not disclosed to the members of the board until the examination papers have been graded.

(3) The board shall grade the written examinations returned by the candidates and keep them for at least one (1) year.

(4) A candidate is held to have passed the examination upon the affirmative vote of three (3) or more members of the board.

(5) An unsuccessful candidate may, upon written request to the board, see the candidate's graded paper.

63-11-210. Certification without examination.

(a) The board may, at its discretion, grant a certificate without an assembled examination to a person residing or employed in the state who has not previously failed the appropriate examination given by the board and who, at the time of application, is licensed or certified by a similar board of another state whose standards, in the opinion of the board, are not lower than those required by this chapter, or who has been practicing psychology in another state and has qualifications not lower than those

required by this chapter, and is able to satisfy to the board that to grant that person a license would be in the public interest, or who has been certified by the American Board of Examiners in Professional Psychology.

(b)

(1) Notwithstanding subsection (a), the board shall grant a license without an assembled written or oral examination to a person who at the time of application:

(A) Is licensed or certified in good standing with a similar board of another state;

(B) Has passed the National Examination for the Practice of Professional Psychology at a level greater than or equal to the level required in this state;

(C) Can show reasonable proof that the applicant has available employment in this state by means of letters or other reasonable proof; and

(D) Received a psychologist license after January 1, 1995, but before June 1, 1995.

(2) The board may permit a psychologist licensed in good standing in another state, who meets standards acceptable to the board, to perform the functions of §§ 63-11-203 and 63-11-208(d)(2)(B) and practice as a psychologist in this state without possessing a current license for a period of time, not to exceed twelve (12) days per year, for such purposes as special training or consultation, special evaluation, or intervention or serving as an expert witness.

(c) This section does not permit the regular, repetitive, or ongoing provision of psychological services, the supervision of psychological services, or the solicitation or advertisement of services to the general public, all of which are governed by the usual and customary processes of licensure for psychologists.

63-11-211. Confidential relations and privileged communications.

For the purpose of this chapter, the confidential relations and communications between licensed psychologist, licensed psychological examiner, licensed senior psychological examiner, certified psychological assistant, or certified psychological testing technician and client are placed upon the same basis as those provided by law between attorney and client. This chapter does not require such privileged communication to be disclosed.

63-11-212. Code of ethics.

The board of examiners shall, by rule, adopt a code of ethics to govern appropriate practices or behavior as referred to in §§ 63-11-109, 63-11-207, and 63-11-208.

63-11-213. Peer review – Immunity.

(a) As used in this section, "peer review committee" or "committee" means a committee, board, commission, or other entity constituted by a statewide psychological association or local psychological association for the purpose of receiving and evaluating professional acts of other practitioners of psychology as defined in this chapter.

(b) A practitioner of psychology who serves on a peer review committee or on any other committee is immune from liability with respect to an action taken in good faith and without malice as a member of such committee, board, commission, or other entity.

(c) Practitioners of psychology, individuals appointed to a committee as defined in subsection (a), and members of the boards of directors of facilities, as described in § 68-11-201, are immune from liability to a client, patient, individual, or organization for furnishing information, data, reports, or records to any such committee or for damages resulting from decisions, opinions, actions, or proceedings rendered, entered, or acted upon by such committee, undertaken or performed within the scope or functions of the duties of such committee, if made or taken in good faith and without malice and on the basis of facts reasonably known or reasonably believed to exist.

(d)

(1) All information, interviews, reports, statements, memoranda, or other data furnished to a peer review committee, associational board, or governing board, and any findings, conclusions, or recommendations resulting from the proceedings of such committees, associational boards, and governing boards are privileged.

(2) The records and proceedings are confidential and must be used only in the exercise of the proper functions of the committees, associational boards, or governing boards and not become public records nor be available for court subpoena or discovery proceedings.

(3) This subsection (d) does not apply to records, documents, or information otherwise available from original sources, and such records, documents, or information are not immune from discovery or use in civil proceedings solely due to presentation to the committee.

63-11-214. Internships.

A person currently enrolled in an internship program satisfying the requirements of § 63-11-208(d)(1) and who completes the internship and all other degree requirements by December 31, 1992, must be designated as a health service provider without completing the one-year full-time supervised postdoctoral experience as provided in § 63-11-208(d)(2) if an application for such designation is submitted by July 1, 1993.

63-11-215. Health service provider designation – Criteria.

(a) For purposes of licensure renewal, a psychologist licensed under this chapter as a school, clinical, or counseling psychologist who was designated by the board as a health service provider by July 1, 1993, must continue to be renewed by the board with a designation of health service provider as long as the psychologist meets all other requirements for a renewal license.

(b) A psychologist licensed under this chapter other than as a school, clinical, or counseling psychologist who was designated by the board as a health service provider by July 1, 1993, must continue to be renewed by the board with a designation of health service provider as long as the psychologist meets all other requirements for licensure renewal.

63-11-216. Definitions for federal legislation and regulations and this code.

For purposes of federal legislation and regulations referencing qualifications with respect to the provision of psychological services and other titles in this code, unless the context otherwise requires, "licensed clinical psychologist," "licensed counseling psychologist," or "licensed school psychologist," in any variation, refers to a psychologist licensed under this chapter and designated as a health service provider.

63-11-217. Third-party reimbursement.

Community mental health centers, as defined in § 33-1-101; institutes or governmental agencies; doctoral clinical psychology students in university-operated training clinics; and those qualifying psychologists employing psychologists with temporary or provisional licenses are eligible to receive third-party reimbursement from a managed care plan or third-party payor for usual and customary professional services rendered by the employed psychologist with a provisional or temporary license.

63-11-218. Medical practitioners in free health clinics – Special volunteer license.

A psychologist licensed pursuant to this chapter under a special volunteer license who is a medical practitioner, as defined by § 63-1-201, engaged in practice at a free health clinic is not subject to license fees under this chapter. The board of examiners in psychology may issue a special volunteer license, as such license is defined in § 63-1-201, to qualified applicants without fee or charge. Such license is for a period of two (2) years and may be renewed on a biennial basis.

63-11-219. Notification of departure from community mental health center –

Patient records.

Psychologists, senior psychological examiners, and psychological examiners are not required to notify patients who were treated by them at community mental health centers, as defined in § 33-1-101, of their departure from such community mental health centers. Patient records must remain with the community mental health centers. The transfer of patients' records must be in compliance with standards set by state and federal law.

SECTION 53. Tennessee Code Annotated, Title 63, Chapter 11, is amended by deleting part 3 and substituting:

63-11-301. Practice of behavior analysis – License.

(a) The practice of behavior analysis in this state is declared to affect the public health, safety, and welfare of citizens of this state.

(b) The board of examiners in psychology is authorized to protect the public from:

(1) The practice of applied behavior analysis by unqualified persons; and

(2) Unprofessional, unethical, and harmful conduct by behavior analysis practitioners.

(c) A person shall not represent to be, or function as, a behavior analyst or assistant behavior analyst in this state unless the person holds a valid license issued by the board. The board shall also regulate the practice of applied behavior analysis.

63-11-302. Part definitions.

As used in this part:

(1) "Certifying entity" or "BACB" means the nationally accredited behavior analyst certification board or its successor;

(2) "Co-employed relationship" means the licensed behavior analyst and licensed assistant behavior analyst are employed by the same employer;

(3) "Employee-employer relationship":

(A) Means the licensed behavior analyst is the employer of the licensed assistant behavior analyst; and

(B) Includes contractual employment or consultant services;

(4) "Human services professional" means an individual licensed or certified in this state as a:

(A) Physical therapist or physical therapist assistant;

(B) Occupational therapist or occupational therapist assistant;

(C) Clinical social worker;

(D) Masters social worker;

(E) Social work associate;

(F) Psychologist or health service psychologist;

(G) Speech pathologist;

(H) Audiologist;

(I) Professional counselor or professional counselor candidate;

(J) Marital and family therapist or marital and family therapist candidate;

(K) Licensed psychological examiner; or

(L) Senior licensed psychological examiner;

(5) "Licensed assistant behavior analyst" or "LABA" means an individual who is certified as a board certified assistant behavior analyst (BCABA) and is licensed in this state to practice applied behavior analysis under the extended authority and supervision of a licensed behavior analyst;

(6) "Licensed behavior analyst" or "LBA" means an individual who is certified as a board certified behavior analyst (BCBA) or board certified behavior analyst-doctoral (BCBA-D) and is licensed in this state to practice applied behavior analysis as an independent practitioner;

(7) "Practice of applied behavior analysis":

(A) Means the design, implementation, and evaluation of environmental modifications by a behavior analyst to produce socially significant improvements in human behavior. Applied behavior analysis (ABA) interventions are based on scientific research and the direct observation and measurement of behavior and environment. They utilize contextual factors, motivating operations, antecedent stimuli, positive reinforcement, and other procedures to help people develop new behaviors, increase or decrease existing behaviors, and emit behaviors under specific environmental conditions;

(B) Includes the empirical identification of functional relations between behavior and environmental factors, known as functional assessment and analysis; and

(C) Expressly excludes psychological testing, neuropsychology, psychotherapy, cognitive therapy, sex therapy, psychoanalysis, hypnotherapy, and long-term counseling as treatment modalities;

(8) "Real-time supervision" means observation and provision of feedback to a supervisee's delivery of ABA services by an LBA or an LABA who is supervised by an LBA during interactions occurring in person or via technology that permits auditory and visual contact between supervisors and supervisees;

(9) "Supervisee" means a person who delivers ABA services under the extended authority and supervision of an LBA or an LABA who is supervised by an LBA; and

(10) "Supervision" means the direct observation and provision of feedback to a supervisee by an LBA or an LABA who is supervised by an LBA.

63-11-303. Application.

In addition to the requirements of § 63-11-107, each person applying to obtain a license pursuant to this part shall submit an application that includes evidence that the applicant:

(1) Is at least twenty-one (21) years of age;

(2) Conducts the person's professional activities in accordance with accepted professional and ethical standards, including the certifying entity's Professional Disciplinary and Ethical Standards and Guidelines for Responsible Conduct for Behavior Analysts; and

(3)

(A) LBA applicants must:

(i) Be certified as a BCBA or board certified behavior analyst-doctoral at the time of application; and

(ii) Not be the subject of disciplinary actions by the certifying entity; and

(B) LABA applicants must:

(i) Be certified as a board certified assistant behavior analyst at the time of application;

(ii) Not be the subject of disciplinary actions by the certifying entity; and

(iii) Provide proof of ongoing supervision by an LBA who is currently certified as a BCBA or board certified behavior analyst-doctoral in a manner consistent with the certifying entity's requirements for supervision of board certified assistant behavior analysts.

63-11-304. Prohibited conduct.

(a) A person shall not use the title "licensed behavior analyst" or "licensed assistant behavior analyst" unless that person holds the applicable license issued by the board.

(b) A person shall not practice applied behavior analysis unless that person is licensed by the board as an LBA or an LABA or meets criteria for one (1) of the exemptions in § 63-11-305.

(c) A person who violates this section commits a Class B misdemeanor, punishable by a fine only, the suspension or revocation of a license issued pursuant to this part, or both.

63-11-305. Exemptions.

This part does not prohibit or restrict the practice of the following:

(1) Other human services professionals who are licensed, registered, or certified by the state if the individuals are working within the scope of practice of their professions and the scope of their training and competence;

(2) Unlicensed persons who deliver applied behavior analysis (ABA) services under the extended authority and direction of an LBA or an LABA who is supervised by an LBA. Such persons shall not represent themselves as professional behavior analysts;

(3) Family members of recipients of ABA services who implement certain ABA procedures with recipients under the extended authority and direction of LBAs or LABAs who are supervised by LBAs. Such persons shall not represent themselves as professional behavior analysts;

(4) Behavior analysts who practice with nonhumans, including applied animal behaviorists and animal trainers. Such persons may use the title "behavior analyst" but shall not represent themselves as LBAs or LABAs unless they hold one (1) of the credentials issued by the board;

(5) Persons who provide general ABA services to organizations, so long as those services are for the benefit of the organizations and do not involve direct services to individuals;

(6) Matriculated college or university students, interns, or postdoctoral fellows whose activities are part of a defined behavior analysis program of study, practicum, or intensive practicum if the practice under this exemption is directly supervised by an LBA in this state or an instructor in a BACB-approved course sequence. Students, interns, and postdoctoral fellows shall not represent themselves as professional behavior analysts but shall use titles that clearly indicate their trainee status, such as "behavior analysis student," "behavior analysis intern," or "behavior analysis trainee";

(7) Persons who teach behavior analysis or conduct behavior analytic research if such teaching or research does not involve the delivery of direct behavior analytic services. Such persons may use the title "behavior analyst" but shall not represent themselves as LBAs or LABAs unless they hold one (1) of the credentials issued by the board; and

(8) Unlicensed persons pursuing experiential training in behavior analysis consistent with the BACB's experience requirements if experience is supervised by an LBA in this state who meets BACB supervisor requirements and the supervised experience is conducted in accordance with other BACB standards and requirements.

63-11-306. Licensed behavior analyst supervision of licensed assistant behavior analysts.

(a) An LBA shall provide supervision of applied behavior analysis (ABA) services under the LBA's extended authority and direction to all designated persons as provided in this part.

(b) An LABA shall work under the supervision of an LBA.

(c)

(1) An LBA shall provide supervision to a maximum of six (6) LABAs concurrently, with the supervision to be conducted as follows:

(A) A minimum of two (2) hours for each forty (40) hours of services provided by an LABA;

(B) A minimum of one-half (1/2) of all supervision hours must involve real-time interactions; and

(C) A minimum of one-half (1/2) of all supervision hours must involve one-to-one interactions between supervisors and supervisees. The remainder may be conducted in small group format.

(2) Acceptable supervision activities include:

(A) Direct observation of the LABA delivering ABA services to service recipients;

(B) Direct observation of the LABA training family members and staff to implement ABA assessment or intervention procedures;

(C) Demonstrating ABA assessment or intervention procedures;

(D) Coaching supervisees in implementing ABA assessment or intervention procedures;

(E) Reviewing and discussing treatment goals and procedures;

(F) Reviewing service recipient progress data; and

(G) Reviewing and discussing relevant research articles and other professional literature.

(d) A formal professional relationship must exist between a supervising LBA and LABA in the form of:

(1) A co-employed relationship; or

(2) An employee-employer relationship.

(e) LABAs shall notify the board within ten (10) business days of a change in supervision status.

63-11-307. Professional conduct – Disciplinary sanctions.

(a) All licensees shall comply with their respective certifying entity's Guidelines for Responsible Conduct and Professional Disciplinary and Ethical Standards.

(b) A person licensed under this part who receives disciplinary sanctions from the person's certifying entity shall report such sanctions to the committee within five (5) business days after the disciplinary sanctions have been imposed. Such sanctions may include, but are not limited to, the denial of initial or renewal certification, revocation, suspension, or any other limitation of certification or combination of sanctions. A licensee who loses certification for any reason immediately forfeits their license issued by the board and shall stop providing ABA services in this state.

(c) In addition to the criteria listed in § 63-11-109(b), the board may deny, suspend, revoke, or otherwise condition or discipline a license issued under this part, as set-forth in § 63-11-109 as a result of:

(1) The failure of the licensee to comply with subsection (b);

(2) Abandonment of a service recipient resulting in the termination of imminently needed care without adequate notice or provision for transition;

(3) Professional recordkeeping or data collection that constitutes an extreme and unjustified deviation from the customary standard of practice for the field, or deceptively altering a service recipient's records or data; or

(4) Limitation, sanction, revocation, or suspension by a healthcare organization, professional organization, or other private body, relating to behavior analysis practice, public health or safety, or behavior analysis certification or licensure.

63-11-308. Persons licensed in other states.

(a) The board may, in its discretion, grant a license to a person residing or employed in this state who has not previously failed to attain a license as a behavior analyst or assistant behavior analyst and who, at the time of application:

(1) Is licensed or certified by a similar entity in another state whose standards, in the opinion of the board, are not less stringent than those required by this part; and

(2) Is able to satisfy to the board that to grant the person a license would be in the public interest.

(b) Notwithstanding subsection (a), the board shall grant a license to a person who at the time of application:

(1) Is licensed or certified in good standing with a similar entity of another state;

(2) Is certified as a BCBA, a BCBA-D, or a BCABA; and

(3) Can show reasonable proof that the applicant has available employment in this state.

63-11-309. Behavior scales.

(a) An LBA may administer behavioral scales that are within the scope of the practice of applied behavior analysis.

(b) When administering behavioral scales, an LBA:

(1) Shall only use the results of the behavioral scales to guide treatment planning or monitor the progress of treatment for behavioral disorders;

(2) Shall not interpret the overall results or use the results of the scales for diagnosis, an overall conceptualization of psychological functioning, intelligence, or neuropsychological functioning; and

(3) May submit the scores derived from behavioral scales to an insurer if the LBA is required to do so.

63-11-310. Transfer of rules, and licensing and regulatory authority, to the board of examiners in psychology.

(a) The secretary of state shall transfer all rules promulgated by the [former] applied behavior analyst licensing committee [repealed] and in effect on the effective date of this act to the board of examiners in psychology.

(b) Pursuant to the transfer of rules under subsection (a), the board of examiners in psychology is authorized to regulate behavior analysts, including applying and enforcing the rules promulgated by the [former] applied behavior analyst licensing committee [repealed] upon the effective date of this act.

SECTION 54. Tennessee Code Annotated, Title 63, Chapter 11, Part 4, is amended by deleting the part and substituting:

63-11-401. License for alcohol and drug abuse counselors.

The board shall license the minimal competence of alcohol and drug abuse counselors based on satisfactory completion of its licensure process, including passing examinations, maintaining competence and professional standards, and paying reasonable fees.

63-11-402. Limits of licensed alcohol and drug abuse counselors.

(a) This part does not permit a person licensed as an alcohol or drug abuse counselor to engage in the practice of medicine or to engage in the practice of a licensed psychologist, licensed psychological examiner, licensed senior psychological examiner, certified psychological assistant, licensed social worker, licensed professional counselor, licensed marriage and family therapist, or psychiatric nurse.

(b) Persons licensed as alcohol and drug abuse counselors are not authorized by this part to dispense or prescribe drugs.

(c) This part does not constrict or limit the practice of medicine, the certification and licensing of social workers, the licensing of nurses, the licensing of psychologists, or the licensing of professional counselors and marriage and family therapists.

(d) This part does not prevent qualified members of such professions from advertising that such persons diagnose and treat alcohol and drug disorders within the scope of their respective professions or from actually diagnosing and treating alcohol and drug disorders within the scope of their respective professions.

63-11-403. Scope of counseling activities.

(a) Alcohol and other drug abuse counseling includes the following:

(1) The evaluation and treatment of problems and misconceptions of persons who abuse mood-altering chemicals within the context of individual, group, familial, and significant other systems;

(2) The evaluation and treatment of those persons who have had their lives significantly impacted by another person's use of alcohol or other abuse of drugs. Alcohol and other drug abuse counseling includes the provision of the primary functions that may be performed by licensed alcohol and drug abuse counselors; and

(3) The evaluation and treatment of those persons who have had their lives significantly impacted by compulsive gambling disorder; provided, however, that to evaluate or treat a person significantly impacted by compulsive gambling disorder, a counselor must have no less than sixty (60) additional hours of specialized education relating to compulsive gambling disorder. The education must be in the form of formal classroom hours or annual continuing educational hours, or a combination of such hours; provided further, that an applicant for license as a part of the applicant's clinically supervised counseling experience must have experience with no less than ten (10) patients impacted by compulsive gambling disorder. A qualified supervisor must have no less than sixty (60) formal classroom hours of instruction related to compulsive gambling disorder to supervise a counselor relative to the evaluation and treatment of compulsive gambling disorder.

(b) This part does not permit a person licensed as an alcohol and drug abuse counselor to perform psychological testing intended to measure or diagnose mental illness. Consistent with each counselor's formal education and training, licensed alcohol and drug abuse counselors may administer and use appropriate assessment instruments which identify elements of perceptual inability to recognize empirical facts, problems of appropriately displaying emotions, and inappropriate responses to the environment of individuals, couples, and families as part of the alcohol and other drug abuse therapy process or in the development of a treatment plan in the context of chemical abuse systems.

63-11-404. Professional practice domains.

(a) The purpose of this section is to adopt nationally recognized and accepted professional practice domains for licensed alcohol and drug abuse counselors.

(b) The professional practice domains and transdisciplinary competencies in which licensed alcohol and drug abuse counselors may engage are as follows:

- (1) Clinical evaluation;
- (2) Treatment planning;
- (3) Referral;
- (4) Service coordination;
- (5) Counseling;
- (6) Client, family, and community education;
- (7) Documentation; and
- (8) Professional and ethical responsibilities.

63-11-405. Transfer of rules, and licensing and regulatory authority, to the board of examiners in psychology.

(a) The secretary of state shall transfer all rules promulgated by the [former] board of alcohol and drug abuse counselors [repealed] and in effect on the effective date of this act to the board of examiners in psychology.

(b) Pursuant to the transfer of rules under subsection (a), the board of examiners in psychology is authorized to regulate behavior analysts, including applying and enforcing the rules promulgated by the [former] board of alcohol and drug abuse counselors [repealed] upon the effective date of this act.

SECTION 55. Tennessee Code Annotated, Title 63, Chapter 11, is amended by deleting part 6.

SECTION 56. Tennessee Code Annotated, Title 63, Chapter 13, is amended by deleting parts 1, 2, and 3 and renumbering parts 4 and 5 as new parts 1 and 2.

SECTION 57. Tennessee Code Annotated, Title 63, is amended by deleting chapters 14 and 16.

SECTION 58. Tennessee Code Annotated, Title 63, Chapter 17, is amended by deleting part 2 and substituting:

63-17-201. Part definitions.

As used in this part, unless the context otherwise requires:

(1) "Apprentice licensee applicant" means a person who is registered with the board, who is engaged in an approved licensing program, who is sponsored by a hearing instrument specialist duly licensed in this state, and who meets the qualifications of § 63-17-207;

(2) "Apprentice program" means sixty (60) classroom hours of prescribed coursework under the direct supervision of a hearing instrument specialist, with such coursework pertaining to all aspects of hearing instrument dispensing;

(3) "Board" means the board of communication disorders and sciences created by § 63-17-104;

(4) "Cerumen" means a wax-like secretion from glands in the external auditory canal;

(5) "Commissioner" means the commissioner of health;

(6) "Division" means the division of health related boards in the department of health;

(7) "Hearing instrument" means an instrument or device designed for or represented as aiding, improving, or correcting defective human hearing and any parts, attachments, or accessories of such an instrument or device;

(8) "Licensed hearing instrument specialist" means a person licensed by this part who has passed a board-approved psychometrically sound examination for hearing instrument specialists;

(9) "Otolaryngologist" means a physician specialist dedicated to the care of patients with disorders of the ears, nose, throat, and related structures of the head and neck, commonly referred to as ENTs;

(10) "Practice of dispensing and fitting hearing instruments" includes:

(A) The evaluation or measurement of the powers or range of human hearing by means of an audiometer for the consequent selection or adaptation for sale of hearing instruments intended to compensate for hearing loss, and the appropriate instructions, consultations, suggestions, recommendations, or opinions related to this practice;

(B) Making an impression of the ear, or an ear mold; and

(C) Cerumen management in the course of examining ears by a person who holds a certification of completion of a cerumen management course, as described in § 63-17-222; and

(11) "Sell" or "sale" includes a transfer of title or of the right to use by lease, bailment, or any other contract, either oral or written, except wholesale sales to distributors or dispensers.

63-17-202. Powers of the board.

In addition to the powers granted to the board in this chapter, the board shall:

- (1) Supervise and administer qualifying examinations to test the knowledge and proficiency of applicants for a license under this part;
- (2) License persons who apply to the board under this part and who are qualified to practice the fitting of hearing instruments;
- (3) Purchase and maintain or rent audiometric equipment and facilities necessary to carry out the examination of applicants for license under this part;
- (4) Issue and renew licenses under this part;
- (5) Deny, suspend, or revoke licenses pursuant to this part;
- (6) Appoint representatives to conduct or supervise the examination of applicants for license under this part;
- (7) Designate the time and place for examining applicants for license under this part;
- (8) Make and publish rules not inconsistent with the laws of this state that are necessary to carry out this part;
- (9) Require an annual calibration of audiometric equipment;
- (10) Provide all examinations required by this part or contract with organizations to conduct examinations;
- (11) Establish minimum requirements of test procedure and test equipment to be used in the fitting of hearing instruments and also the retention of records of all fittings;
- (12) Inspect the facilities or records, or both, of persons who practice the fitting and selling of hearing instruments, upon a showing of probable cause of a violation of this part;
- (13) Require lists, set standards, and approve programs for trainees as provided in § 63-17-206;
- (14) Promulgate rules requiring continuing education as a prerequisite for renewal of licenses to practice under this part; and

(15) Employ a consultant to assist the board in its duties under this part.

63-17-203. Prohibited practice – Display of license.

(a) A person shall not engage in the sale of or practice of dispensing and fitting hearing instruments or display a sign or, in any other way, advertise or hold out to be a hearing instrument specialist unless direct hands-on testing and follow-up services are provided to persons to whom the hearing instrument specialist has dispensed or sold a hearing instrument, and such person holds a current, unsuspended, unrevoked license issued by the board as provided in this part.

(b) The license required by this section must be kept conspicuously posted in the specialist's principal office or place of business at all times.

63-17-204. Exemptions.

(a) This part does not prevent a person who is a physician licensed to practice by the board of medical examiners from treating or fitting hearing instruments to the human ear.

(b) It is lawful for a person, who meets the requirements of §§ 63-17-203 and 63-17-205 and who is listed as an apprentice licensee applicant with the board and who is under the direct supervision of a sponsoring licensed hearing instrument specialist, to engage in the practice of dispensing and fitting hearing instruments if the person does so under the direct supervision of the licensed hearing instrument specialist and with the licensed dispenser making final selection and fitting to the client. A licensed dispenser shall not supervise more than two (2) such apprentice licensee applicants or apprentice licensees combined at the same time.

(c) This part does not prevent a person who is an audiologist licensed by the board from practicing audiology, as defined in § 63-17-103, or prevent a person who is a speech-language pathologist licensed by the board from practicing speech-language pathology, as defined in § 63-17-103.

63-17-205. Applicant requirements.

(a) An applicant for a license must:

(1) Be at least eighteen (18) years of age; and

(2) Have an education equivalent to two (2) years of accredited college level coursework or national board for certification in hearing instrument sciences (NBC-HIS) board certification.

(b) Such applicant shall pay a nonrefundable application fee as set by the board no later than forty-five (45) days in advance of the next scheduled examination.

63-17-206. Apprentice license.

(a) Applicants who meet the requirements of § 63-17-205 with respect to age and education may apply for an apprentice license.

(b) The apprentice licensee functions under the direct supervision of the sponsoring licensed hearing instrument specialist for a period of at least three (3) months after submitting an application, and during such three-month period, the sponsoring licensed dispenser shall make the final selection and fitting of the hearing instrument.

(c) During the apprentice period, the apprentice licensee shall complete sixty (60) classroom hours of prescribed coursework.

(d) The apprentice license remains in effect no longer than one (1) year and cannot be renewed.

(e) Applicants for an apprentice license shall pay the board a nonrefundable fee as set by the board by rule.

63-17-207. Examination.

(a) Upon the payment of an examination fee as set by the board, an applicant for a license who is notified by the board that the applicant has fulfilled the requirements of § 63-17-205 shall appear at a time and place, and before such persons, as the board may designate to be examined by written and practical tests in order to demonstrate that the applicant is qualified to practice the fitting of hearing instruments.

(b) The board shall give at least one (1) examination of the type prescribed in this section each year and such additional examinations as the volume of applications may make appropriate.

63-17-208. Examination requirements.

(a) The examination described in § 63-17-207 must consist of:

(1) Tests of knowledge in the following areas as they pertain to the fitting of hearing instruments:

(A) Basic physics of sound;

(B) The human hearing mechanism, including the science of hearing and the causes and rehabilitation of abnormal hearing and hearing disorders; and

(C) Structure and function of hearing instruments;

(2) Tests of proficiency in the following techniques as they pertain to the fitting of hearing instruments:

(A) Pure tone audiometry, including air conduction testing and bone conduction testing;

(B) Live voice or recorded voice speech audiometry, including speech reception threshold testing and speech discrimination testing;

(C) Effective masking;

(D) Recording and evaluation of audiograms and speech audiometry to determine hearing instrument candidacy;

(E) Selection and adaption of hearing instruments and testing of hearing instruments;

(F) Taking earmold impressions; and

(G) Other skills as may be required for the fitting of hearing instruments; and

(3) Tests of knowledge of this part.

(b) The tests under this section do not include questions requiring a medical or surgical education.

63-17-209. License fee – License without examination.

(a) Upon payment of an initial license fee as determined by the board, the board shall issue a license to each applicant who passes the examination. An initial license expires as provided by rules promulgated by the board.

(b) The board may, in its discretion, license as a hearing instrument specialist, without examination, on the payment of a fee as set by the board, an applicant who is a hearing instrument specialist, certified, registered, or licensed under the laws of another state or country, if the applicant's qualifications for certification, registration, or licensure meet the licensure requirements in force in this state upon the date of the specialist's application for certification, registration, or licensure in this state.

63-17-210. Notification by licensee of changes.

Each licensee shall notify the board of a change of address of the licensee's place of business within thirty (30) days of such change. Failure to give notice is deemed just cause for disciplinary action by the board.

63-17-211. Registry of licensees.

The board shall issue all licenses approved by the board and maintain a registry of all license holders. It is the duty of the board to review and approve the qualifications of applicants for certification or renewal as hearing instrument specialists.

63-17-212. Fees – Renewals.

(a)

(1) Each licensed hearing instrument specialist shall pay to the board a biennial renewal fee as set by the board, payable in advance, for the ensuing two (2) years.

(2) The board shall notify each licensee that such renewal is due. The failure of the licensee to receive such notice does not excuse the licensee's obligation to renew every two (2) years.

(b) When a licensed hearing instrument specialist fails to pay the license fee within sixty (60) days after it becomes due, as provided in this section, the license of the person is automatically revoked at the expiration of the sixty (60) days after the renewal fee was required, without further notice or hearing.

(c) A person whose license is automatically revoked as provided in § 63-1-107(c) may make application in writing to the council for the reinstatement of such license, and, upon good cause being shown, the council, in its discretion, may reinstate such license upon payment of all past due renewal fees and upon further payment of a sum set by the council.

(d) A person licensed to practice by this part who has retired or may retire from such practice in this state on or after the effective date of this act is not required to register as provided by this part if the person files with the board an affidavit on a form to be furnished by the board, which affidavit states the date on which the person retired from such practice and such other facts as tend to verify the retirement as the board deems necessary. If the person thereafter reengages in such practice in this state, then the person must apply for registration with the board as provided by this part and meet other requirements as may be set by the board.

(e)

(1) Notwithstanding this part to the contrary, the division, with the approval of the commissioner, shall establish a system of license renewals at alternative intervals that allow for the distribution of the license workload as uniformly as is practicable throughout the calendar year. Licenses issued under the alternative method are valid for twenty-four (24) months and expire on the last day of the last month of the license period. However, during a transition

period, or at any time thereafter when the board determines that the volume of work for any given interval is unduly burdensome or costly, either the licenses or renewals, or both of them, may be issued for terms of not less than six (6) months nor more than eighteen (18) months. The fee imposed for a license under the alternative interval method for a period of other than twenty-four (24) months must be proportionate to the annual fee and modified in no other manner, except that the proportional fee must be rounded off to the nearest twenty-five cents (25¢).

(2) A renewal application must not be accepted after the last day of the month following the license expiration date under the alternative method authorized in this subsection (e).

63-17-213. Fees relative to the practice of dispensing hearing instruments – Use of funds.

The board shall set the fees relating to the practice of dispensing hearing instruments in an amount sufficient to pay all of the expenses of the board that are directly attributable to the performance of its duties pursuant to this chapter. Expenditures of the board for investigations and disciplinary actions against licensees of this part are the financial responsibility of the licensees who are regulated by this chapter. The division may maintain the existing separate accounts established for the board and former council for licensing hearing instrument specialists if the division combines the revenue collected such that the two (2) accounts may offset expenditures for the board. Funds remaining in these accounts at the end of any fiscal year do not revert to the general fund but remain available for expenditure in accordance with law.

63-17-214. Sales of hearing instruments.

(a) A person who engages in the practice of dispensing and fitting hearing instruments shall deliver to each person supplied with a hearing instrument, by the

dispensing person's or at the dispensing person's order or direction, a bill of sale that contains each of the following:

(1) The signature of the licensed hearing instrument specialist, the address of the specialist's regular place of business, and the specialist's license number and license expiration date;

(2) The make and model of the hearing instrument supplied, and the amount charged for the hearing instrument and whether the hearing instrument is new, used, or rebuilt;

(3) A clear statement of the terms of sale, including the provisions required by § 63-17-215; and

(4) The name and address of the board and a statement that the board will receive complaints on any matter relating to the fitting and dispensing of hearing instruments.

(b) The sale of a hearing instrument is subject to the same conditions and provisions as are prescribed by the United States food and drug administration and the federal trade commission.

(c) A hearing aid that is sold, fitted, or dispensed to a consumer in this state must be clearly and permanently marked with:

(1) The name of the manufacturer or distributor or the model's name or number;

(2) The serial number; and

(3) The year of manufacture.

63-17-215. Returns.

(a) Within thirty (30) days of the date of delivery, a purchaser of a hearing instrument from a licensee is entitled to return the hearing instrument for any reason if such instrument is returned in satisfactory condition and such purchaser pays only

reasonable charges for the hearing instrument and related services. Such return privileges apply only to a first-time purchaser of a hearing instrument.

(b) Charges to be imposed upon return of a hearing instrument as provided in subsection (a) must be clearly stated in the bill of sale.

(c) This section does not supersede any duly promulgated regulation issued by the federal trade commission.

63-17-216. Sponsor liability.

A person sponsoring an apprentice licensee or apprentice licensee applicant under this part is held responsible for the acts or omissions of the sponsor's apprentice licensee or apprentice licensee applicant within the course and scope of the licensee's or applicant's employment, during the apprenticeship, as well as the sponsor's employees.

63-17-217. License denial, revocation, or suspension.

(a) Subject to the due process requirements of the Uniform Administrative Procedures Act, compiled in title 4, chapter 5, a person registered under this part may have such license denied, revoked, or suspended for a fixed period to be determined by the board for the following causes:

(1) Having a conviction of an offense involving moral turpitude. The record of such conviction or certified copy thereof from the clerk of the court where the conviction occurred or by the judge of such court is sufficient evidence to warrant revocation or suspension;

(2) Securing a license under this part through fraud or deceit;

(3) Engaging in unethical conduct or gross or repeated acts of ignorance or inefficiency in the conduct of the person's practice;

(4) Knowingly practicing while suffering with a contagious or infectious disease;

(5) Using a false name or alias in the practice of the profession; and

(6) Violating this part or the board's rules or orders.

(b)

(1) The board shall promulgate rules governing the assessment of costs against a licensee or other person found by the board to have violated this chapter. The costs assessed by the board may include only those costs directly related to the prosecution of the charges against the licensee or other person, including investigatory costs where appropriate. The board shall determine the appropriate amount of costs, if any, to be assessed in a contested case. These costs must be reasonable and proportionate in light of the violation committed by the licensee or other person.

(2)

(A) An elected officer of the board or a duly appointed or elected chair has the authority to administer oaths to witnesses. Upon probable cause being established, the board, by a vote of two-thirds (2/3) of the total membership of the board, may issue subpoenas for the attendance of witnesses and the production of documents and records.

(B) Service of a subpoena issued by the board must be made by the sheriff of the county of residence of the licensee or person upon whom the subpoena is served.

(C)

(i) A licensee or person served by subpoena has thirty (30) days to request in writing a hearing before the board for the sole purpose of making a special appearance to quash or modify the subpoena. The subpoena for attendance of the person or the production of documents and records is stayed until the board votes upon the request to quash or modify the subpoena. A majority vote of the total membership of the board is required to quash or modify a subpoena.

(ii) A motion to appeal from a decision by the board regarding a request to quash or modify a subpoena must be made to the chancery court in Davidson County within fifteen (15) days of such decision.

(D) If a witness fails or refuses to obey a subpoena issued by it, the board may make application to any court of record in this state within the jurisdiction of which the witness is found or resides, and the court has power to attach the body of the witness and compel the witness to appear before the board and give testimony or produce books, records, or papers as ordered, and any failure to obey the court order may be punished by the court issuing the order as a civil contempt.

(E) Each witness who appears before the board by order of the board must receive for attendance the compensation provided by law for attendance of witnesses in a court of record, which must be paid from the funds of the board in the same manner as all other expenses of the board are paid.

63-17-219. Violations – Injunctions.

(a) A violation of this part is a Class B misdemeanor.

(b) The board may seek to enforce this part by petitioning a court of appropriate jurisdiction for an injunction to enjoin continuing violations of this part or by any other appropriate proceeding.

63-17-220. Persons licensed or in training up to July 1, 1995.

(a) A person who was a trainee or apprentice, or who had entered into a training program to become a licensed hearing aid dispenser, by July 1, 1995, is eligible to receive a license as a hearing instrument specialist if the person complies with the requirements of [former] chapter 15 of this title [repealed], as it existed on July 1, 1995.

(b) A person who was a licensed hearing aid dispenser on July 1, 1995, is eligible to receive a license as a hearing instrument specialist. A person who was a licensed hearing aid dispenser on July 1, 1995, who wishes to obtain a license as a hearing instrument specialist shall submit a request to the board to receive the appropriate license as a hearing instrument specialist. An additional fee shall not be imposed upon persons for the issuance of a license as a hearing instrument specialist pursuant to this subsection (b).

63-17-221. Cerumen management principles.

A licensed hearing instrument specialist shall comply with the following cerumen management principles:

(1) The indications for cerumen management for a licensed hearing instrument specialist include:

- (A) Enabling audiometric testing;
- (B) Making ear impressions;
- (C) Fitting hearing protection or prosthetic devices; and
- (D) Monitoring continuous use of hearing aids;

(2) The licensed hearing instrument specialist shall refer a patient who exhibits any of the following contraindications to cerumen removal for medical consultation or medical intervention to an otolaryngologist or a licensed physician:

- (A) An age less than twelve (12) years of age;
- (B) A perforated tympanic membrane;
- (C) History of pain, active drainage, or bleeding from the ear;
- (D) Evidence of congenital or traumatic deformity of the ear;
- (E) Ear surgery within the last six (6) months;
- (F) Tympanostomy tubes, such that irrigation should not be used;
- (G) A bleeding disorder;

- (H) Actual or suspected foreign body in the ear;
- (I) Stenosis or bony exostosis of the ear canal;
- (J) Cerumen impaction that totally occludes the ear canal;
- (K) Cerumen located medial to the cartilaginous external auditory canal; or

- (L) A tympanic membrane that the licensee is unable to see;

(3) In performing cerumen removal, a licensed hearing instrument specialist shall only remove cerumen lateral to the external auditory canal using the following instruments:

- (A) Cerumen loop;

- (B) Cerumenolytic liquid;

- (C) Irrigation, for patients with intact tympanic membranes and a closed mastoid cavity, no tympanostomy tubes, no recent ear surgery, and no recent dizziness; or

- (D) Suction used lateral to the bony canal, only for patients with no recent surgery, intact tympanic membranes, and no clear otorrhea;

(4) If the patient, while undergoing cerumen management that did not present contraindications, complains of significant pain, exhibits uncontrolled bleeding or a laceration of the external auditory canal, or notices the acute onset of dizziness or vertigo or sudden hearing loss, then the licensed hearing instrument specialist must immediately stop the procedure and refer the patient to an otolaryngologist or a licensed physician;

(5) The licensed hearing instrument specialist shall maintain the following proper infection control practices:

- (A) Universal health precautions;

- (B) Decontamination;

(C) Cleaning, disinfection, and sterilization of multiple use equipment; and

(D) Universal precautions for prevention of the transmission of human immunodeficiency virus (HIV), hepatitis B virus, and other bloodborne pathogens, as defined by occupational safety and health standards promulgated pursuant to 29 CFR 1910;

(6) The licensed hearing instrument specialist who performs cerumen management shall maintain a case history for every patient and informed consent signed by the patient as part of the patient's records;

(7) The licensed hearing instrument specialist shall carry appropriate professional liability insurance before performing cerumen removal; and

(8) The licensed hearing instrument specialist shall not require patients to sign any form that eliminates liability if the patient is harmed.

63-17-222. Cerumen management course required – Rules.

(a) A licensed hearing instrument specialist who engages in cerumen management, pursuant to § 63-17-221, must have completed a cerumen management course approved by the International Hearing Society, the American Academy of Otolaryngology-Head and Neck Surgery, or another organization approved by the board.

The course must:

(1) Be overseen by a physician, preferably an otolaryngologist;

(2) Consist of at least six (6) hours of a participant practicing removing cerumen from an ear canal model using a variety of safe techniques; and

(3) Result in a certificate of completion and attestation of competence signed by the overseeing physician.

(b) The board may promulgate rules to effectuate the requirements of the course outlined in this section only after consultation with the board of medical examiners,

established by § 63-6-101. The rules must be promulgated in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

63-17-223. Transfer of rules, licensing and regulatory authority, to the board of communications disorders and sciences.

(a) The secretary of state shall transfer all rules promulgated by the [former] council for licensing hearing instrument specialists [repealed] and in effect on the effective date of this act to the board of communications disorders and sciences.

(b) Pursuant to the transfer of rules under subsection (a), the board of communications disorders and sciences is authorized to regulate hearing instrument specialists, including applying and enforcing the rules promulgated by the [former] council for licensing hearing instrument specialists, upon the effective date of this act.

SECTION 59. Tennessee Code Annotated, Title 63, Chapter 18, is amended by deleting the chapter.

SECTION 60. Tennessee Code Annotated, Section 63-19-106(d)(1), is amended by deleting "§ 63-3-101" and substituting "§ 63-33-1401".

SECTION 61. Tennessee Code Annotated, Section 63-19-108, is amended by deleting "§ 63-3-119" and substituting "§ 63-33-1409", and deleting "§ 63-3-119(a)" and substituting "§ 63-33-1409(a)".

SECTION 62. Tennessee Code Annotated, Section 63-19-203(c)(1), is amended by deleting "63-3-101" and substituting "63-33-1401".

SECTION 63. Tennessee Code Annotated, Title 63, Chapter 22, is amended by deleting parts 1, 2, and 3 and renumbering part 4 as part 1.

SECTION 64. Tennessee Code Annotated, Title 63, Chapter 24, is amended by deleting the chapter.

SECTION 65. Tennessee Code Annotated, Title 63, Chapter 25, is amended by deleting part 1 and renumbering part 2 as part 1.

SECTION 66. Tennessee Code Annotated, Title 63, is amended by deleting chapters 26, 27, 28, 30, and 31.

SECTION 67. Tennessee Code Annotated, Section 63-32-102(b), is amended by deleting the subsection and substituting:

(b) As used in this chapter, "provider" or "healthcare provider" means:

- (1) A physician, regulated pursuant to chapter 6 of this title;
- (2) An osteopathic physician, regulated pursuant to chapter 9 of this title;
- (3) A chiropractor, regulated pursuant to chapter 4 of this title;
- (4) A dentist, regulated pursuant to chapter 5 of this title;
- (5) A podiatrist, regulated pursuant to chapter 33, part 14 of this title;
- (6) An optometrist, regulated pursuant to chapter 8 of this title;
- (7) A dietitian or nutritionist, regulated pursuant to chapter 33, part 7 of this title;
- (8) A physician assistant, regulated pursuant to chapter 19 of this title;
- (9) A respiratory care practitioner, regulated pursuant to chapter 33, part 9 of this title;
- (10) A pharmacist, regulated pursuant to chapter 10 of this title;
- (11) An audiologist and speech pathology therapist, regulated pursuant to chapter 17 of this title;
- (12) A certified nurse practitioner, as such nurses are regulated pursuant to § 63-7-123;
- (13) A registered nurse anesthetist, regulated pursuant to chapter 7 of this title;
- (14) A social worker, regulated pursuant to chapter 23 of this title;
- (15) A psychologist, regulated pursuant to chapter 11 of this title;

(16) A professional counselor, marital and family therapist, and clinical pastoral therapist, regulated pursuant to chapter 33, part 18 and part 19 of this title;

(17) A massage therapist, regulated pursuant to chapter 33, part 2 of this title;

(18) Medical laboratory personnel, regulated pursuant to title 68, chapter 29;

(19) Alcohol and drug abuse counselors, regulated pursuant to chapter 11, part 4 of this title;

(20) An occupational therapist and physical therapist, regulated pursuant to chapter 33, part 17 of this title;

(21) A veterinarian, regulated by chapter 12 of this title; and

(22) A nursing home administrator, regulated pursuant to chapter 33, part 13 of this title.

SECTION 68. Tennessee Code Annotated, Title 63, is amended by creating a new chapter 33, entitled "Commissioner Regulated Professions".

SECTION 69. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-101. Chapter definitions.

As used in this chapter:

(1) "Commissioner" means the commissioner of health or a person to whom the commissioner has delegated authority granted to the commissioner in accordance with § 63-33-108;

(2) "Department" means the department of health;

(3) "Division" means the division of health related boards in the department of health, as created by § 68-1-101;

(4) "License" means a certificate, registration, or license created in this chapter; and

(5) "Licensee" means a person granted a certificate, registration, or license pursuant to this chapter.

63-33-102. Creation – Commissioner's authority and duties – Rules.

(a) There is created an agency of commissioner issued licenses.

(b) The commissioner is responsible for all administrative, fiscal, and clerical functions related to this chapter, which is accomplished through the division. All documents and fees applicants or licensees are required to submit to the commissioner by this chapter must be submitted to the division.

(c) For each of the licenses created by this chapter, the commissioner shall:

(1) Promulgate rules for the implementation of each part, including rules to establish:

(A) Requirements and procedures for the issuance of licenses and the renewal of licenses under each part;

(B) Standards of practice, ethics, advertising, and professional responsibility to govern the conduct of persons licensed under each part;

(C) Procedures for investigating reports of professional misconduct and unlawful acts by persons licensed under each part; and

(D) Criteria and procedures for certification by endorsement and reciprocity, waiver of examinations, and temporary certificates;

(2) Establish reasonable and necessary fees, including, but not limited to, licensure and licensure renewal that, in the aggregate, cover the cost of administering this chapter;

(3) Promulgate rules necessary for obtaining criminal background information from applicants prior to the issuance of a license; and

(4) Issue all licenses approved by the commissioner and maintain a list of persons holding licenses under each part, to be provided to any interested party upon written request.

(d) The commissioner shall also:

(1) Receive complaints and review issues related to complaints and disciplinary actions involving each person licensed under this chapter;

(2) Conduct investigations, as appropriate, of persons licensed pursuant to this chapter;

(3) Review the qualifications of licensure applicants and issue licenses to persons who meet the requirements for licensure under each part;

(4) Promulgate rules and criteria for licensee continuing education and for the renewal of licenses;

(5) Issue private letter rulings in accordance with § 63-1-174;

(6) Promulgate rules and perform any other action required to appropriately ensure the protection of the health, safety, and welfare of the public as related to a license created by this chapter;

(7) Collect or receive all fees, fines, and moneys owed pursuant to this chapter; and

(8) Utilize screening panels in the investigative and disciplinary process in the manner as follows:

(A) The screening panel must be made up of between one (1) and three (3) commissioner-appointed consultants who have authority to administer oaths to witnesses and deliberative privilege, and have the same immunity as provided by law for board members;

(B) The members of the screening panel are not subject to deposition or subpoena to testify regarding any matter or issue raised in a

contested case, criminal prosecution, or civil lawsuit that may result from or be incident to the screening panel matters processed before them;

(C) Documents or records produced at the screening panel are exempt from title 10, chapter 7, until there is a filing of a notice of charges and such documents or records form the basis for such filing of a notice of charges;

(D) The screening panel must be instructed as to the statutes and rules pertaining to the disciplinary action and the procedures to be followed by the panel, and provided Rule 31 of the Tennessee Supreme Court for guidance on the principles of alternative dispute resolution;

(E) The commissioner does not have the authority to compel a party to participate in a screening panel, and prejudice is not incurred if a party chooses not to participate or accept the offer of the screening panel; and

(F) The commissioner may promulgate rules regarding the screening panel process.

(e) The rules promulgated by the commissioner for the implementation of each part must comply with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

(f) All moneys owed pursuant to this chapter, including the fees required by applicants or licensees and the costs, fines, and penalties, assessed for a violation of this chapter must be collected by the division. The division may maintain the existing separate accounts established for the boards and registries that formerly licensed the professions now licensed by the agency created by this chapter; provided, the division combines the revenue collected such that the division may use the revenue of each account to offset expenditures across the agency created by this chapter. Funds

remaining in these accounts at the end of any fiscal year do not revert to the general fund but remain available for expenditure in accordance with law.

(g) The commissioner shall use the department-employed investigators, inspectors, and agents, or use any other means necessary to bring about and maintain the rigid administration and enforcement of this chapter and all laws regulating the practice of the healings arts licensed by the commissioner.

63-33-103. License application.

(a) Applicants for a license created by this chapter shall submit an application on the form created by the commissioner and any required fees to the division.

(b) In addition to the specific requirements for licensure of each part of this chapter, the application must include evidence that the applicant meets the following requirements:

(1) The applicant has attained the age of majority;

(2) The applicant is of good moral character and conducts the applicant's professional activities in accordance with accepted professional and ethical standards;

(3) The applicant has not had a professional credential refused, revoked, suspended, or restricted and does not have a complaint, allegation, or investigation pending in any regulatory jurisdiction in the United States or in another country for unprofessional conduct, and has not committed an act that would constitute grounds for discipline of a license under § 63-33-106; and

(4) The applicant is either a citizen of the United States or is lawfully present in the United States in accordance with § 4-58-103(a)(1).

(c) The division and applicant shall comply with the criminal background check requirements of § 63-1-116.

(d) All licensees shall comply with the requirements of § 63-1-109.

(e) Portions of an application involving an applicant's health condition, including mental health conditions and substance use disorders, and records involving an applicant's health condition, are confidential, privileged, and not public records subject to inspection by citizens of this state, as described in § 10-7-503.

(f)

(1) When the division receives a completed application for initial licensure from an applicant, the division shall, within sixty (60) days from the date the division receives the completed application:

(A) Render a decision on the application; or

(B) Inform the applicant of the need to appear before the commissioner or the commissioner's designee.

(2) As used in this subsection (f), "completed application" means an application that satisfies all statutory and board rule requirements.

63-33-104. Reciprocity.

(a) The commissioner may, at the commissioner's discretion, grant a license created by this chapter to a person residing or employed in this state who is certified or licensed by a similar entity in another state whose standards, as determined by the commissioner, are not less stringent than the standards required by the part creating the license.

(b) The commissioner may enter into a reciprocal agreement with another state or jurisdiction that certifies or licenses persons engaged in the practice of a profession licensed under this chapter if the commissioner determines that the other state has substantially similar or more stringent certification or licensure requirements than this state.

63-33-105. License renewal.

(a) A license issued under this chapter is valid for twenty-four (24) months from the date of issuance and is renewable biennially at the end of the birth month of the licensee.

(b) In addition to any requirements of this chapter creating the license, a licensee requesting renewal of a certificate shall provide the following to the division in advance of the expiration date of the license:

(1) A completed application on a form provided by the division and payment of the fees set by the commissioner; and

(2) A certified or sworn statement that the licensee:

(A) Has completed all the continuing education hours required for the licensee, whether required by statute or by rule, prior to the expiration date of the license; and

(B) Has not had a professional credential revoked, suspended, or restricted by a certification board, and does not have a complaint, allegation, or investigation pending in this state or in any regulatory jurisdiction for unethical or unprofessional conduct.

(c) If a licensee fails to submit an application for license renewal, to pay any fee required by the commissioner, or to submit a certified or sworn statement as set forth in subdivision (b)(2) prior to the expiration date of the license, then the license is automatically revoked by the commissioner without further notice or hearing, unless renewal is completed and all fees are paid prior to the expiration of sixty (60) days from the date of the expiration of the license.

(d) A person whose license is automatically revoked under this section may apply in writing to the division for reinstatement of the license. Upon the showing of good cause by the person and submitting documentation that all continuing education requirements have been satisfied, the commissioner may reinstate the license upon the payment of all fees set by the commissioner.

(e) Applicants and licensees must notify the division of the following within thirty (30) days of their occurrence:

- (1) A change of name, address, or other vital information;
- (2) The filing of any criminal charge or civil action against the applicant or licensee;
- (3) The initiation of any disciplinary charges, investigations, findings, or sanctions by a healthcare organization, federal or state agency, or other professional association against the applicant or licensee; and
- (4) Any other change in information provided by the applicant or licensee to the division.

(f) For the purpose of effecting service of process upon a licensee, the commissioner or the division may notify the licensee by certified mail, return receipt requested, at the address on file with the division.

(g) A person licensed or certified to practice by this chapter who has retired or may retire from such practice on or after the effective date of this act is not required to register as is required by this chapter if the person files with the division an affidavit on a form to be furnished by the division, stating the date on which the person retired from such practice and such other facts as tend to verify the retirement as the division deems necessary. If the person thereafter reengages in such practice in this state, then the person shall apply for licensure with the division as provided by this chapter and meet other requirements as may be set by the commissioner.

63-33-106. License denial, suspension, or revocation – Grounds – Examinations – Hearings.

- (a) The commissioner has the power to:
- (1) Deny an application for a license to any applicant who applies for the same through reciprocity or otherwise;
 - (2) Permanently or temporarily withhold issuance of a license;

(3) Issue a license contingent upon such circumstances or requirements as the commissioner may determine;

(4) Issue a conditional license that is limited or restricted for such time and in such manner as the commissioner may determine, including a limited license of temporary duration for applicants out of practice for an extended period of time;

(5) Issue discipline to an applicant or licensee, or on a previously issued license, including, but not limited to, any combination of, private censures and warnings, public reprimands, limits, restrictions, conditions, probation periods, suspension periods, require participation in a peer assistance program, and issue civil penalties for such time and in such manner as the commissioner may deem proper in the commissioner's discretion;

(6) Discipline a licensee who has been disciplined by another state of the United States for any acts or omissions that would constitute grounds for discipline licensed in this state, following the procedure in § 63-1-120; or

(7) Revoke, or permanently revoke, a license.

(b) The grounds upon which the commissioner may exercise the powers enumerated in subsection (a) include, but are not limited to:

(1) Engaging in unprofessional, dishonorable, or unethical conduct;

(2) Obtaining or attempting to obtain a license by making a false or misleading statement, failing to make a required statement, or engaging in fraud or deceit in any communication to the department;

(3) Violating or attempting to violate, directly or indirectly, or assisting in or abetting the violation of, or conspiring to violate, a criminal statute of this state, this part, a rule promulgated by the commissioner, or a lawful order of the commissioner issued pursuant to rules;

(4) Engaging in gross professional liability, or a pattern of continued or repeated professional liability, ignorance, negligence, or incompetence, in professional work;

(5) Being habitually intoxicated or personally misusing drugs, or using intoxicating liquors, narcotics, controlled substances, controlled substance analogues, or other drugs or stimulants, in such manner as to adversely affect the person's ability to safely practice in the person's profession;

(6) Being convicted of a felony, any offense under state or federal laws relative to drugs or the professional practice, an offense involving moral turpitude, or any offense for which the licensee or applicant is required to register as a sexual offender or violent sexual offender pursuant to the Tennessee Sexual Offender and Violent Sexual Offender Registration, Verification and Tracking Act of 2004, compiled in title 40, chapter 39, part 2;

(7) Engaging in professional practice when mentally or physically unable to do so safely; and

(8) Engaging in any other action deemed to be grounds for disciplinary action under rules promulgated or enforced by the commissioner.

(c) All proceedings for disciplinary action against a licensee under this chapter must be conducted in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

(d) Assessment and collection of costs must follow the requirements and procedure contained in § 63-1-144.

(e) In enforcing this section, the commissioner shall, upon probable cause, have authority to compel an applicant or licensee to submit to a mental or physical examination, or both, by a designated committee of at least three (3) practicing physicians, including a psychiatrist where a question of mental condition is involved. The applicant or licensee may have an independent medical practitioner present during

such examination, and the applicant may have an independent physical or mental examination, which examination report must be filed with the commissioner for consideration. The committee shall submit a report of its findings to the commissioner, who must then hold a hearing governed by the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

(f) Notwithstanding § 63-1-122, for purposes of an action before the commissioner for a license created and regulated under this chapter, in which the standard of care is at issue, qualified expert testimony on the standard of care is required. The standard of care for such actions is a statewide standard of minimal competency and practice. The provisions of title 29, chapter 26, and specifically § 29-26-115(a), concerning the locality rule, do not apply to disciplinary actions involving licenses created under this chapter. However, expert testimony is not required to prove a violation of unprofessional, dishonorable, or unethical conduct.

(g) A person whose license or certificate has been revoked by the commissioner is not eligible to apply for reinstatement earlier than one (1) year from the date of revocation.

63-33-107. Unlicensed practice.

(a) A person who engages in the practice of a profession licensed by the commissioner without a current, valid license, and a person who violates this chapter, commits a Class B misdemeanor. Each such engagement in unlicensed practice constitutes a separate offense.

(b) The commissioner may assess civil penalties against a person engaged in the practice of a profession licensed by the commissioner without a current, valid license for each separate offense. The commissioner may also refer the case to law enforcement.

(c) The commissioner may petition any circuit or chancery court having jurisdiction of a person within this state who is practicing without a license, to whom a

license has been denied, or whose license has been suspended or revoked, to enjoin such person from continuing to practice the healing arts, or any branch thereof, within this state. Jurisdiction is conferred upon the circuit and chancery courts of this state to hear and determine all such causes as equity causes and exercise full and complete jurisdiction in such injunctive proceedings.

(d) This section does not confer criminal jurisdiction upon a court not now possessing such criminal jurisdiction, nor does any such court, as an incident to the injunctive proceedings authorized in this section, have the power to assess the criminal penalties set out in this chapter.

63-33-108. Delegation of powers.

(a) The commissioner may delegate any of the powers granted to the commissioner in this chapter to any employee of the department. The commissioner may also appoint consultant members of any health-licensed profession to work for a per diem or hourly rate to review and evaluate licensure applications, disciplinary cases, licensee inquiries, serve on state or national advisory boards or compact commissions, and otherwise assist division staff as necessary to carry out this chapter.

(b) When appointing consultants pursuant to this section, the commissioner shall consider recommendations made by interested professional associations related to the licensure type at issue for the consultant's review.

63-33-109. Fees.

(a) By rule, the commissioner shall establish fees for all the license types created under this chapter. These fees include, but are not limited to:

- (1) Initial licensure application fee;
- (2) Examination fee;
- (3) Biennial licensure renewal fee;
- (4) Late renewal fee;
- (5) Licensure reinstatement fee;

(6) Duplicate decorative license fee; and

(7) Reciprocal licensure fee.

(b) The commissioner shall ensure that the fees collected by the division for the agency of commissioner issued licenses are sufficient in an amount to pay all the costs associated with the administration, monitoring, and enforcement for all the licenses created under this chapter.

63-33-110. Emergency rules.

To effectuate this chapter, the commissioner may promulgate emergency rules pursuant to § 4-5-208(a)(5).

63-33-111. Validity of prior licenses, certificates, and registrations.

All licenses, certificates, and registrations regulated under chapters 3, 13, 16, 18, 22, 24, 25, 27, and 31, and valid as of June 30, 2026, must be transferred to the agency of commissioner issued licenses and remain in effect on their current renewal cycles under the authority of the commissioner pending the promulgation of any new rules on the issuance and renewal of such licenses, certificates, and registrations, as necessary.

63-33-112. Transfer of rules and regulatory authority to the commissioner.

(a) As of July 1, 2026, the secretary of state shall cause the following rules to be transferred to Tenn. Comp. R. & Reg. 1200-10:

(1) Tenn. Comp. R. & Regs. 0150-01;

(2) Tenn. Comp. R. & Regs. 0470-01;

(3) Tenn. Comp. R. & Regs. 0870-01 and 0870-02;

(4) Tenn. Comp. R. & Regs. 1330-01;

(5) Tenn. Comp. R. & Regs. 0880-14;

(6) Tenn. Comp. R. & Regs. 0880-12;

(7) All rules promulgated by the [former] professional art therapist advisory committee of the board of examiners in psychology [repealed];

(8) All rules promulgated by the [former] professional music therapy advisory committee of the board of examiners in psychology [repealed];

(9) Tenn. Comp. R. & Regs. 1020-01 and 1020-02;

(10) Tenn. Comp. R. & Regs. 1155-01, 1155-02, 1155-03, and 1155-04;

(11) Tenn. Comp. R. & Regs. 1150-01 and 1150-02; and

(12) Tenn. Comp. R. & Regs. 0450-01, 0450-02, and 0450-03.

(b) The rules listed in subsection (a) must remain in effect pending the promulgation of any new rules on the same subject in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

(c) Effective July 1, 2026, the commissioner has the authority to regulate the professions affected by the rules listed in subsection (a), including through use of those rules, until new rules on the same subject are promulgated.

SECTION 70. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-201. Part definitions.

As used in this part, unless the context otherwise requires:

(1) "Compensation" means the payment, loan, advance, donation, contribution, deposit, or gift of money or anything of value;

(2) "Massage," "bodywork," and "somatic" mean the manipulation of the soft tissues of the body with the intention of positively affecting the health and well-being of the client;

(3) "Massage establishment" means a place of business held out to the public wherein massage is practiced; and

(4) "Massage therapist" means a person who practices massage for compensation and is licensed by the commissioner.

63-33-202. Practice of massage – Practice of reflexology.

(a) Persons or massage establishments engaged in massage for compensation must be licensed by the commissioner.

(b) It is unlawful to use the word "massage," or any other term that implies massage technique or method, when advertising a service by a person who is not licensed under this part or another chapter of state law.

(c) The practice of reflexology is not subject to the licensure requirements of this chapter. As used in this subsection (c), "reflexology" means the application of specific pressures to reflex points in the hands and feet only.

63-33-203. Documentation required from applicants.

(a)

(1) Except as provided in subdivision (a)(2) and in addition to the requirements of § 63-33-103, each applicant for licensure as a massage therapist shall provide documentation in the form required by the commissioner that the applicant:

(A) Successfully completed the curriculum or curricula of one (1) or more postsecondary academic institutions for massage, bodywork, or somatic therapy as defined by board rules, totaling six hundred fifty (650) hours or more, such institutions being approved by the commissioner pursuant to § 63-33-209, and either authorized by the Tennessee higher education commission, or its equivalent in other states, or approved, or under the governance of, the board of regents; and

(B) Received a passing score on a competency examination approved by the board.

(2)

(A) A person who has completed a program of study as required by subdivision (a)(1)(A) in a postsecondary academic institution located in this state and received a diploma or certificate prior to September 1,

2005, must be issued a license without completing the examination requirement in subdivision (a)(1)(B).

(B) A person who meets the requirements of subdivision (a)(2)(A) has until January 1, 2006, to apply for such a license.

(C) Persons licensed under this subdivision (a)(2) are not considered to have national certification and not hold themselves out to be nationally certified.

(b) In addition to the requirements of § 63-33-103, each applicant for licensure as a massage establishment shall provide documentation in the form required by the commissioner that each person engaged in massage at the massage establishment has provided the documentation required in subsection (a) and has been licensed as a massage therapist by the commissioner.

(c) Notwithstanding this part, an establishment license is not required for the office of a physician licensed under chapter 4, 6, or 9 of this title if a massage for compensation is provided within that office by a licensed massage therapist.

63-33-204. Inspections.

In order to effectuate this part, the commissioner may conduct an investigation of persons engaged in massage and massage establishments and inspect the license of practitioners and establishments for compliance. The refusal of a practitioner or establishment to permit inspections is grounds for revocation, suspension, or refusal to issue a license pursuant to this part.

63-33-205. Exempted persons.

This part does not affect, prevent, or restrict:

(1) Persons qualified and licensed to practice other health professions from engaging in the practice of massage when incidental to the practice of their profession;

(2) Persons qualified and licensed to practice as barbers or cosmetologists from engaging in the practice of massage when incidental to the practice of their profession;

(3) Students enrolled in an institution described in § 63-33-203(a)(1) or a public school of this state; provided, that the student does not hold the student's self out as a licensed massage therapist and does not receive compensation for massage; and

(4) Massage therapists licensed in other states or countries when providing educational programs or services for a period of time not to exceed thirty (30) days within a calendar year.

63-33-206. Temporary license.

(a) The commissioner may, in the commissioner's discretion, grant a temporary license to a person who has been licensed, registered, or certified, in good standing, in another state, territory of the United States, or the District of Columbia, that meets all other requirements for reciprocity under this chapter and rules promulgated pursuant to this chapter, but which does not require successful passage of a competency examination approved by the commissioner.

(b) A person granted a temporary license pursuant to this section may work if the person is at all times working under the supervision of a person licensed pursuant to § 63-33-203.

(c)

(1) A temporary license issued pursuant to this section expires six (6) months after the date of issuance.

(2) A person granted a temporary license pursuant to this section may apply for one (1) extension of the person's temporary license for an additional six (6) months if:

(A)

(i) The person has been unable to take a competency exam approved by the commissioner within the initial temporary license period; and

(ii) Is registered to take an upcoming competency exam approved by the commissioner; or

(B) The person is still waiting for the results of the most recent competency exam approved by the commissioner.

63-33-207. Massage deemed medically therapeutic – Taxation.

Notwithstanding a law to the contrary, the act of a duly licensed massage therapist in performing a massage is deemed to be medically therapeutic in nature and not subject to the collection of any form of state or local taxation regulations not also imposed on other medically therapeutic activities.

63-33-208. Designation as "primary care provider" prohibited.

A person licensed by the commissioner under this part must not be referred to as a primary care provider or permitted to use such designation.

63-33-209. Powers and duties of commissioner – Passage rate of schools of massage therapy.

In addition to the powers and duties granted to or imposed by this chapter, the commissioner has the following powers and duties:

(1) To prescribe the minimum curricular and minimum standards for schools of massage therapy and for courses of training that prepare individuals for licensure under this part;

(2) To approve such schools and courses as meet the requirements of this part and the rules of the commissioner;

(3) To issue certificates of approval to such schools and courses that meet the requirements of this part and the rules of the commissioner;

(4) To conduct hearings for disciplinary action against schools or courses that fail to meet the minimum requirements of this part and the rules of the commissioner; and

(5) To annually publish passage rates for each school of massage therapy, based upon the results of each student's success in taking a national examination approved by the commissioner. Schools that fail to achieve an overall passing rate of seventy percent (70%) are required to submit a remedial plan to be approved by the commissioner.

63-33-210. Alternative licensure process.

(a) The commissioner may issue a license to practice massage therapy to an applicant who:

(1) Meets the qualifications set forth in § 63-33-103; and

(2) Has been certified by the National Certification Board for Therapeutic Massage and Bodywork for the five-year period immediately preceding application for licensure and can submit documentation satisfactory to the commissioner that the applicant has engaged in the practice of massage therapy in another state for the five-year period immediately preceding application for licensure, and who either:

(A) Has met the qualifications set forth in § 63-33-203(a), but is unable, because the educational institution either was not state approved or is no longer in existence, to produce a transcript to document compliance; or

(B) Graduated from a qualified massage school or course prior to October 1, 1995.

(b) The commissioner may promulgate rules that are necessary to effectuate this section.

SECTION 71. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-301. Part definitions.

As used in this part, unless the context otherwise requires:

(1) "Athletic injury" means an injury sustained by a person as a result of the person's participation in exercises, sports, games, or recreation requiring physical strength, agility, flexibility, range of motion, speed, or stamina, or comparable athletic injury that prevents the person from participating in such activities; and

(2) "Athletic trainer" means a person with specific qualifications as set forth in this part, who, upon the advice, consent, and oral or written prescriptions or referrals of a physician licensed under this title, carries out the practice of prevention, recognition, evaluation, management, disposition, treatment, or rehabilitation of athletic injuries, and, in carrying out these functions, the athletic trainer may use physical modalities, such as heat, light, sound, cold, electricity, or mechanical devices related to prevention, recognition, evaluation, management, disposition, rehabilitation, and treatment; an athletic trainer shall practice only in those areas in which such athletic trainer is competent by reason of training or experience that can be substantiated by records or other evidence found acceptable by the board in the exercise of the commissioner's considered discretion.

63-33-302. Prohibited representations as an athletic trainer.

(a) A person shall not represent the person's self as, or claim to be, an athletic trainer or perform, for compensation, any of the activities of an athletic trainer without first obtaining a license under this part.

(b) This part does not prevent a person from serving as a student-trainer, or a similar position, if such service is not primarily for compensation and is carried out under

the supervision of an athletic trainer duly licensed as provided in this part and a physician licensed under this title. Such supervision must be provided under the guidelines of the National Athletic Trainers' Association (NATA) Board of Certification, Inc., or approved by the commissioner.

63-33-303. Documentation required from applicants.

In addition to the requirements of § 63-33-103, an applicant for an athletic trainer license shall provide documentation in the form required by the commissioner that the applicant:

(1) Has met the athletic training curriculum requirements of a college or university approved by the commissioner; and

(2) Has satisfactorily completed all of the National Athletic Trainers' Association (NATA) Board of Certification, Inc., qualifications and is certified as an athletic trainer in good standing by the NATA Board of Certification, Inc., or approved by the commissioner.

63-33-304. Exempted persons.

(a) This part does not limit or prevent a person duly licensed or certified under the laws of this state from practicing the profession for which the person was licensed or certified.

(b) This part does not apply to a person engaging in the practice of athletic training while employed for such purpose by a postsecondary educational institution or professional athletic organization from another state that is temporarily present in this state for an athletic competition or exhibition.

(c) This part does not apply to physicians duly licensed under this title who employ or contract for services provided by persons in the physician's office assisting in the care of patients and rendered under the supervision, control, and responsibility of the licensed physician.

63-33-305. Civil penalties.

The commissioner has the authority to assess civil penalties commensurate with those assessable under § 63-1-134 against persons who have knowingly employed, contracted for, or otherwise utilized unlicensed persons in the practice of athletic training, with or without compensation.

63-33-306. Athletic trainers – Scope of practice.

(a) An athletic trainer may use dry needling, as defined in § 63-33-1602, and physical modalities, such as heat, light, sound, cold, electricity, or mechanical devices, for the prevention, recognition, evaluation, management, disposition, rehabilitation, and treatment of athletic injuries.

(b) An athletic trainer shall only practice in those areas in which an athletic trainer is competent by reason of training or experience that can be substantiated by records or other evidence found acceptable by the board in the exercise of the commissioner's considered discretion.

(c) An athletic trainer shall obtain such training and certification in the practice of dry needling as the commissioner may require by rule. The practice of dry needling by an athletic trainer under this chapter does not constitute the practice of acupuncture under part 4 of this chapter.

(d) The commissioner is authorized to promulgate rules to effectuate this section. The rules must be promulgated in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

SECTION 72. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-401. Part definitions.

As used in this part, unless the context otherwise requires:

(1) "ACAOM" means the Accreditation Commission for Acupuncture and Oriental Medicine;

(2) "Acupuncture" means a form of health care developed from traditional and modern oriental medical concepts that employs oriental medical diagnosis and treatment and adjunctive therapies and diagnostic techniques for the promotion, maintenance, and restoration of health and the prevention of disease.

"Adjunctive therapies" within the scope of "acupuncture" may include acupressure, cupping, thermal, and electrical treatment and the recommendation of dietary guidelines and supplements and therapeutic exercise based on traditional oriental medical concepts;

(3) "ADS" means an acupuncture detoxification specialist trained in, and who performs only, the five-point auricular detoxification treatment;

(4) "NADA" means the National Acupuncture Detoxification Association;

(5) "NCCAOM" means the National Certification Commission for Acupuncture and Oriental Medicine; and

(6) "Practice of acupuncture" means the insertion of acupuncture needles and the application of moxibustion to specific areas of the human body based on oriental medical diagnosis as a primary mode of therapy.

63-33-402. Exempted persons – Unlawful practice of acupuncture – Limited licensure.

(a) This part does not apply to:

(1) Physicians licensed under chapter 6 or chapter 9 of this title, and does not prevent the practice of acupuncture by such physicians or to prevent such physicians from using the title "acupuncturist";

(2) Registered nurses who are nationally certified as holistic nurses and who have successfully completed an accredited education program in acupuncture; or

(3) Chiropractic physicians licensed under chapter 4 of this title, and does not prevent the practice of acupuncture by chiropractic physicians who

have completed two hundred fifty (250) hours of an accredited acupuncture course and have passed the National Board of Chiropractic Examiners acupuncture exam.

(b) It is otherwise unlawful to practice acupuncture for compensation or gratuitously unless licensed under this part. This restriction does not apply to the following:

(1) Students practicing acupuncture under the supervision of a licensed acupuncturist as part of a course of study approved by the commissioner; or

(2) Individuals who do not otherwise possess the credentials required for the practice of acupuncture by this part, or rules promulgated under this part, by the commissioner are granted limited licensure as an ADS for the purpose of the treatment of alcoholism, substance abuse, or chemical dependency if they meet the following conditions:

(A) Provide documentation of successful completion of a commissioner-approved training program in auricular detoxification acupuncture that meets or exceeds standards of training set by NADA;

(B) Practice auricular detoxification treatment in a hospital, clinic, or treatment facility that provides comprehensive alcohol and substance abuse or chemical dependency services, including counseling, under the supervision of a licensed acupuncturist or medical director;

(C) Satisfy all appropriate ethical standards; and

(D) Limit their practice to the five-point auricular detoxification treatment.

63-33-403. Documentation required from applicants.

(a) In addition to the requirements of § 63-33-103, an applicant for licensure as an acupuncturist shall provide documentation in the form required by the commissioner that the applicant:

(1) Has successfully completed a three-year postsecondary training program or acupuncture college program that is ACAOM-accredited or in candidacy status or that meets ACAOM's standards;

(2) Has successfully completed an NCCAOM-approved clean needle technique course; and

(3) Either:

(A) Has current active status as a diplomate in acupuncture of the NCCAOM; or

(B) Has current state licensure in good standing by another state with substantially equivalent or higher standards.

(b) The commissioner shall issue a limited acupuncture license to an ADS who meets the requirements listed in § 63-33-402(b)(2).

63-33-404. License renewals.

In addition to the requirements of § 63-33-105, in order to renew a license, a person shall submit proof of current active NCCAOM certification in acupuncture or document compliance with § 63-33-403. To renew an ADS license, a person shall submit proof of current active practice in auricular detoxification treatment, as determined by the commissioner.

63-33-405. Health practices.

(a) All licensed individuals under this part shall use only presterilized, disposable needles in their administration of acupuncture treatments. The use of staples in the practice of acupuncture is prohibited.

(b) Health practices must include:

(1) Hands being washed with soap and water or other disinfectant before handling needles and between treatment of different patients;

(2) Skin in the area of penetration being thoroughly swabbed with alcohol or other germicidal solution before inserting needles; and

(3) Individuals passing a nationally recognized clean needle technique course before being allowed to practice acupuncture and related techniques.

63-33-406. Use of titles "licensed acupuncturist" or "ADS".

The titles "licensed acupuncturist" or "ADS" may be used by persons licensed under this part. A person who is not properly licensed to practice medicine or osteopathic medicine shall not use licensure under this part to identify the person as a doctor or physician.

SECTION 73. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-501. Practice of art therapy.

(a) The practice of art therapy in this state is declared to affect the public health, safety, and welfare of citizens of this state.

(b) The commissioner is authorized to protect the public from:

(1) The practice of art therapy by unqualified persons; and

(2) Unprofessional, unethical, and harmful conduct by art therapy practitioners.

(c) A person shall not represent to be or function as a professional art therapist in this state unless the person holds a valid license issued by the commissioner. The commissioner shall also regulate the practice of art therapy.

63-33-502. Part definitions.

As used in this part:

(1) "Appraisal activities":

(A) Means selecting, administering, scoring, and interpreting art therapy instruments that are used to understand, measure, or facilitate an individual's normal human growth and development; and

(B) Does not include the use of projective techniques in the assessment of personality, nor the use of psychological or clinical tests

designed to identify or classify abnormal or pathological human behavior, nor the use of individually administered intelligence tests;

(2) "Art therapy" means the integrated application of psychotherapeutic principles and methods with specialized training in visual art media, the neurobiological implications of art-making and the creative process, and art-based assessment models to assist individuals, families, or groups to improve mental functioning, increase self-awareness and self-esteem, resolve conflicts and distress, and enhance social functioning;

(3) "Art therapy certified supervisor" means a person credentialed by the Art Therapy Credentials Board, Inc., to provide supervised clinical experience for an associate art therapist;

(4) "Associate art therapist" means a person holding a master's or doctoral degree in art therapy while engaged in the practice of professional art therapy under supervision as part of clinical training to fulfill the supervised experience requirement for becoming a licensed professional art therapist;

(5) "Certified art therapist" means a person holding national board certification (ATR-BC) from the Art Therapy Credentials Board, Inc.;

(6) "Credentialed art therapist" means a person holding credentials in good standing with the Art Therapy Credentials Board, Inc., as a registered art therapist (ATR), certified art therapist (ATR-BC), or art therapy certified supervisor;

(7) "Licensed professional art therapist" means a person who is licensed to practice professional art therapy in this state;

(8) "Practice of professional art therapy":

(A) Means rendering or offering to render to individuals, groups, organizations, or the general public any service as an independent or

adjunctive therapist involving the application of art therapy principles, techniques, methods, or procedures; and

(B) Includes, but is not limited to, therapeutic interventions to facilitate alternative modes of receptive and expressive communication, development of treatment plans, appraisal activities, consulting and referral activities, and prevention and treatment of mental and emotional conditions; and

(9) "Supervision" means the regular observation and oversight of the functions and activities of a graduate art therapy student as part of an internship or practicum experience, or an associate art therapist engaged in the practice of art therapy to fulfill the requirements for licensure under § 63-33-503.

63-33-503. Documentation required from applicants.

(a) In addition to the requirements of § 63-33-103, an applicant for licensure under this part shall provide documentation in the form required by the commissioner that the applicant has:

(1) Passed the board certification examination of the Art Therapy Credentials Board, Inc.; and

(2) Either:

(A) Received a master's degree or doctoral degree in art therapy from a program approved by the American Art Therapy Association or accredited by the Commission on Accreditation of Allied Health Education Programs at the time the degree was conferred, and has completed no less than one thousand (1,000) client contact hours of experience after the master's degree was conferred under the supervision of an art therapy certified supervisor, licensed professional art therapist, credentialed art therapist, or other licensed mental health professional approved by the commissioner. At least one-half (1/2) of the required

supervision hours must be provided by an art therapy certified supervisor, licensed professional art therapist, or certified art therapist; or

(B) Received a master's degree or doctoral degree from a college or university that was accredited by a national or regional accrediting organization recognized by the Council for Higher Education Accreditation in a program with a graduate-level course of study and clinical training that, as determined by the commissioner, is equivalent to an approved or accredited program in art therapy, and has completed not less than one thousand five hundred (1,500) hours of client contact experience after the qualifying degree was conferred under the supervision of an art therapy certified supervisor, licensed professional art therapist, credentialed art therapist, or other licensed mental health professional approved by the commissioner. At least one-half (1/2) of the required supervision hours must be provided by an art therapy certified supervisor, licensed professional art therapist, or certified art therapist.

(b) A person holding a national credential in good standing as a certified art therapist is deemed to have met the requirements of this section upon submitting an application and fee and providing evidence satisfying the requirements of § 63-33-103 in the form required by the commissioner.

63-33-504. Use of title "licensed professional art therapist" or "licensed art therapist."

(a) A person shall not use the title "licensed professional art therapist" or "licensed art therapist," or use an abbreviation to indicate or imply that the person is a licensed professional art therapist in this state unless the person holds a license issued by the commissioner under this part. This subsection (a) does not prohibit an associate art therapist from using the title "associate art therapist" in accordance with this part.

(b) A person shall not practice professional art therapy unless the person is:

- (1) Licensed by the commissioner as a licensed professional art therapist;
- (2) An associate art therapist, as long as the person uses the title of "associate art therapist" and does not represent themselves to the public as a licensed professional art therapist; or
- (3) Exempt from licensure under § 63-33-506.

63-33-505. Authorized work settings.

(a) A licensed professional art therapist may practice independently for and with organizations that include, but are not limited to, addiction and rehabilitation centers, art studios and centers, behavioral health hospitals and outpatient settings, cancer treatment centers, community centers, community services boards, correctional facilities, disability services, eating disorder clinics, government agencies, medical hospitals and outpatient settings, military bases and veterans and family services, museums, nonprofits, nursing homes, physical rehabilitation, residential treatment centers, schools, senior living facilities, state programs, and wellness centers.

(b) Until the commissioner promulgates rules to establish the requirements for private outpatient independent practice by a licensed professional art therapist, a licensed professional art therapist shall only work in a private, outpatient independent setting if the patient is referred to the therapist by:

- (1) A physician licensed under chapter 6 or 9 of this title;
- (2) A psychologist licensed under chapter 11 of this title;
- (3) A professional counselor licensed under chapter 33, part 18 of this title;
- (4) A professional counselor designated as a mental health service provider under chapter 33, part 18 of this title;
- (5) A marital and family therapist licensed under chapter 33, part 18 of this title;

- (6) A pastoral counselor licensed under chapter 33, part 18 of this title;
 - (7) A licensed master's social worker licensed under chapter 23 of this title;
 - (8) A licensed clinical social worker licensed under chapter 23 of this title;
- or
- (9) Another advanced medical or mental health professional licensed under this title, as determined by the commissioner.

63-33-506. Authorized activities.

(a) This part does not:

- (1) Require licensure or restrict the activities of a student enrolled in a graduate program in art therapy approved by the commissioner while practicing art therapy under qualified supervision and as part of an approved course of study; or

- (2) Prohibit or restrict the practice of persons authorized to practice under this title who perform services consistent with the scope of practice of their profession and within the scope of their training and competence if the persons do not represent themselves to the public as a professional art therapist or as holding a license issued under this part.

(b) This part does not permit any method of treatment that involves the administration or prescription of drugs or the practice of medicine in this state or permits a person licensed as a professional art therapist to perform psychological testing.

(c) Subsection (b) does not prohibit a licensed professional art therapist from conducting appraisal activities. Consistent with each licensed professional art therapist's formal education and training, licensed professional art therapists may administer and utilize appropriate art therapy assessment instruments that measure or determine problems or dysfunctions within the context of human growth and development as part of the therapeutic process or in the development of a treatment plan.

SECTION 74. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-601. Practice of music therapy.

- (a) The practice of music therapy in this state is declared to affect the public health, safety, and welfare of citizens of this state.
- (b) The commissioner is authorized and given the duty to protect the public from:
 - (1) The practice of clinical music therapy by unqualified persons; and
 - (2) Unprofessional, unethical, and harmful conduct by music therapy practitioners.
- (c) A person shall not represent to be or function as a professional music therapist in this state unless the person holds a valid license issued by the commissioner. The commissioner shall also regulate the practice of music therapy.
- (d) This part does not restrict the non-clinical, non-therapeutic music intervention of the Tennessee person-centered music program administered by the Tennessee commission on aging and disability, created by § 52-8-102, and associated area agencies on aging and disability or their representatives.

63-33-602. Part definitions.

As used in this part:

- (1) "Certified professional music therapist" means a person certified to practice music therapy under this part;
- (2) "Music intervention" means the use of music in an individualized approach that includes music improvisation, receptive music listening, song writing, lyric discussion, music and imagery, singing, music performance, learning through music, music combined with other arts, music-assisted relaxation, music-based education, electronic music technology, adapted music intervention, and movement to music, that may be used in a clinical, therapeutic relationship or in a non-clinical, nontherapeutic manner; and

(3) "Music therapy":

(A) Means the clinical and evidence-based use of music interventions to accomplish individualized goals for people of all ages and ability levels within a therapeutic relationship by a board-certified music therapist; and

(B) Does not include the screening, diagnosis, or assessment of any physical, mental, or communication disorder.

63-33-603. Rules.

In addition to the authority granted by and requirements found in part 1 of this chapter, the rules promulgated by the commissioner for the implementation of this part must:

(1) Provide for the requirements for licensure as set forth in § 63-33-604; and

(2) Take into consideration the standards of ethics and professional conduct of the code of ethics of the American Music Therapy Association and code of professional ethics of the Certification Board for Music Therapists in effect on January 1, 2025, or any successor organization.

63-33-604. Documentation required from applicants.

In addition to the requirements of § 63-33-103, the applicant shall provide documentation in the form required by the commissioner that the applicant:

(1) Holds a bachelor's degree or higher in music therapy, or its equivalent, including clinical training hours from a music therapy program within an accredited college or university;

(2) Has passed the examination for board certification offered by the Certification Board for Music Therapists, or any successor organization, or provides proof of being transitioned into board certification; and

(3) Is currently a board-certified music therapist.

63-33-605. Use of title "certified professional music therapist" or "certified music therapist."

(a) A person shall not use the title "certified professional music therapist," or "certified music therapist," or use any abbreviation to indicate that the person is certified as a professional music therapist in this state unless the person holds a certificate issued by the commissioner under this part.

(b) Subsection (a) does not prohibit:

(1) A person who is licensed, certified, or regulated under the laws of this state in another profession or occupation, including physicians, psychologists, psychoanalysts, registered nurses, marriage and family therapists, social workers, occupational therapists, professional or rehabilitation counselors, speech-language pathologists or audiologists, or personnel supervised by a licensed professional from performing work, including the use of music, that is incidental to the practice of the person's licensed, certified, or regulated profession or occupation, if the person does not represent themselves as a music therapist;

(2) A person whose training and national certification attests to the person's preparation and ability to practice their certified profession or occupation, if the person does not represent themselves as a music therapist;

(3) A practice of music therapy as an integral part of a program of study for students enrolled in an accredited music therapy program, if the student does not represent themselves as a music therapist;

(4) A person who practices music therapy under the supervision of a certified professional music therapist, if the person does not represent themselves as a music therapist; or

(5) The use of music intervention in a non-clinical, non-therapeutic manner by persons who do not represent themselves as music therapists or

through a program that is not represented to be a music therapy program, including the Tennessee person-centered music program administered by the Tennessee commission on aging and disability, created by § 52-8-102.

63-33-606. Authorized work settings – Required reviews.

(a) A certified professional music therapist may practice independently for and with organizations that include, but are not limited to, addiction and rehabilitation centers, music studios and centers, behavioral health hospitals and outpatient settings, cancer treatment centers, community centers, community services boards, correctional facilities, disability services, eating disorder clinics, government agencies, medical hospitals and outpatient settings, military bases and veterans and family services, museums, nonprofits, nursing homes, physical rehabilitation, residential treatment centers, schools, senior living facilities, state programs, and wellness centers.

(b)

(1)

(A) Before a certified professional music therapist provides music therapy services to a client for an identified clinical or developmental need, the certified professional music therapist shall review the client's diagnosis, treatment needs, and treatment plan with the healthcare providers involved in the client's care.

(B) During the provision of music therapy services to a client, the certified professional music therapist shall collaborate, as applicable, with the client's treatment team, including the client's physician, psychologist, licensed clinical social worker, or other mental health professional. During the provision of music therapy services to a client with a communication disorder, the certified professional music therapist shall collaborate and discuss the music therapy treatment plan with the client's

audiologist or speech-language pathologist so that a music therapist may work with the client and address communication skills.

(2) Before a certified professional music therapist provides music therapy services to a student for an identified educational need in a special education setting, the certified professional music therapist shall review the student's diagnosis, treatment needs, and treatment plan with the individualized family service plan or individualized education program team.

(3) When providing educational or healthcare services, a certified professional music therapist shall not replace the services provided by an audiologist or a speech-language pathologist. Unless authorized to practice speech-language pathology, music therapists shall not evaluate, examine, or provide instruction or counsel on speech, language, communication, and swallowing disorders and conditions. A person certified as a professional music therapist shall not represent to the public that the person is authorized to treat a communication disorder. This subdivision (b)(3) does not prohibit a person, certified as a professional music therapist from representing to the public that the person is qualified to work with clients with communication disorders and to address communication skills.

63-33-607. Authorized activities.

In addition to any other authority granted under this part, certified professional music therapists may:

(1) Accept referrals for music therapy services from medical, developmental, mental health, or education professionals; family members; clients; caregivers; or others involved with and authorized to provide client services;

(2) Conduct a music therapy assessment of a client to determine if treatment is indicated. If treatment is indicated, then the certified professional

music therapist may collect systematic, comprehensive, and accurate information to determine the appropriateness and type of music therapy services to provide for the client;

(3) Develop an individualized music therapy treatment plan for the client that is based upon the results of the music therapy assessment. The music therapy treatment plan must include individualized goals and objectives that focus on the assessed needs and strengths of the client and specify the music therapy approaches and interventions to be used to address these goals and objectives;

(4) Implement an individualized music therapy treatment plan that is consistent with any other developmental, rehabilitative, habilitative, medical, mental health, preventive, wellness care, or educational services being provided to the client;

(5) Evaluate a client's response to music therapy and the music therapy treatment plan, documenting change and progress, and suggesting modifications, as appropriate;

(6) Develop a plan for determining when the provision of music therapy services is no longer needed in collaboration with the client, physician, or other provider of health care or education of the client, family members of the client, and any other appropriate person upon whom the client relies for support;

(7) Minimize barriers to ensure that the client receives music therapy services in the least restrictive environment;

(8) Collaborate with and educate the client and the family, caregiver of the client, or any other appropriate person regarding the needs of the client that are being addressed in music therapy and the manner in which the music therapy treatment addresses those needs; and

(9) Utilize their appropriate knowledge and skills to practice, which include the use of research, reasoning, and problem-solving skills in determining appropriate actions in the context of each clinical setting.

63-33-608. Psychological practice.

This part does not permit a certified professional music therapist to engage in a psychological practice as described in chapter 11, part 2 of this title.

SECTION 75. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-701. Part definitions.

As used in this part, unless the context otherwise requires:

(1) "Dietetics/nutrition practice":

(A) Means the integration and application of scientific principles of food, nutrition, biochemistry, physiology, management, and behavioral and social sciences in achieving and maintaining health through the life cycle and in the treatment of disease; and

(B) Includes nutritional assessment, development, implementation, and evaluation of nutrition care plans, nutritional counseling and education, and the development and administration of nutrition care standards and systems;

(2) "Dietitian" and "nutritionist" may be used interchangeably; and

(3) "Dietitian/nutritionist" means a healthcare professional engaging in dietetics/nutrition practice and licensed under this part.

63-33-702. Use of titles – Exempted persons.

(a) It is unlawful for any person to engage in the practice of dietetics/nutrition, to act or represent the person to be a dietitian or a nutritionist or to use such titles as "dietitian/nutritionist," "licensed dietitian," "licensed nutritionist," or such letters as "L.D.," "L.N.," or any facsimile thereof, or any word, letters, or title indicating or implying that

such person is a dietitian/nutritionist, unless the person holds a license and otherwise complies with this chapter and the rules adopted by the commissioner.

(b) This part does not affect, prevent, or restrict:

(1) Persons qualified and licensed to practice other health professions from engaging in the practice of dietetics/nutrition when incidental to the practice of their profession. This part does not apply to persons licensed to practice medicine or osteopathic medicine pursuant to chapters 6 and 9 of this title;

(2) Students enrolled in an approved academic program in dietetics, if such practice constitutes a part of a supervised course of study and such student is designated by title clearly indicating such student's status as a student or trainee;

(3) A dietitian serving in the armed forces or the public health service of the United States or employed by the veterans administration from engaging in the practice of dietetics if such practice is related to such service or employment;

(4) Persons performing the activities and services of a nutrition educator in the employment of a federal, state, county, or municipal agency or an elementary or secondary school or accredited degree-granting educational institution if such activities and services are part of a salaried position;

(5) Federal, state, county, or local employees involved with nutrition-related programs, including, but not limited to, the cooperative extension service, child nutrition programs, and Project Head Start from engaging in the practice of dietetics/nutrition within the discharge of their official duties if the person engaging in dietetic/nutrition practice outside the scope of such official duty is licensed as provided in this section;

(6) Persons employed in a hospital or nursing home as defined in § 68-11-201 and subject to licensure by the board for licensing health care facilities; or

(7)

(A) Persons who do not hold themselves out to be licensed or registered dietitians/nutritionists of the right to provide services and information related to nonmedical nutrition while:

- (1) Employed by or operating a health weight loss or fitness program;
- (2) Employed by or operating a health food store;
- (3) Employed by or operating a business that sells health products, including dietary supplements, food or food materials, or provides nonmedical nutritional information or distributes nutritional literature; or
- (4) Conducting classes or disseminating information related to nonmedical nutrition.

(B) As used in this subdivision (b)(7), "nonmedical" means engaging in a practice that does not constitute the practice of medicine, osteopathic medicine, chiropractic, dentistry, or podiatry.

63-33-703. Therapeutic dietary regimen.

A therapeutic dietary regimen must not be developed unless pursuant to the appropriate orders or referral of practitioners licensed as medical or osteopathic physicians, dentists, chiropractors, podiatrists, physician assistants, or advanced practice registered nurses, when incidental to the practice of their respective professions.

63-33-704. Documentation required from applicants.

In addition to the requirements of § 63-33-103, an applicant for licensure as a dietitian/nutritionist shall provide documentation in the form required by the commissioner that the applicant:

- (1) Has received a baccalaureate or post-baccalaureate degree from a regionally accredited college or university that provides a major course of study

in human nutrition, food and nutrition, dietetics, or food systems management, or an equivalent major course of study as approved by the commissioner;

(2) Has successfully completed a planned continuous preprofessional experience in nutrition practice of no less than nine hundred (900) hours under the supervision of a registered dietitian, or successfully completed a program of supervised clinical experience approved by the commission on dietetic registration of the American Dietetic Association; and

(3) Has passed the national examination prepared and administered by the commission on dietetic registration of the American Dietetic Association.

63-33-705. Temporary permits.

(a) The commissioner may, in the commissioner's discretion, issue temporary permits to engage in dietetics/nutrition practice to an applicant upon submission of satisfactory evidence that the person:

(A) Has successfully completed the academic and experiential requirements to be licensed as a dietitian/nutritionist;

(B) Has applied for or taken a licensing examination;

(C) Is awaiting notification of examination results; and

(D) Has paid the fee required for temporary permits.

(b) Temporary permits expire one (1) year from the date of issuance and may be renewed for a period not to exceed six (6) months upon submitting to the commissioner a satisfactory explanation of the applicant's failure to become licensed within the one-year term.

SECTION 76. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-801. Part definitions – Setting for practice of polysomnography.

(a) As used in this part, unless the context otherwise requires:

(1) "Direct supervision" means that the polysomnographic technologist providing supervision must be present in the area where the polysomnographic procedure is being performed and immediately available to furnish assistance and direction throughout the performance of the procedure;

(2) "General supervision" means that the polysomnographic procedure is provided under a physician's overall direction and control, but the physician's presence is not required during the performance of the procedure;

(3) "Polysomnographic student" means a person who is enrolled in an educational program that is accredited by the Commission on Accreditation of Allied Health Education Programs (CAAHEP), as provided in § 63-33-803(a)(1), and who may provide sleep-related services under the direct supervision of a polysomnographic technologist as a part of the person's educational program;

(4) "Polysomnographic technician" means a person who has graduated from an accredited educational program described in § 63-33-803(a)(1) but has not yet passed the national certifying examination given by the Board of Registered Polysomnographic Technologists, who has obtained a temporary permit from the commissioner, and who may provide sleep-related services under the general supervision of a licensed physician;

(5) "Polysomnographic technologist" means a person who is credentialed by the Board of Registered Polysomnographic Technologists and is licensed by the commissioner to engage in the practice of polysomnography under the general supervision of a licensed physician;

(6) "Polysomnographic trainee" means a person who is enrolled in an accredited sleep technologist education program (A-STEP) that is accredited by the American Academy of Sleep Medicine and who may provide sleep-related services under the direct supervision of a polysomnographic technologist as a part of the person's educational program;

(7) "Practice of polysomnography" means the staging and scoring of sleep by continuous and simultaneous monitoring of the stages of sleep and wake through use of an electroencephalogram (EEG), an electrooculogram (EOG), and a submental electromyogram (EMG), in conjunction with the recording and monitoring of other physiological variables, and the assignment of values for duration, frequency, and type of event to each stage of sleep in which the event occurred. The following tasks are considered a part of the practice of polysomnography only when performed as part of the staging and scoring of sleep under the general supervision of a licensed physician:

(A) Monitoring and recording physiologic data during the evaluation of sleep-related disorders, including sleep-related respiratory disturbances, by applying the following techniques, equipment, and procedures:

(i) Continuous or bilevel positive airway pressure titration on spontaneously breathing patients using a mask or oral appliance when the mask or oral appliance does not extend into the trachea or attach to an artificial airway;

(ii) Supplemental low flow oxygen therapy of less than six (6) liters per minute, utilizing nasal cannula or continuous or bilevel positive airway pressure during a polysomnogram;

(iii) Capnography during a polysomnogram;

(iv) Cardiopulmonary resuscitation;

(v) Pulse oximetry;

(vi) Gastroesophageal pH monitoring;

(vii) Esophageal pressure monitoring;

(viii) Sleep staging, including surface electroencephalography, surface electrooculography, and surface submental electromyography;

(ix) Surface electromyography;

(x) Electrocardiography;

(xi) Respiratory effort monitoring, including thoracic and abdominal movement;

(xii) Plethysmography blood flow monitoring;

(xiii) Snore monitoring;

(xiv) Audio or video monitoring;

(xv) Body movement monitoring;

(xvi) Nocturnal penile tumescence monitoring;

(xvii) Nasal and oral airflow monitoring;

(xviii) Body temperature monitoring; and

(xiv) Monitoring the effects that a mask or oral appliance used to treat sleep disorders has on sleep patterns; however, the mask or oral appliance must not extend into the trachea or attach to an artificial airway;

(B) Observing and monitoring physical signs and symptoms, general behavior, and general physical response to polysomnographic evaluation and determining whether initiation, modification, or discontinuation of a treatment regimen is warranted;

(C) Analyzing and scoring data collected during the monitoring described in subdivisions (a)(7)(A) and (B) for the purpose of assisting a licensed physician in the diagnosis and treatment of sleep and wake disorders that result from developmental defects, the aging process, physical injury, disease, or actual or anticipated somatic dysfunction;

(D) Implementation of a written or verbal order from a licensed physician that requires the practice of polysomnography; and

(E) Education of a patient regarding the treatment regimen that assists the patient in improving the patient's sleep; and

(8) "Sleep-related services" means acts performed by polysomnographic technicians, polysomnographic trainees, polysomnographic students, and other persons permitted to perform those services under this part, in a setting described in subsection (c) that would be considered the practice of polysomnography if performed by a polysomnographic technologist.

(b) A licensed dentist shall make or direct the making and use of any oral appliance used to treat sleep disordered breathing and shall evaluate the structures of the patient's oral and maxillofacial region for purposes of fitting the appliance.

(c) The practice of polysomnography must take place only in a hospital, a stand-alone sleep laboratory or sleep center, or in the patient's home in accordance with a physician's order; however, the scoring of data and the education of patients may take place in settings other than in a sleep laboratory, sleep center, or the patient's home.

63-33-802. Exemptions.

(a) This part does not limit or restrict a healthcare practitioner licensed under this title from engaging in the full scope of practice of the person's profession.

(b) This part does not apply to diagnostic electroencephalograms conducted in accordance with the guidelines of the American Clinical Neurophysiology Society.

63-33-803. Documentation required from applicants – License renewals.

(a) In addition to the requirements of § 63-33-103, an applicant for licensure under this part shall provide documentation in the form required by the commissioner that the applicant:

(1) Meets one (1) of the following educational requirements:

(A) Graduation from a polysomnographic educational program that is accredited by the Commission on Accreditation of Allied Health Education Programs;

(B) Graduation from a respiratory care educational program that is accredited by the Commission on Accreditation of Allied Health Education Programs and completion of the curriculum for a polysomnography certificate established and accredited by the Committee on Accreditation for Respiratory Care of the Commission on Accreditation of Allied Health Education Programs;

(C) Graduation from an electro neurodiagnostic technologist educational program with a polysomnographic technology track that is accredited by the Commission on Accreditation of Allied Health Education Programs; or

(D) Successful completion of an accredited sleep technologist educational program (A-STEP) that is accredited by the American Academy of Sleep Medicine;

(2) Has passed the national certifying examination given by the Board of Registered Polysomnographic Technologists;

(3) Is credentialed by the Board of Registered Polysomnographic Technologists; and

(4) Has met any additional educational or clinical requirements established by the commissioner.

(b) A person who was engaged in the practice of polysomnography on July 1, 2007, is eligible for licensure under this part without meeting the educational requirement of subdivision (a)(1) if the person meets the requirements of subdivisions (a)(2)-(4).

(c) To be eligible for renewal of a license to engage in the practice of polysomnography, a polysomnographic technologist must continue to be credentialed by the Board of Registered Polysomnographic Technologists.

63-33-804. Unlicensed persons authorized to provide sleep-related services.

(a) The following persons may provide sleep-related services without being licensed as a polysomnographic technologist under this part:

(1) A polysomnographic technician who provides sleep-related services under the general supervision of a licensed physician for a period of up to one (1) year from the date of the person's graduation from one (1) of the accredited programs described in § 63-33-803(a)(1). The commissioner may, in the commissioner's sole discretion, grant a one-time extension of up to three (3) months beyond this one-year period;

(2) A polysomnographic trainee who provides sleep-related services under the direct supervision of a polysomnographic technologist as a part of the person's educational program while actively enrolled in an accredited sleep technologist educational program (A-STEP) that is accredited by the American Academy of Sleep Medicine;

(3) A polysomnographic student who provides sleep-related services under the direct supervision of a polysomnographic technologist as a part of the person's educational program while actively enrolled in a polysomnographic educational program that is accredited by the Commission on Accreditation of Allied Health Education Programs (CAAHEP);

(4) A person who is credentialed in one (1) of the health-related fields accepted by the Board of Registered Polysomnographic Technologists who provides sleep-related services under the direct supervision of a polysomnographic technologist, for a period of up to one (1) year, while obtaining

the clinical experience necessary to be eligible to sit for the examination given by the Board of Registered Polysomnographic Technologists; and

(5) Respiratory therapists who provide polysomnography services after being credentialed as a registered polysomnographic technologist by the Board of Registered Polysomnographic Technologists, or as a sleep disorders specialist by the National Board for Respiratory Care, or have undergone a standardized, uniform mechanism to document competency in polysomnography as approved by the commissioner. Documentation of such a credential or of passage of this commissioner-approved mechanism must be made available at the request of the commissioner. Respiratory therapists are not required to have a second license as a polysomnographic technologist.

(b) Before providing sleep-related services, a polysomnographic technician shall obtain a temporary permit from the commissioner. While providing sleep-related services, the technician shall wear a badge that appropriately identifies the person as a polysomnographic technician.

(c) Before providing sleep-related services, a polysomnographic trainee shall give notice to the commissioner that the trainee is enrolled in an A-STEP educational program accredited by the American Academy of Sleep Medicine. Trainees shall wear a badge that appropriately identifies the trainee as a polysomnographic trainee while providing such services.

(d) Before providing sleep-related services, a person who is obtaining clinical experience pursuant to subdivision (a)(4) shall give notice to the commissioner that the person is working under the direct supervision of a polysomnographic technologist in order to gain the experience to be eligible to sit for the examination given by the Board of Registered Polysomnographic Technologists. The person shall wear a badge that appropriately identifies the person while providing such services.

(e) Polysomnographic students must not receive compensation for sleep-related services they provide and wear badges that appropriately identify them as students.

63-33-805. Use of title "Polysomnographic technologist."

A person who is licensed to engage in the practice of polysomnography in this state has the right to use the title "polysomnographic technologist" or the abbreviation "PSGP." Any other person shall not use that title or abbreviation, or any other words or letters, indicating that the person is a polysomnographic technologist.

SECTION 77. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-901. Part definitions – Authorized settings for the practice of respiratory care – Exempted persons.

(a) As used in this part, unless the context otherwise requires:

(1) "Certified respiratory therapist" means a person licensed under this part who has obtained and maintains the credential "certified respiratory therapist" (CRT) or the credential "certified respiratory therapy technician" (CRTT) from the National Board for Respiratory Care;

(2) "Practice of respiratory care":

(A)

(i) Means:

(a) Under the supervision, control, and responsibility of a licensed physician, the therapy, management, education and instruction, rehabilitation, diagnostic testing evaluation, and care of patients with deficiencies and abnormalities that affect the cardiorespiratory system and associated aspects of other system functions; or

(b) Under the supervision, control, and responsibility of a licensed physician, the performance of cardiorespiratory research, cardiorespiratory health promotion and disease prevention, and community wellness and education programs;

(ii) Includes, under the supervision, control, and responsibility of a licensed physician:

(a) The administration of pharmacologic agents and medical gasses necessary to diagnose, implement treatment, promote disease prevention, and provide rehabilitation to the cardiorespiratory system; however, a respiratory care practitioner shall not prescribe controlled substances of any kind;

(b) The performance of specific diagnostic and testing techniques employed in the medical management of patients to assist in diagnosis, patient and family education, monitoring, treatment, maintenance of patient health status, and cardiorespiratory research, including, but not limited to, pulmonary function testing, hemodynamic and physiologic monitoring and diagnosis of cardiac function, and specimen collection and analysis;

(c) The establishment and maintenance of the airways, bronchopulmonary hygiene, and cardiopulmonary resuscitation, and cardiac and respiratory life support; and

(d) The receipt and implementation of written and verbal physician orders and the development and implementation of care plans and protocols;

(3) "Registered respiratory therapist" means a person licensed under this part who has obtained and maintains the credential "registered respiratory therapist" (RRT) from the National Board for Respiratory Care;

(4) "Respiratory assistant" or "assistant" means a person certified as such pursuant to this part, who has:

(A) Received on-the-job training equivalent to at least one thousand (1,000) hours of clinical training in respiratory care, during an eighteen-consecutive-month period that was received under the supervision of:

- (i) A registered respiratory therapist;
- (ii) A certified respiratory therapist; or
- (iii) A licensed physician experienced in respiratory care;

and

(B) Passed a commissioner-approved examination for respiratory assistants. The National Board for Respiratory Care (NBRC) entry level examination, as provided to the board by the NBRC, by contract, is the examination accepted by the commissioner. This examination does not grant the respiratory assistant the NBRC credential. In addition, the commissioner may write the commissioner's own state board examination or contract with other national testing organizations; and

(5) "Respiratory care practitioner" means a registered respiratory therapist, a certified respiratory therapist, or a respiratory assistant licensed under this part.

(b) The practice of respiratory care may be performed in an inpatient or outpatient setting, clinic, hospital, nursing home facility, private dwelling, or other place deemed appropriate or necessary.

(c) Registered respiratory therapists, certified respiratory therapists, and respiratory assistants who have been issued a license or certificate by the commissioner, pursuant to the authority of § 63-33-902, are exempt from any licensing, certification, or permit requirements of the Tennessee Medical Laboratory Act, compiled in title 68, chapter 29, for medical laboratory personnel or medical laboratory trainees insofar as they engage in performing blood gases and PH analyses.

63-33-902. Licenses required – Documentation required from applicants – License upgrades.

(a) In order to safeguard life, health, and property and promote public welfare by requiring that only properly qualified persons practice respiratory care in this state, a person practicing respiratory care must be licensed as provided in this part, and it is unlawful for any person to practice or offer to practice respiratory care unless the person has been duly licensed under this part, except as provided in this section.

(b)

(1) In addition to the requirements of § 63-33-103, an applicant for licensure as a registered respiratory therapist shall provide documentation in the form required by the commissioner that the applicant has successfully completed a respiratory care educational program accredited by the Commission on Accreditation of Allied Health Education Programs (CAAHEP) in collaboration with the Committee on Accreditation for Respiratory Care (CoARC), or their successor organizations, and has successfully passed the National Board for Respiratory Care registry examination for advanced respiratory therapists.

(2) In addition to the requirements of § 63-33-103, an applicant for licensure as a certified respiratory therapist must provide documentation in the form required by the commissioner that the applicant has successfully completed a respiratory care educational program accredited by the CAAHEP in collaboration with the CoARC, or their successor organizations, and has

successfully passed the National Board for Respiratory Care certification examination for entry level respiratory therapists.

(3) After January 1, 2000, a person must not be licensed as a respiratory assistant if that person has not been previously licensed as a respiratory assistant in this state. However, persons who have been licensed as respiratory assistants may continue to renew their licenses indefinitely so long as they meet all the requirements for renewal.

(c)

(1) A person licensed to practice as a certified respiratory therapist may upgrade the license to registered respiratory therapist if the person furnishes evidence satisfactory to the commissioner that the person meets all the requirements to be licensed as a registered respiratory therapist.

(2) A person licensed to practice as a respiratory assistant may upgrade the license to certified respiratory therapist or registered respiratory therapist if the person furnishes evidence satisfactory to the commissioner that the person meets all the requirements to be licensed as a certified respiratory therapist or registered respiratory therapist.

(3) A person must not hold more than one (1) license to practice respiratory care in this state at the same time.

63-33-903. Knowledge and skills required of registered respiratory therapists – Independent clinical judgment authorized.

(a) The knowledge and skills for performing the functions of a registered respiratory therapist must be acquired by academic and clinical preparation in a respiratory care program approved by the Commission on Accreditation of Allied Health Education Programs (CAAHEP), or its successor organization, or other accrediting organization recognized by the commissioner or by registry from the National Board for Respiratory Care. Registered respiratory therapists may assume primary clinical

responsibility for all respiratory care modalities. Such modalities must include, but are not limited to, accepting telephone and verbal orders and responsibilities involving the supervision of certified respiratory therapists and respiratory assistants.

(b) Under the supervision of a physician, a registered respiratory therapist may be required to exercise considerable independent clinical judgment in the respiratory care of patients. The registered respiratory therapist must also be capable of serving as a technical resource person to the physician regarding current practices in respiratory care and to the hospital staff regarding effective and safe methods for administering respiratory care.

**63-33-904. Knowledge and skills required of certified respiratory therapists –
Independent clinical judgment prohibited.**

(a) The knowledge and skills for performing the functions of a certified respiratory therapist must be acquired by academic and clinical preparation in a respiratory care program approved by the Commission on Accreditation of Allied Health Education Programs (CAAHEP), or its successor organization, or other accrediting organization recognized by the commissioner or by certification from the National Board for Respiratory Care. Under the supervision of a physician, certified respiratory therapists may assume clinical responsibility for specified respiratory care modalities involving the application of well-defined therapeutic techniques.

(b) The certified respiratory therapist's role must not require the exercising of independent clinical judgment; however, the certified respiratory therapist may be expected to adjust or modify therapeutic techniques within well-defined procedures based on a limited range of patient responses. The effective use of the certified respiratory therapist, especially in the critical care setting, requires the supervision of a physician experienced in respiratory care. The certified respiratory therapist may accept telephone or verbal orders and responsibilities involving the supervision of respiratory care therapists and respiratory assistants.

63-33-905. Knowledge and skills required of respiratory assistants – Independent clinical judgment prohibited.

(a) The knowledge and skills for performing the functions of a respiratory assistant must be acquired through compliance with the requirements of § 63-33-901(a)(4).

(b) The assistant's role must not include the exercising of independent clinical judgment. The assistant may be expected to adjust or modify therapeutic techniques within well-defined procedures based on a limited range of patient responses. The use of the assistant requires the supervision of a registered respiratory therapist, certified respiratory therapist, or physician experienced in respiratory care. An assistant is not permitted to perform the functions for which the assistant is certified on a critical care patient being maintained on mechanical ventilation, unless the assistant possesses a certificate that indicates the assistant has passed the critical care section of the board-approved examination for respiratory assistants.

63-33-906. Exempted persons.

(a)

(1) This part does not prohibit:

(A) A person licensed or certified to practice any of the other health-related professions in this state under any other law from engaging in the practice for which the person is licensed or certified; or

(B) Persons employed as subsidiary workers who do not represent themselves to be registered respiratory therapists, certified respiratory therapists, or respiratory assistants from assisting in the respiratory care of patients under the direction and supervision of a licensed physician.

(2) As used in this subsection (a), "subsidiary worker":

(A) Means an individual who occasionally administers oxygen or other therapeutic gas under the orders of a licensed physician, osteopath, or doctor of dentistry and who is not involved in any other aspect of the direct delivery of respiratory care procedures or the administration of diagnostic tests of the cardiorespiratory function; and

(B) Does not include an individual who delivers and installs respiratory care equipment in hospitals, in the home, and in alternate care sites.

(b) This part does not apply to a registered nurse or to a licensed practical nurse employed by a licensed nursing home when such person is under the supervision of a registered nurse or a licensed physician as long as such licensed practical nurse does not maintain and manage life support systems.

(c) With respect to licensed healthcare professionals who lawfully engage in the practice of respiratory care within the scope of practice of their professions, the commissioner may develop mechanisms and standards for ensuring the competency of such licensed professionals in their practice of respiratory care, and may recommend to the health-related board for each such profession that that board adopt, by rule or otherwise, mechanisms and standards for ensuring competency in the practice of respiratory care; however, the commissioner has no authority to regulate a healthcare professional subject to rule by another health-related board.

(d) This part does not prohibit the practice of respiratory care by students while they are under the supervision of instructors in a respiratory care educational program that has been approved by the Commission on Accreditation of Allied Health Education Programs (CAAHEP), or its successor organization, as long as the students' activities are not performed for compensation and the students wear an appropriate badge indicating their status as students.

(e) This part does not prohibit self-care by the patient or the gratuitous care by a friend or member of the family who does not represent or hold out to be a respiratory care practitioner licensed under this part.

(f) This part does not prohibit an individual who holds a credential from the National Board for Respiratory Care entitled "certified pulmonary function technologist (CPFT)" or "registered pulmonary function technologist (RPFT)" from engaging in the performance of pulmonary function diagnostic testing.

63-33-907. Licensees authorized to perform blood gas analysis.

Blood gas analysis and maintenance and quality assurance of a blood gas laboratory must not be performed by a practitioner licensed under this part unless:

(1) In the case of a registered respiratory therapist or certified respiratory therapist, the registered respiratory therapist or certified respiratory therapist:

(A) Has been trained in blood gas analysis as approved by the commissioner;

(B) Has been granted a license by the commissioner bearing the endorsement "ABG analyst"; and

(C) Holds either the NBRC credentials "CRT," "CRTT," or "RRT" or has passed a commissioner-approved examination testing competency for blood gas analysis; and

(2) In the case of a respiratory assistant:

(A) Has been trained in blood gas analysis as approved by the commissioner;

(B) Has been granted a license by the commissioner bearing the endorsement "ABG analyst"; and

(C) Has passed a commissioner-approved examination testing competency for blood gas analysis. The National Board for Respiratory Care entry level examination, as provided to the commissioner by the

NBRC by contract, is the examination accepted by the commissioner.

This examination does not grant the respiratory assistant the NBRC credential. In addition, the commissioner may write the commissioner's own state council examination or contract with other national testing organizations.

63-33-908. Temporary license.

The commissioner may issue a temporary license to an individual applicant who has completed the required respiratory care educational program but who has not yet successfully completed the NBRC examination. A temporary license shall automatically expire one (1) year from its date of issuance and must not be renewed.

63-33-909. Uniform standards for persons delivering and installing respiratory equipment.

As a part of the board's licensing and regulation of home medical equipment providers under § 68-11-226, the board for licensing health care facilities shall establish uniform standards to ensure the competency of persons who deliver and install respiratory equipment.

SECTION 78. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-1201. Part definitions.

As used in this part:

(1) "Cosmetic medical service" means a service that uses a biologic or synthetic material, a chemical application, a mechanical device, or a displaced energy form of any kind that alters or damages, or is capable of altering or damaging, living tissue to improve the patient's appearance or achieve an enhanced aesthetic result;

(2) "Media" or "advertising":

(A) Means oral, written, and other types of communication disseminated for the purpose of soliciting medical services; and

(B) Includes newspaper or magazine advertisements, telephone directory displays, printed brochures or leaflets, websites, email correspondence, and television or radio announcements;

(3) "Medical director" means a physician who:

(A) Holds an active medical license under chapter 6 or 9 of this title in this state;

(B) Has an active medical practice in this state; and

(C) Is responsible for the provision of cosmetic medical services;

and

(4) "Medical spa":

(A) Means an entity, however named or organized, that offers or performs cosmetic medical services; and

(B) Does not include an office or practice owned by a physician at which the physician is physically present at least fifty percent (50%) of the medical spa's regular hours of operation.

63-33-1202. Signage required.

(a) An entity doing business as, or advertised as, a medical spa shall display the name of the medical director and indicate one (1) of the following by signage at its practice site and in its media and advertising:

(1) Whether the medical director is certified or eligible for certification by a private or public board, parent association, multidisciplinary board, or association that is a member of the American Board of Medical Specialties (ABMS) or the American Osteopathic Association (AOA);

(2) Whether the medical director is certified by a board or association with equivalent requirements to the ABMS or AOA as approved and recognized

by the board of medical examiners or the board of osteopathic examination, as appropriate; or

(3) Whether the medical director is certified by a board or association requiring an Accreditation Council for Graduate Medical Education (ACGME) or AOA-approved training program that provides complete training in the specialty or subspecialty certified, followed by prerequisite certification by a certifying board of the ABMS or AOA in that training field and successful completion of an additional examination in the specialty or subspecialty certified.

(b) If the medical director is not certified by any of the entities identified in subsection (a), then the lack of certification must be displayed by signage at its practice site and in its media and advertising.

63-33-1203. Regulation by board – Violations – Notice of changes in employment.

(a) Each licensed healthcare practitioner who provides services at a medical spa must continue to be regulated only by the board that has issued a license to that practitioner.

(b) A violation of this part, or a rule promulgated under this part, by a licensed healthcare practitioner while providing services at a medical spa that is required be licensed by this part, is grounds for disciplinary action against that practitioner by the board that licensed the practitioner.

(c) A licensed healthcare practitioner shall notify the board that has licensed the practitioner within ten (10) days of starting or ending work at a medical spa.

63-33-1204. Rules.

In addition to the rulemaking authority conveyed to the commissioner in § 63-33-102, the commissioner shall promulgate rules under this part that address the following topics:

- (1) Personnel requirements for the medical spa;
- (2) Training requirements for healthcare providers;

- (3) Patient records;
- (4) Standards to ensure quality of patient care;
- (5) Infection control;
- (6) Health and safety requirements;
- (7) Data collection and reporting requirements;
- (8) Inspections; and
- (9) Patient billing procedures.

63-33-1205. Documentation required from applicants.

(a) In addition to the requirements of § 63-33-103, an applicant for licensure under this part shall provide the following documentation in the form required by the commissioner:

- (1) That the applicant for the medical spa license is the medical director of the medical spa to be licensed;
- (2) The medical license number of the medical director of the medical spa and whether the medical director is a medical doctor or a doctor of osteopathic medicine;
- (3) The certification information of the medical director as required by § 63-33-1202;
- (4) A list of all medical spas in this state for which the applicant serves as the medical director;
- (5) A list of the names of all owners of the medical spa;
- (6) A list of the names and Tennessee license numbers for employees who will provide cosmetic medical service at the medical spa;
- (7) The location of the medical spa; and
- (8) Any other information the commissioner requires by rule.

(b) A medical spa license is only valid at the location identified in the licensure application. Any changes in location require a new medical spa license, which must be obtained via the initial licensure application process.

(c) A medical spa license is only valid while the original applicant serves as the medical director of the medical spa. If a medical spa changes its medical director, then a new medical spa license must be obtained via the initial licensure application process by the new medical director.

(d) An existing licensed medical spa that changes medical directors may continue to operate on the existing license for sixty (60) days after changing medical directors so that there can be an orderly transition to the newly licensed medical spa.

63-33-1206. Information required from medical directors.

In addition to the updated information required by § 63-33-105(e), the medical director of every licensed medical spa shall also provide the following information to the division within thirty (30) days of their occurrence:

(1) The departure of a person identified in the application for licensure as someone providing cosmetic medical services at the medical spa;

(2) The name and Tennessee license number of a person not identified in the original application form who will provide cosmetic services at the medical spa;

(3) Any change in the ownership of the medical spa; and

(4) Any other information the commissioner requires by rule.

63-33-1207. Investigations by board.

Each board has the authority to investigate a complaint alleging a violation of this part, or a rule adopted under this part, by a medical spa utilizing the services of a healthcare practitioner licensed by that board. Each board also has the authority to investigate a complaint alleging that a medical spa utilizing the services of a healthcare

practitioner licensed by that board is not properly licensed by the division as required by this part.

63-33-1208. Investigations and discipline by commissioner.

The commissioner may investigate and discipline a medical spa license for violations of this part or rules promulgated by the commissioner pursuant to this part, or the reasons listed in § 63-33-106. The commissioner shall not investigate or discipline a healthcare practitioner licensed by a board created under this title or under title 68.

SECTION 79. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-1301. Part definitions.

As used in this part, unless the context otherwise requires:

(1) "Nursing home":

(A) Means an institution or facility as defined in § 68-11-201 or the rules for nursing homes promulgated by the health facilities commission; and

(B) Includes Christian Science sanatoria and services;

(2) "Nursing home administrator" means an individual responsible for planning, organizing, directing, or controlling the operation of a nursing home or who in fact performs such functions, whether or not such functions are shared by one (1) or more other people;

(3) "Practice of nursing home administration" means the planning, organizing, directing, or controlling of the operation of a nursing home; and

(4) "Single state agency" means the department of health.

63-33-1302. Commissioner's duties.

In addition to the authority granted and duties imposed by part 1 of this chapter, the commissioner shall:

(1) Conduct a continuing study and investigation of nursing homes and administrators of nursing homes within the state with a view to the improvement of standards imposed for the licensing of such administrators and of procedures and methods for the enforcement of such standards with respect to administrators of nursing homes who have been licensed as such;

(2) Conduct or cause to be conducted courses of instruction and training sufficient to meet the requirements of this part. In lieu thereof, the commissioner may approve courses conducted by other groups or organizations; and

(3) Receive and disburse federal funds pursuant to § 1908(e)(1) of the Social Security Act (42 U.S.C. § 1396g(e)(1)).

63-33-1303. Documentation required from applicants – Examination fee – Temporary license.

(a) In addition to the requirements of § 63-33-103, each applicant for licensure as a nursing home administrator shall provide documentation in the form required by the commissioner that the applicant:

(1) Has graduated from an accredited high school or has completed equivalent recognized education; and

(2) Has completed additional educational requirements established by the commissioner via rule.

(b) Upon payment of an examination fee set by the commissioner, an applicant who has satisfied the requirements of subsection (a) must be admitted to the examination.

(c) The commissioner may grant a temporary license, not to exceed six (6) months, to an individual applying for licensure to fill a position of nursing home administrator that unexpectedly becomes vacant. To receive a temporary license, the individual must either be licensed in another state or meet the majority of the commissioner's standards for licensure. If the individual holding a temporary license

must take the commissioner-required examination to be fully licensed and fails to achieve the required scores, then the temporary license is revoked.

63-33-1304. Examination.

(a) The commissioner shall determine the subjects of examination for applicants for licensure as nursing home administrators. The commissioner shall also determine the scope, content, and format of such an examination, which in any examination must be the same for all candidates; however, all examinations must include testing of the applicant's proficiency in the rules of health and safety.

(b) An applicant having failed to achieve a passing grade, score, or level of proficiency as determined by the commissioner on such examination is not prohibited from taking successive examinations as long as such applicant pays the examination fee set by the commissioner pursuant to § 63-33-1303 for each successive examination.

63-33-1305. Required notice from health facilities commission.

The health facilities commission shall notify the commissioner of all cases in which a nursing home is sanctioned by suspension of admissions or imposition of a civil penalty or in which the health facilities commission determines that the quality of care provided to residents is seriously inadequate due to acts or omissions of the administrator.

63-33-1306. Violations.

In addition to the reasons enumerated in § 63-33-106(b), the commissioner may take any action identified in § 63-33-106(a) with respect to an applicant for license under this part or against a licensee under this part for the following reasons:

(1) Upon proof that the licensee has willfully or repeatedly violated this chapter or the rules promulgated in accordance with this chapter, or willfully or repeatedly acted in a manner inconsistent with the health and safety of the patients in the home in which the licensee is the administrator; or

(2) Upon proof that the licensee is guilty of fraud or deceit in the practice of nursing home administration.

63-33-1307. Exemption for persons certified by churches or religious denominations.

(a) This part and the rules promulgated under this part do not require an applicant for a license who is certified by a recognized church or religious denomination that teaches reliance on spiritual means alone for healing to demonstrate proficiency in any medical techniques or to meet any medical educational qualifications or medical standards not in accordance with remedial care and treatment provided in such institutions.

(b) Such administrative practice is limited to institutions certified by such church or denominations for the care and treatment of the sick in accordance with its teachings.

63-33-1308. Operation of nursing home without licensed administrator supervision prohibited.

A nursing home in the state shall not operate unless it is under the supervision of an administrator who holds a currently valid nursing home administrator license and registration or provisional license issued pursuant to this part.

63-33-1309. Civil action.

The commissioner may cause to be instituted a civil action in a court of competent jurisdiction for injunctive relief to prevent a violation of this chapter or a rule promulgated by the commissioner pursuant to this part.

63-33-1310. Temporary administration following loss permitted.

Notwithstanding this part to the contrary, a nursing home licensed in this state may be administered on a temporary basis after the unexpected loss of an administrator in accordance with title 68, chapter 11, part 2.

SECTION 80. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-1401. "Podiatrist" defined for this part – Requirements to perform surgeries

– Collaboration permitted.

(a)

(1) As used in this part, "podiatrist" means a person who examines, diagnoses, or treats, medically, mechanically, or surgically, the ailments of the human foot, ankle, and soft tissue structures extending no higher than the distal tibial metaphyseal flair, including the use and prescribing of drugs and medications, but excluding the direct applications of general anesthesia by a podiatrist and the amputation of the foot.

(2) A podiatrist may perform Achilles tendon repair, subject to subsection (b), but shall not perform surgery on Pilon fractures or tibial fractures that do not enter the ankle joint.

(b) A licensed podiatrist may perform ankle surgery only when the podiatrist meets either of the following requirements:

(1) Was, prior to October 2, 1995, credentialed to perform ankle surgery by a JCAHO-accredited hospital in this state or JCAHO-accredited ambulatory surgical treatment center in this state; provided, that a podiatrist who qualifies to perform ankle surgery under this subdivision (b)(1) must, if the podiatrist wishes to continue to perform ankle surgery:

(A) Notify the department as to the hospital or ambulatory surgical treatment center at which the podiatrist was credentialed to perform ankle surgery prior to October 2, 1995;

(B) Obtain and maintain evidence of obtaining at least ten (10) hours of approved continuing education related to ankle surgery annually; and

(C) Comply with any other requirements established by the commissioner to ensure continued proficiency in performing ankle surgery; or

(2) Has completed a surgical residency program at least twenty-four (24) months in length that has been approved by the Council on Podiatric Medical Education and is currently eligible for certification to perform reconstructive rearfoot/ankle surgery by the American Board of Podiatric Surgery; provided, that a podiatrist who qualifies to perform ankle surgery under this subdivision (b)(2) must obtain certification to perform reconstructive rearfoot/ankle surgery from the American Board of Podiatric Surgery within six (6) years of May 24, 2000, or within six (6) years of first becoming eligible for certification, whichever date is later, and shall not continue to perform ankle surgery if the podiatrist has not obtained certification by the end of the six-year period.

(c)

(1) A licensed podiatrist may perform ankle surgery only in an accredited hospital or ambulatory surgical treatment center licensed under title 68 at which the podiatrist has surgical privileges to perform ankle surgery. A podiatrist who performs ankle surgery in an ambulatory surgical treatment center must first have surgical privileges to perform ankle surgery at a local hospital. A licensed podiatrist who performs ankle surgery shall meet the same standard of care applicable to orthopedic surgeons who perform ankle surgery.

(2) As used in this subsection (c):

(A) "Accredited ambulatory surgical treatment center" means an ambulatory surgical treatment center that is accredited by joint commission, Accreditation Association for Ambulatory Health Care, or American Association for Accreditation of Ambulatory Surgical Facilities; and

(B) "Accredited hospital" means a hospital that is accredited by joint commission or the American Osteopathic Association.

(d) Licensed podiatrists may perform nonsurgical care on the ankle without meeting the requirements of subsection (b).

(e) A licensed podiatrist may collaborate with a physician assistant or an orthopedic physician assistant licensed pursuant to chapter 19 of this title. A podiatrist collaborating with a physician assistant or orthopedic physician assistant pursuant to this subsection (e) shall comply with the requirements of, and any rules adopted pursuant to, chapter 19 of this title governing collaboration with a physician assistant.

63-33-1402. Powers and duties of commissioner.

In addition to the powers and duties granted to or imposed by this chapter, the commissioner has the authority to:

- (1) Adopt examinations to ascertain the qualifications and fitness of applicants for a license to practice podiatry in this state;
- (2) Provide standards by which podiatry schools and colleges are approved; and
- (3) Sponsor, conduct, or approve such educational programs as are necessary to carry out and make effective this part and part 1 of this chapter.

63-33-1403. Exempted persons.

The requirements of this part do not apply to:

- (1) The commissioned physicians and surgeons of the United States army, navy, air force, or marine hospital service in the actual performance of their official duties;
- (2) Physicians and surgeons licensed pursuant to chapter 6 of this title;
- (3) Osteopathic physicians and surgeons licensed pursuant to chapter 9 of this title; or

(4) Visiting podiatrists called into consultation in this state from another state where they are duly qualified under the laws of that state to practice podiatry.

63-33-1404. Licenses.

(a) A person desiring an academic license to participate in a residency program shall:

(1) Apply on a form prescribed by the commissioner or via online application for an academic license;

(2) Pay a fee as determined by the commissioner; and

(3) Have a residency program accepted by the commissioner certify that the applicant has been accepted for its program.

(b) The academic license terminates upon the applicant leaving the residency program.

(c) In addition to the requirements of § 63-33-103, an applicant for licensure as a podiatrist shall also:

(1) Pass the examination conducted or accepted by the commissioner; and

(2) Successfully complete at least a one-year residency program approved by the Council on Podiatric Medical Education or its successor organization.

(d) The commissioner has the authority to issue locum tenens licenses to podiatrists licensed in another state who wish to practice podiatry in this state for a limited period of time and who meet all the requirements established by the commissioner by rule for a locum tenens license.

63-33-1405. Requirements to take examination.

A person is not entitled to take an examination for a license unless that person furnishes the division with satisfactory proof that the person has met the requirements

listed in § 63-33-103 and has graduated from a college of podiatric medicine that is accredited by the Council on Podiatric Medical Education or its successor organization.

63-33-1406. Examination.

(a)

(1) The commissioner may conduct examinations at such times and places as the commissioner may designate. If the commissioner chooses to provide an examination, then the examination must be in the English language, written and clinical, and include subjects of histology, surgery, hygiene, dermatology, anatomy, physiology, chemistry, bacteriology, pathology, diagnosis and treatment, materia medica and therapeutics, clinical podiatry, and such additional subjects as are pertinent to podiatry. Such examinations must be limited in scope as to cover only the requirements for podiatry education as provided in this part.

(2) The commissioner may adopt the examinations given by the National Board of Podiatric Medical Examiners (NBPME) and its score reports on any examinee.

(b) The commissioner shall establish the minimum passing grade by rule.

63-33-1407. Public health requirements.

Licensed podiatrists shall observe and are subject to all state and municipal regulations relating to the control of contagious and infectious diseases and all matters pertaining to public health, including making reports to proper health officers in the same manner as other practitioners of the healing arts are required to make.

63-33-1408. Collaborating physician assistants.

A physician assistant collaborating with a licensed podiatrist shall not provide services that are outside of the scope of practice of a podiatrist as set forth in § 63-33-1401 and shall prescribe only drugs that are rational to the practice of podiatry. An orthopedic physician assistant collaborating with a licensed podiatrist shall not provide

services that are outside of the scope of practice of a podiatrist as set forth in § 63-33-1401.

63-33-1409. Prohibited conduct.

(a) In addition to the disciplinary grounds listed in § 63-33-106(b), the commissioner may also exercise the powers enumerated in § 63-33-106(a) for the following:

(1) Solicitation by agents or persons or profiting by the acts of those representing themselves to be agents of the licensee or the certificate holder;

(2) Division of fees or agreeing to split or divide fees received for professional services with a person for bringing or referring a patient;

(3) Professional connection or association with a person, firm, or corporation in any manner in an effort to avoid and circumvent this part or part 1 of this chapter, or lending one's name to another for the illegal practice of podiatry by such person;

(4) Giving of testimonials, directly or indirectly, concerning the supposed virtue of secret therapeutic agents or proprietary preparations, such as remedies, vaccines, or other articles or materials that are offered to the public, claiming radical cure or prevention of disease by their use;

(5) Making false statements or representations or being guilty of fraud or deceit in the practice of podiatry;

(6) Invasion of a field of practice in which the licensee is not licensed to practice or is not within the limits of the licensee's respective scope of practice;

(7) Dispensing, prescribing, or otherwise distributing a controlled substance or any other drug not in the course of professional practice or not in good faith to relieve pain and suffering, or not to cure an ailment, physical infirmity, or disease;

(8) Dispensing, prescribing, or otherwise distributing a controlled substance, controlled substance analogue, or any other drug to a person in violation of the law of this state or of the United States;

(9) Practicing podiatry under a trade name or a name designating a given location or the name of another podiatrist or a name other than that which appears on the practitioner's license or using a title other than "podiatrist"; provided, that "foot specialist" may be used as an explanatory term of the title "podiatrist" and not alone or as a substitute for the title "podiatrist";

(10) Willful violation of the rules promulgated by the commissioner; or

(11) Making or signing in a person's professional capacity a certificate that is known to be false at the time the person makes or signs such certificate.

(b) The commissioner may adopt rules to regulate the nature, manner, content, and extent of advertising by practitioners who are under the jurisdiction of the commissioner. All methods of advertising must be allowed, including, but not limited to, newspaper, radio, and television.

63-33-1410. Retail stores.

This part does not prohibit the usual and customary commercial sale and adjustment of foot appliances and remedies or shoes in retail stores.

63-33-1411. X-ray procedures.

A person shall not perform X-ray procedures in a podiatrist office who does not meet the minimum educational standards and criteria for persons operating X-ray equipment for diagnostic purposes in podiatrist offices established by the commissioner.

63-33-1412.

(a) A handwritten prescription order for a drug prepared by a podiatrist who is authorized by law to prescribe a drug must be legible so that it is comprehensible by the pharmacist who fills the prescription. The handwritten prescription order must contain the name of the prescribing podiatrist; the name and strength of the drug prescribed; the

quantity of the drug prescribed, handwritten in letters or in numerals; instructions for the proper use of the drug; and the month and day that the prescription order was issued, recorded in letters or in numerals or a combination thereof. The prescribing podiatrist shall sign the handwritten prescription order on the day it is issued, unless it is a standing order issued in a hospital, a nursing home, or an assisted care living facility as defined in § 68-11-201.

(b) A typed or computer-generated prescription order for a drug issued by a podiatrist who is authorized by law to prescribe a drug must be legible so that it is comprehensible by the pharmacist who fills the prescription order. The typed or computer-generated prescription order must contain the name of the prescribing podiatrist; the name and strength of the drug prescribed; the quantity of the drug prescribed, recorded in letters or in numerals; instructions for the proper use of the drug; and the month and day that the typed or computer-generated prescription order was issued, recorded in letters or in numerals or a combination thereof. The prescribing podiatrist shall sign the typed or computer-generated prescription order on the day it is issued, unless it is a standing order issued in a hospital, nursing home, or an assisted care living facility as defined in § 68-11-201.

(c) This section does not prevent a podiatrist from issuing a verbal prescription order.

(d)

(1) All handwritten, typed, or computer-generated prescription orders must be issued on either tamper-resistant prescription paper or printed utilizing a technology that results in a tamper-resistant prescription that meets the current centers for medicare and medicaid service guidance to state medicaid directors regarding § 7002(b) of the federal United States Troop Readiness, Veterans' Care, Katrina Recovery, and Iraq Accountability Appropriations Act of 2007 (Pub.

L. No. 110-28), and meets or exceeds specific TennCare requirements for tamper-resistant prescriptions.

(2) Subdivision (d)(1) shall not apply to prescriptions written for inpatients of a hospital; outpatients of a hospital where the doctor or other person authorized to write prescriptions writes the order into the hospital medical record and then the order is given directly to the hospital pharmacy and the patient never has the opportunity to handle the written order; a nursing home or an assisted care living facility as defined in § 68-11-201; or inpatients or residents of a mental health hospital or residential facility licensed under title 33 or individuals incarcerated in a local, state, or federal correctional facility.

SECTION 81. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-1501. Part definitions.

As used in this part, unless the context otherwise requires:

(1) "Custom fabricated and fitted device" means an orthosis, prosthesis, or pedorthic device that is fabricated to original measurements or to a mold for use by a patient in accordance with a prescription from a healthcare practitioner authorized by law to write such prescriptions and that requires substantial clinical and technical judgment in its design and fitting;

(2) "Custom fitted device" means a prefabricated orthosis, prosthesis, or pedorthic device sized or modified for use by the patient in accordance with a prescription from a healthcare practitioner authorized by law to write such prescriptions that requires substantial clinical judgment and substantive alteration for appropriate use;

(3) "Facility" means the business location where orthotic, prosthetic, or pedorthic care is provided. In the case of an orthotic/prosthetic facility, the orthotic/prosthetic facility has the appropriate clinical and laboratory space and

equipment to provide comprehensive orthotic or prosthetic care, and, in the case of a pedorthic facility, the pedorthic facility has the appropriate clinical space and equipment to provide pedorthic care. Licensed orthotists, prosthetists, and pedorthists must be available to either provide care or to supervise the provision of care by nonlicensed staff;

(4) "Licensed orthotist" means a person who is licensed under this part to practice orthotics and who represents the person to the public by title or description of services that includes the term "orthotic," "orthotist," "brace," or a similar title or description of services;

(5) "Licensed pedorthist" means a person who is licensed under this part to practice pedorthics and who represents the person to the public by the title or description of services that includes the term "pedorthic," "pedorthist," or a similar title or description of services;

(6) "Licensed prosthetist" means a person who is licensed under this part to practice prosthetics and who represents the person to the public by title or description of services that includes the term "prosthetic," "prosthetist," "artificial limb," or a similar title or description of services;

(7) "Off-the-shelf device" means a prefabricated prosthesis or orthosis sized or modified for use by the patient in accordance with a prescription from a healthcare practitioner authorized by law to write such prescriptions that does not require substantial clinical judgment and substantive alteration for appropriate use;

(8) "Orthosis":

(A) Means a custom designed, fabricated, fitted, or modified device to correct, support, or compensate for a neuromusculoskeletal disorder or acquired condition; and

(B) Does not include fabric or elastic supports, corsets, arch supports, low-temperature plastic splints, trusses, elastic hoses, canes, crutches, soft cervical collars, dental appliances, or other similar devices that are carried in stock and sold without modification as over-the-counter items by a drug store, department store, corset shop, or surgical supply facility;

(9) "Orthotic and prosthetic education program" means a course of instruction accredited by the Commission on Accreditation of Allied Health Education Programs (CAAHEP), consisting of:

(A) An adequate curriculum of college-level training and instruction in math, physics, biology, chemistry, and psychology; and

(B) A specific curriculum in orthotic or prosthetic courses, including:

(i) Lectures covering pertinent anatomy, biomechanics, pathomechanics, prosthetic-orthotic components and materials, training and functional capabilities, prosthetic or orthotic performance evaluation, prescription considerations, etiology of amputations, and disease processes necessitating prosthetic or orthotic use and medical management;

(ii) Subject matter related to pediatric and geriatric problems;

(iii) Instruction in acute care techniques, such as immediate and early post-surgical prosthetics and fracture bracing techniques; and

(iv) Lectures, demonstrations, and laboratory experiences related to the entire process of measuring, casting, fitting, fabricating, aligning, and completing prostheses or orthoses;

(10) "Orthotics" means the science and practice of evaluating, measuring, designing, fabricating, assembling, fitting, adjusting, or servicing an orthosis, under an order from a licensed healthcare practitioner authorized by law to issue such an order, for the correction or alleviation of neuromuscular or musculoskeletal dysfunction, disease, injury, or deformity;

(11) "Orthotist" means an allied health professional who is specifically trained and educated to provide or manage the provision of a custom-designed, fabricated, modified, and fitted external orthosis to an orthotic patient, based on a clinical assessment and a prescription from a healthcare practitioner authorized by law to write such prescriptions, to restore physiological function or cosmesis;

(12) "Over-the-counter device" means a prefabricated, mass-produced device that is prepackaged and requires no professional advice or judgment in either size selection or use, including fabric or elastic supports, corsets, generic arch supports, and elastic hose;

(13) "Pedorthic device":

(A) Means therapeutic footwear, foot orthoses for use at the ankle or below, and modified footwear made for therapeutic purposes, as prescribed by a licensed healthcare practitioner authorized by law to issue such prescription; and

(B) Does not include nontherapeutic accommodative inlays or nontherapeutic accommodative footwear, regardless of method of manufacture, shoe modifications made for nontherapeutic purposes, unmodified, over-the-counter shoes, or prefabricated foot care products;

(14) "Pedorthic education program" means a course of instruction accredited by the Board for Certification in Pedorthics, consisting of:

(A) A basic curriculum of instruction in foot-related pathology of diseases, anatomy, and biomechanics;

(B) A specific curriculum in pedorthic courses, including lectures covering shoes, foot orthoses and shoe modifications, pedorthic components and materials, training and functional capabilities, pedorthic performance evaluation, prescription considerations, etiology of disease processes necessitating use of pedorthic devices, medical management, and subject matter related to pediatric and geriatric problems; and

(C) Lectures, demonstrations, and laboratory experiences related to the entire process of measuring and casting, fitting, fabricating, aligning, and completing pedorthic devices;

(15) "Pedorthics" means the science and practice of evaluating, measuring, designing, fabricating, assembling, fitting, adjusting, or servicing a pedorthic device, under an order from a licensed healthcare practitioner authorized by law to issue such order for the correction or alleviation of neuromuscular or musculoskeletal dysfunction, disease, injury, or deformity;

(16) "Pedorthist" means a person who measures, designs, fabricates, fits, or services pedorthic devices and assists in the formulation of the order of pedorthic devices, as ordered by a licensed healthcare practitioner authorized by law to issue such order for the support or correction of disabilities caused by neuromusculoskeletal diseases, injuries, or deformities;

(17) "Prosthesis":

(A) Means a custom designed, fabricated, fitted, or modified device to replace an absent external limb, for the purpose of restoring physiological function or cosmesis; and

(B) Does not include artificial eyes, ears, or dental appliances; cosmetic devices such as artificial breasts, eyelashes, or wigs; or other devices that do not have a significant impact on the musculoskeletal functions of the body;

(18) "Prosthetics" means the science and practice of evaluating, measuring, designing, fabricating, assembling, fitting, adjusting, or servicing a prosthesis, under an order from a licensed healthcare practitioner authorized by law to issue such order;

(19) "Prosthetist" means an allied health professional who is specifically trained and educated to provide or manage the provision of a custom designed, fabricated, modified, and fitted external limb prosthesis to a prosthetic patient, based on a clinical assessment and a prescription from a healthcare practitioner authorized to write such prescriptions, to restore physiological function or cosmesis;

(20) "Prosthetist/Orthotist" means a person who practices both disciplines of prosthetics and orthotics and who represents the person to the public by title or by description of services; and

(21) "Resident" means a person who has completed an education program in either orthotics or prosthetics and is continuing the person's clinical education in a residency program approved by the commissioner.

63-33-1502. Powers and duties of commissioner.

In addition to the powers and duties granted to or imposed by this chapter, the commissioner has the authority to:

(1) Establish the qualifications, educational courses, curriculum, hours, and standards that are prerequisite to issuance of all levels and types of licensure established by this part. Such qualifications must include the following:

(A) To qualify for a license to practice orthotics or prosthetics, a person must:

(i) Possess a baccalaureate degree or semester hours equivalent to four (4) years of study at a four-year college or university;

(ii) Complete the amount of formal training, including, but not limited to, any necessary hours of classroom education and clinical practice required by the commissioner;

(iii) Complete a clinical residency in the professional area for which a license is sought, in accordance with standards, guidelines, or procedures for residencies inside or outside this state established and approved by the commissioner. The majority of training must be devoted to services performed under the supervision of a licensed practitioner of orthotics or prosthetics or a person who has obtained certification from the American Board for Certification in Orthotics and Prosthetics, Inc., or the Board for Orthotist/Prosthetist Certification;

(iv) Pass all written, practical, and oral examinations that are required and approved by the commissioner; and

(v) Be qualified to practice in accordance with commonly accepted standards of orthotic and prosthetic care acceptable to the commissioner;

(B) To qualify for a license to practice pedorthics, a person must:

(i) Possess a high school diploma or comparable credential approved by the commissioner;

(ii) Complete the amount of formal training, including, but not limited to, any necessary hours of classroom education and clinical practice required by the commissioner;

(iii) Pass all examinations that are required and approved by the commissioner;

(iv) Complete a qualified work experience program or internship in pedorthics, in accordance with standards and procedures established by the commissioner; and

(v) Be qualified to practice in accordance with commonly accepted standards of pedorthic care acceptable to the commissioner; and

(C) A person may be licensed in more than one (1) discipline;

(2) Establish the circumstances or conditions, if any, under which persons are entitled to exemption from licensure during training, while waiting to take or receive the results of any required examination or upon meeting specified minimum educational and clinical qualifications;

(3) Select the examination or examinations to be utilized as the commissioner's licensure examination or examinations and the prerequisites, if any, for admission to the examination or examinations;

(4) Establish any other criteria for issuance of licensure that are reasonably related to the safe and competent performance of prosthetics, orthotics, and pedorthics;

(5) Accredite continuing education courses;

(6) Establish the fees to be paid for application for continuing education course accreditation;

(7) Establish the continuing education requirements for license holders, including the frequency of reporting, number of hours, types of courses, approval of courses, methods of proving compliance, penalties for violation, and fees necessary for implementation of the continuing education process;

(8) Regulate the nature, manner, content, and extent of advertising by persons licensed under this part;

(9) Delineate the actions relative to therapeutic footwear and medical devices for the foot and ankle that must be performed by licensed healthcare practitioners; and

(10) Establish a registration process for residents as prescribed in § 63-33-1508(2)(B).

63-33-1503. Disciplinary authority.

In addition to the disciplinary grounds listed in § 63-33-106(b), the commissioner may also exercise the powers enumerated in § 63-33-106(a) for the following:

(1) Has knowingly aided and abetted a person who is not a license holder or is not otherwise authorized pursuant to this part to perform the duties of a license holder under this part;

(2) Has undertaken or engaged in a practice beyond the scope of duties permitted to a license holder under this part; or

(3) Has impersonated a license holder or former license holder or is under an assumed name performing the duties authorized to be performed only by a licensed person.

63-33-1504. Licensure required to practice orthotics, prosthetics, or pedorthics.

A person shall not practice orthotics, prosthetics, or pedorthics in this state, or hold out as being able to practice such profession, or dispense an orthosis, prosthesis, or pedorthic device, unless the person is licensed in accordance with this part and part 1 of this chapter.

63-33-1505. Order from licensed healthcare practitioner required.

A licensed orthotist, prosthetist, or pedorthist shall provide care or services only if the care or services are provided pursuant to an order from a licensed healthcare practitioner authorized to issue such an order.

63-33-1506. Scope of practice limits – Prohibited sales.

(a) The scope of practice of a licensed orthotist, prosthetist, or pedorthist does not include the right to diagnose a medical problem or condition or the right to give medical advice as to the nature, cause, or treatment for the problem or condition for which the orthotic, prosthetic, or pedorthic device is being dispensed; however, the scope of practice of a licensed orthotist, prosthetist, or pedorthist does include the right to provide information or demonstration regarding the proper use and care of the device and to make adjustments to the device as needed.

(b) A person shall not dispense or sell an over-the-counter or off-the-shelf device based upon an image of the customer's limb captured by the person through a mold, cast, scanning device, digital appliance, or pressure sensitive device, unless the customer has first presented to that person a written prescription for that device from a healthcare practitioner authorized by law to write such a prescription.

63-33-1507. Exempted persons.

(a) This part and the rules promulgated by the commissioner pursuant to this part do not limit or restrict a healthcare practitioner licensed under this title from engaging in the full scope of practice of such person's profession, training, or services.

(b) This part and the rules promulgated by the commissioner pursuant to this part do not limit or restrict individuals acting under the supervision and control of a pharmacist or pharmacy licensed under this title or a home medical equipment provider licensed under title 68 from measuring, fitting, or adjusting a non-custom-fabricated and fitted device, including, but not limited to, over-the-counter or off-the-shelf devices, so long as the individual does not create a cast, mold, or scan of a part of the human body for the purpose of constructing a medical device to treat a patient's medical condition and so long as the individual meets one (1) of the following criteria for the device:

(1) Documented training from a manufacturer or training from a licensed or certified orthotist, prosthetist, or pedorthist;

(2) Certification or registration as a fitter of orthotic, prosthetic, or pedorthic devices from a nationally recognized board or association, such as the Board for Orthotist/Prosthetist Certification (BOC), the Board of Certification for Pedorthists, the National Community Pharmacists Association (NCPA), or the American Board for Certification in Orthotics and Prosthetics (ABC); or

(3) Direct supervision by a trained and experienced, or certified or registered, fitter of orthotic, prosthetic, or pedorthic devices.

(c) This part and the rules promulgated by the commissioner pursuant to this part do not limit or restrict individuals acting under the supervision and control of a pharmacist or pharmacy licensed under this title or a home medical equipment provider licensed under title 68 from measuring, fitting, or adjusting a non-custom-fabricated and fitted pedorthic device, including, but not limited to, diabetic shoes, so long as the individual meets the criteria of either subdivision (b)(2) or (b)(3) and so long as the individual does not create a cast, mold, or scan of a part of the human body for the purpose of constructing a medical device to treat a patient's medical problem.

63-33-1508. Exemptions for students and employees.

This part does not restrict:

(1) The practice of orthotics, prosthetics, or pedorthics by a person who is employed by the federal government or any bureau, division, or agency of the federal government while in the discharge of the employee's official duties;

(2) The practice of orthotics, prosthetics, or pedorthics by:

(A) A student enrolled in a school of orthotics, prosthetics, or pedorthics;

(B) A resident continuing the resident's clinical education in a residency accredited by the National Commission on Orthotic and Prosthetic Education if the person has first registered as a resident with the division; or

(C) A student in a qualified work experience program or internship in pedorthics; or

(3) The measuring, fitting, or adjusting of an orthotic device by an employee or authorized representative of an orthosis manufacturer registered with the federal food and drug administration, when the employee or representative is supervised by a licensed healthcare professional authorized by law to prescribe, measure, or fit such device, and the measuring, fitting, or adjusting of the device occurs in the office of the licensed healthcare professional or in a healthcare facility.

63-33-1509. Utilization of non-licensed persons to assist.

A licensed orthotist or prosthetist may utilize one (1) or more non-licensed persons to assist in:

- (1) The performance of minor repairs on devices that have been previously dispensed to patients; and
- (2) The performance of other tasks approved by the commissioner.

SECTION 82. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-1601. Short title.

This part is known and may be cited as the "Physical Therapy Practice Act."

63-33-1602. Part definitions.

As used in this part, unless the context otherwise requires:

- (1) "Competence" is the application of knowledge, skills, and behaviors required to function effectively, safely, ethically, and legally within the context of the patient's role and environment;
- (2) "Dry needling" means a skilled intervention that uses a thin filiform needle to penetrate the skin and stimulate underlying neural, muscular, and

connective tissues for the management of neuromusculoskeletal conditions, pain, and movement impairments;

(3) "Onsite supervision" means the supervising physical therapist or physical therapist assistant:

(A) Is continuously onsite and present in the department or facility where assistive personnel are performing services;

(B) Is immediately available to assist the person being supervised in the services being performed; and

(C) Maintains continued involvement in appropriate aspects of each treatment session in which a component of treatment is delegated to assistive personnel;

(4) "Other assistive personnel":

(A) Means other trained or educated healthcare personnel not defined in subdivisions (6) and (8) who perform specific designated tasks related to physical therapy under the supervision of a physical therapist; and

(B) Includes, at the discretion of the supervising physical therapist, if properly credentialed and not prohibited by any other law, personnel identified by the title specific to their training or education;

(5) "Physical therapist" or "physiotherapist" means a person who is licensed pursuant to this part to practice physical therapy;

(6) "Physical therapist assistant" means a person who meets the requirements of this part for licensure as a physical therapist assistant and who performs physical therapy procedures and related tasks that have been selected and delegated only by the supervising physical therapist;

(7) "Physical therapy" means the care and services provided by or under the direction and supervision of a physical therapist who is licensed pursuant to this part;

(8) "Physical therapy aide," also known as an "aide," "technician," or "transporter," means a person trained by and under the direction of a physical therapist who performs designated and supervised routine physical therapy tasks;

(9) "Physical therapy assistive personnel" means physical therapy aides and other assistive personnel;

(10) "Practice of physical therapy" means, whether by in-person encounter or via telehealth, telemedicine, or provider-based telemedicine, as authorized by § 63-1-155, the following:

(A) Examining, evaluating, and testing individuals with mechanical, physiological, and developmental impairments, functional limitations and disability, or other health and movement-related conditions in order to determine a physical therapy treatment diagnosis, prognosis, and a plan of therapeutic intervention, and to assess the ongoing effect of intervention;

(B) Alleviating impairments and functional limitations by designing, implementing, and modifying therapeutic interventions that include, but are not limited to, therapeutic exercise, functional training, manual therapy, therapeutic massage, assistive and adaptive orthotic, prosthetic, protective, and supportive equipment, airway clearance techniques, debridement and wound care, physical agents or modalities, dry needling, mechanical and electrotherapeutic modalities, and patient-related instruction;

(C) Reducing the risk of injury, impairments, functional limitation, and disability, including the promotion and maintenance of fitness, health, and quality of life in all age populations; and

(D) Engaging in administration, consultation, education, and research;

(11) "Restricted physical therapist assistant license" means a license on which the commissioner has placed restrictions due to action imposed by the commissioner;

(12) "Restricted physical therapy license" means a license on which the commissioner places restrictions or conditions, or both, as to scope of practice, place of practice, supervision of practice, duration of licensed status, or type of condition of patient to whom the licensee may provide services;

(13) "Supervision" of the physical therapist assistant:

(A) Means the supervising physical therapist is readily available to the physical therapist assistant being supervised, and, when the physical therapist assistant is practicing in an offsite setting, means the supervising physical therapist is immediately accessible by telecommunications; and

(B) Includes patient conferences being regularly scheduled and documented and supervisory visits being made as further outlined in the rules; and

(14) "Upper limb" means the hand, wrist, elbow, and shoulder girdle.

63-33-1603. Scope of practice.

(a) This part does not allow physical therapists to practice medicine, osteopathic medicine, podiatry, chiropractic, or nursing.

(b)

(1) Except as provided in § 63-33-1606, the scope of practice of physical therapy is under the written or oral referral of:

(A) A licensed doctor of medicine, chiropractic, dentistry, podiatry, or osteopathic medicine; or

(B) A licensed nurse practitioner or physician assistant pursuant to § 63-6-204(b).

(2) The scope of practice of physical therapy does not include the performance of treatment where the physical therapist or physical therapist assistant uses direct thrust to move a joint of the patient's spine beyond its normal range of motion without exceeding the limits of anatomical integrity.

63-33-1604. Practice of physical therapy – Unlawful practice.

(a)

(1) A physical therapist, or physiotherapist, licensed under this part is fully authorized to practice physical therapy.

(2) A physical therapist, or physiotherapist, is not licensed under this part unless the individual holds a degree from a professional physical therapy program accredited by a national accreditation agency recognized by the United States department of education and by the board of physical therapy.

(b) A person shall not practice or in any manner claim to be engaging in the practice of physical therapy or designate as being a physical therapist unless duly licensed as a physical therapist in accordance with this part.

63-33-1605. Required referrals – Standards of ethics.

(a) A physical therapist shall refer persons under the physical therapist's care to appropriate healthcare practitioners, after consultation with the referring practitioner, if the physical therapist has reasonable cause to believe symptoms or conditions are present that require services beyond the scope of practice or when physical therapy treatment is contraindicated.

(b) Physical therapists shall adhere to the recognized standards of ethics of the physical therapy profession and as further established by rule.

63-33-1606. Conduct within scope of practice – Unprofessional conduct.

(a)

(1) The practice of physical therapy must be under the written or oral referral of a referring practitioner as described in § 63-33-1603(b)(1), except that a licensed physical therapist may:

(A) Conduct an initial patient visit without referral;

(B) Provide physical assessments or instructions, including a recommendation of exercise to an asymptomatic person, without the referral of a referring practitioner;

(C) In emergency circumstances, including minor emergencies, provide assistance to a person to the best of a physical therapist's ability without the referral of a referring practitioner. Except as provided in subdivision (a)(1)(D), the physical therapist shall refer the person to the appropriate healthcare practitioner, as indicated, immediately after providing assistance; and

(D) Treat a patient without a referral, within the scope of practice of physical therapy, when the following conditions are met:

(i) The patient's physician, as defined in § 63-6-204(f)(7), has been notified by the physical therapist;

(ii) If the physical therapist determines, based on clinical evidence, that progress has not been made with respect to that patient's condition within thirty (30) days, immediately following the date of the patient's initial visit with the physical therapist, then the physical therapist shall not provide additional physical therapy

services and shall refer the patient to a healthcare practitioner who qualifies as a referring practitioner;

(iii) The physical therapist does not continue physical therapy services for the patient beyond ninety (90) days without consulting with the patient's appropriate healthcare practitioner;

(iv) If the patient was previously diagnosed by a licensed physician with chronic, neuromuscular, or developmental conditions, and the evaluation, treatment, or services are being provided for problems or symptoms associated with one (1) or more of those previously diagnosed conditions, then subdivisions (a)(1)(D)(ii) and (iii) do not apply; and

(v) A physical therapist refers patients under the physical therapist's care to appropriate healthcare practitioners, if, at any time, the physical therapist has reasonable cause to believe symptoms or conditions are present that require services beyond the scope of practice of a physical therapist, reasonable therapeutic progress is not being achieved for the patient, or physical therapy treatment is contraindicated.

(2) As used in this subsection (a):

(A) "Emergency circumstances" means instances where emergency medical care is required;

(B) "Emergency medical care" means bona fide emergency services provided after the sudden onset of a medical condition manifesting itself by acute symptoms of sufficient severity, including severe pain, such that the absence of immediate medical attention could reasonably be expected to result in:

(i) Placing the patient's health in serious jeopardy;

(ii) Serious impairment to bodily functions; or

(iii) Serious dysfunction of a bodily organ or part; and

(C) "Notified" means to give notice to or inform a patient's physician, as defined in § 63-6-204(f)(7), when a licensed physical therapist treats a patient pursuant to subdivision (a)(1)(D).

(b) For the purposes of § 63-33-106(b)(1), it is unprofessional conduct for a physical therapist to knowingly initiate services to a patient in violation of subdivision (a)(1)(D).

63-33-1607. Minimum competency requirements.

In addition to the powers and duties granted to or imposed by this chapter, the commissioner has the authority to establish minimum competency requirements that a physical therapist must demonstrate in order to practice dry needling.

63-33-1608. Exempted persons – Dry needling by physical therapists not deemed practice of acupuncture.

(a) This part does not restrict a person licensed or certified under a law of this state from engaging in the profession or practice for which the person is licensed or certified as long as the person does not claim to be a physical therapist, a physical therapist assistant, or a provider of physical therapy.

(b) The following persons are exempt from licensure as a physical therapist or physical therapist assistant under this part:

(1) A person who is pursuing a course of study leading to a degree as a physical therapist or physical therapist assistant in a professional education program approved by the commissioner and is satisfying supervised clinical education requirements related to physical therapy education;

(2) A physical therapist or physical therapist assistant while practicing in the United States armed services, United States public health service, or

veterans administration as based on requirements under federal regulations for state licensure of healthcare providers;

(3) A physical therapist or physical therapist assistant licensed in another United States jurisdiction, or a foreign-educated or internationally trained physical therapist credentialed in another country, performing physical therapy as part of teaching or participating in an educational seminar of no more than sixty (60) days in a calendar year; and

(4) A physical therapist or physical therapist assistant licensed in another United States jurisdiction who is temporarily performing physical therapy for members of established athletic teams, athletic corporations, or performing arts companies that are training, competing, or performing in this state; however, a person performing physical therapy in this state pursuant to this subdivision (b)(4) shall agree to use the secretary of state for service of process pursuant to title 20, chapter 2, part 2.

(c) The practice of dry needling by a physical therapist licensed under this part does not constitute the practice of acupuncture under part 4 of this chapter.

(d) This part does not restrict persons licensed under other laws of this state from performing physical agent modalities for which they have received education and training.

63-33-1609. Applications – Examinations.

(a) After the application process has been completed, an applicant shall take the examination for physical therapist licensure that covers current physical therapy practice.

(b) After the application process has been completed, an applicant shall take the examination for physical therapist assistant licensure that covers current technical application of physical therapy services.

(c) Examinations must be available within the state at such time and place as the commissioner determines. The commissioner shall determine the passing score.

(d) Applicants who do not pass the examination after the first attempt may retake the examination up to six (6) times. Applications remain active for twelve (12) months. After twelve (12) months, applicants must submit a new application with all applicable fees.

63-33-1610. Documentation required from applicants.

(a) In addition to the requirements of § 63-33-103, an applicant for licensure as a physical therapist shall provide documentation in the form required by the commissioner that the applicant:

(1) Is a graduate of a professional physical therapy program accredited by a national accreditation agency recognized by the United States department of education and by the board of physical therapy; and

(2) Has successfully passed an examination approved by the commissioner.

(b) In addition to the requirements of § 63-33-103, an applicant for licensure as a physical therapy assistant shall provide documentation in the form required by the commissioner that the applicant:

(1) Is a graduate of a physical therapist assistant education program accredited by an accreditation agency approved by the board; and

(2) Has successfully passed an examination approved by the commissioner.

(c) In addition to the requirements of § 63-33-103 and this part, an applicant for licensure as a physical therapist who has been educated outside the United States, is foreign-educated, or is internationally trained shall meet the following qualifications:

(1) Provide satisfactory evidence that the applicant's education is substantially equivalent to the requirements of physical therapists educated in accredited educational programs as determined by the commissioner. If the commissioner determines that a foreign-educated applicant's education is not

substantially equivalent, the commissioner may require completion of additional coursework before proceeding with the application process;

(2) Provide written proof that the school of physical therapy education is recognized by its own ministry of education;

(3) Provide written proof of authorization to practice as a physical therapist without limitations in the country where the professional education occurred;

(4) Have the applicant's educational credentials evaluated by a board-approved credential evaluation agency;

(5) Have passed the board-approved English proficiency examinations, if the applicant's native language is not English; and

(6) Have participated in and completed an interim supervised clinical practice period prior to licensure.

(d) Notwithstanding this section, if the foreign-educated physical therapist applicant is a graduate of a professional physical therapy education program accredited by an agency approved by the commissioner, the requirements in subdivisions (c)(1), (2), (4), and (6) may be waived.

63-33-1611. Use of titles and abbreviations.

(a) A physical therapist shall use the letters "PT" or "DPT," as appropriate for the individual's education, in connection with such person's name or place of business to denote licensure under this part.

(b) It is unlawful for a person, or for a business entity, its employees, agents, or representatives, to use in connection with the person's name or the name or activity of the business the words "physical therapy," "physical therapist," "physiotherapy," "physiotherapist," "registered physical therapist," "licensed physical therapist," "doctor of physical therapy," or the letters "PT," "LPT," "DPT," or "RPT" or any other words, abbreviations, or insignia indicating or implying directly or indirectly that physical therapy

is provided or supplied, including the billing of services labeled as physical therapy, unless such services are provided by or under the direction of a physical therapist licensed in accordance with this part.

(c) A physical therapist assistant shall use the letters "PTA" in connection with the person's name to denote licensure.

(d) A person shall not use the title "physical therapist assistant" or use the letters "PTA" in connection with the person's name or any other words, abbreviations, or insignia indicating or implying, directly or indirectly, that the person is a physical therapist assistant unless the person is licensed as a physical therapist assistant in accordance with this part.

63-33-1612. Supervision of assistants, assistive personnel, and clinical students.

(a) Physical therapists are responsible for the patient care given by physical therapist assistants, assistive personnel, and students on clinical affiliations under their supervision. The physical therapist may delegate to the physical therapist assistant, assistive personnel, and clinical students selected acts, tasks, or procedures that fall within the scope of their education or training.

(b) Physical therapist assistants must at all times be under the supervision of a licensed physical therapist as set out in this part and rules promulgated by the commissioner.

(c) Physical therapy aides, other assistive personnel, and clinical students shall at all times perform patient care activities under the onsite supervision of a licensed physical therapist or physical therapist assistant.

(d) Physical therapist students and physical therapist assistant students must at all times be under the supervision of a physical therapist as set out in rules promulgated by the commissioner.

63-33-1613. Disciplinary grounds.

In addition to the disciplinary grounds listed in § 63-33-106(b), the commissioner may also exercise the powers enumerated in § 63-33-106(a) for the following:

(1) Practicing or offering to practice beyond the scope of physical therapy practice;

(2) Inadequately supervising or delegating duties that exceed the scope of practice for assistive personnel in accordance with this part and rules promulgated by the commissioner;

(3) Engaging in sexual misconduct. "Sexual misconduct," for the purpose of this section, includes:

(A) Engaging in or soliciting sexual relationships, whether consensual or nonconsensual, while a physical therapist or physical therapist assistant/patient relationship exists;

(B) Making sexual advances, requesting sexual favors, and engaging in other verbal conduct or physical conduct or physical contact of a sexual nature with patients; and

(C) Intentionally viewing a completely or partially disrobed patient in the course of treatment, if the viewing is not related to patient diagnosis or treatment under current practice standards;

(4) Directly or indirectly requesting, receiving, or participating in the dividing, transferring, assigning, rebating, or refunding of an unearned fee or profiting by means of a credit or other valuable consideration, such as an unearned commission, discount, or gratuity in connection with the furnishing of physical therapy services. This subdivision (4) does not prohibit the members of a regularly and properly organized business entity recognized by law and composed of physical therapists from dividing fees received for professional services among themselves as they determine necessary to defray their joint operating expenses. Physical therapists employed by or contracting with a

physician, physician group, as defined in accordance with the federal Physician Self-Referral Law (42 U.S.C. § 1395nn), or entity primarily owned by physicians and receiving wages, other compensation, or benefits pursuant to the employment or contract are not deemed to be in violation of this part or part 1 of this chapter solely by virtue of the employment or contract and are not subject to licensure denial, suspension, revocation, or other disciplinary action or penalty described under this part or part 1 of this chapter solely by virtue of the employment or contract. This subdivision (4) does not create a prohibition on the corporate practice of a healthcare professional where no such prohibition previously existed;

(5) Charging unreasonable or fraudulent fees for services performed or not performed;

(6) Making misleading, deceptive, untrue, or fraudulent representations in violation of this part, or otherwise, in practice of the profession;

(7) Aiding or abetting a person not licensed in this state who directly or indirectly performs activities requiring a license;

(8) Failing to report to the board an act or omission of a licensee, applicant, or other person who violates this part;

(9) Interfering with, or refusing to cooperate in, an investigation or disciplinary proceeding, including willful misrepresentation of facts or by the use of threats or harassment against a patient or witness to prevent the patient or witness from providing evidence in a disciplinary proceeding or a legal action;

(10) Failing to maintain adequate patient records that contain a minimum of an evaluation of objective finding; a physical therapy treatment diagnosis; the plan of care, including desired outcomes; the treatment record; a discharge plan, including results of intervention; and sufficient information to identify the patient;

(11) Promoting unnecessary devices, treatment intervention, or service for the financial gain of the practitioner or of a third party;

(12) Providing treatment intervention unwarranted by the condition of the patient, or continuing treatment beyond the point of reasonable benefit;

(13) Dividing fees or agreeing to split or divide fees received for professional services with a person for bringing or referring a patient outside the scope of § 63-33-1614; and

(14) Paying or accepting commissions, in any form or manner, on fees for professional services, references, consultations, pathological reports, prescriptions, or other services or articles supplied to patients.

63-33-1614. Required disclosures to patients – Complaints – Display of license required.

(a) Physical therapists shall inform the patient of any financial arrangements connected to the referral process.

(b) Physical therapists shall disclose in writing any financial interest in products they endorse and recommend to their patients.

(c) The physical therapist has the responsibility to ensure that the patient has knowledge of freedom of choice in services and products.

(d) Information relating to the physical therapist/patient relationship is confidential and must not be communicated to a third party not involved in that patient's care without the prior written consent of the patient. The physical therapist/patient confidentiality does not extend to cases in which the physical therapist has a duty to report information as required by law.

(e) A person may submit a complaint regarding a licensee or other person potentially in violation of this part or part 1 of this chapter. Confidentiality must be maintained subject to law.

(f) The division shall keep all information relating to the receiving and investigation of complaints filed against licensees confidential until the information becomes public record as required by law.

(g) Each licensee shall display a copy of the licensee's license or current renewal verification in a location accessible to public view at the licensee's place of employment.

SECTION 83. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-1701. Short title.

This part is known and may be cited as the "Occupational Therapy Practice Act."

63-33-1702. Part definitions.

(a) As used in this part, unless the context otherwise requires:

(1) "ACOTE" means the Accreditation Council for Occupational Therapy Education, a nationally recognized accrediting agency for professional programs in the field of occupational therapy;

(2) "AOTA" means the American Occupational Therapy Association;

(3) "Competence" is the application of knowledge, skills, and behaviors required to function effectively, safely, ethically, and legally within the context of the patient's role and environment;

(4) "Dry needling" means a skilled intervention that uses a thin filiform needle to penetrate the skin and stimulate underlying neural, muscular, and connective tissues for the management of neuromusculoskeletal conditions, pain, and movement impairments;

(5) "NBCOT" means the National Board for Certification in Occupational Therapy;

(6) "Occupational therapist" means a person licensed to engage in occupational therapy practice under this part;

(7) "Occupational therapy assistant" means a person licensed to assist in occupational therapy practice under the supervision of an occupational therapist;

(8) "Occupational therapy practice":

(A) Means the therapeutic use of everyday life activities, including occupations, for the purpose of enabling individuals or groups to participate in roles and situations in home, school, workplace, community, and other settings. Occupational therapy addresses the physical, cognitive, psychosocial, and sensory aspects of performance in a variety of contexts to support engagement in occupations that affect health, well-being, and quality of life; and

(B) Includes:

(i) Screening, evaluation, assessment, planning, implementation, or discharge planning in order to determine an occupational therapy treatment diagnosis, prognosis, plan of therapeutic intervention, or discharge plan, or to assess the ongoing effect of intervention;

(ii) Selection and administration of standardized and nonstandardized tests and measurements to evaluate factors affecting activities of daily living, instrumental activities of daily living, education, work, play, leisure, and social participation, including:

(a) Body functions and body structures;

(b) Habits, routines, roles, and behavior patterns;

(c) Cultural, physical, environmental, social, and spiritual context and activity demands that affect performance; and

(d) Performance skills, including motor, process, and communication and interaction skills;

(iii) Methods or strategies selected to direct the process of interventions, such as:

(a) Modification or adaptation of an activity or the environment to enhance performance;

(b) Establishment, remediation, or restoration of a skill or ability that has not yet developed or is impaired;

(c) Maintenance and enhancement of capabilities without which performance in occupations would decline;

(d) Health promotion and wellness to enable or enhance performance and safety of occupations; and

(e) Prevention of barriers to performance, including disability prevention;

(iv) Interventions and procedures to promote or enhance safety and performance in activities of daily living, instrumental activities of daily living, education, work, play, leisure, and social participation, including:

(a) Therapeutic use of occupations, exercises, and activities;

(b) Training in self-care, self-management, home management, and community and work reintegration;

(c) Development, remediation, or compensation of physical, cognitive, neuromuscular, and sensory functions and behavioral skills;

- (d) Therapeutic use of self, including an individual's personality, insights, perceptions, and judgments as part of the therapeutic process;
- (e) Education and training of individuals, family members, caregivers, and others;
- (f) Care coordination, case management, discharge planning, and transition services;
- (g) Consulting services to groups, programs, organizations, or communities;
- (h) Assessment, recommendations, and training in techniques and equipment to enhance functional mobility, including wheelchair management;
- (i) Driver rehabilitation and community mobility;
- and
- (j) Management of feeding and eating skills to enable feeding and eating performance;
- (v) Management of occupational therapy services, including the planning, organizing, staffing, coordinating, directing, or controlling of individuals and organizations;
- (vi) Providing instruction in occupational therapy to students in an accredited occupational therapy or occupational therapy assistant educational program by persons who are trained as occupational therapists or occupational therapy assistants; and
- (vii) Administration, interpretation, and application of research to occupational therapy services; and
- (C) Includes specialized services provided by occupational therapists or occupational therapy assistants who are certified or trained

in areas of specialization that include, but are not limited to, hand therapy, neurodevelopmental treatment, dry needling of the upper limb, sensory integration, pediatrics, geriatrics, and neurorehabilitation, through programs approved by AOTA or other nationally recognized organizations;

(9) "Occupations":

(A) Means everyday life activities, named, organized, and given value and meaning by individuals and their culture; and

(B) Includes everything that people do to occupy their time, including caring for their needs, enjoying life, and contributing to the social and economic fabric of their communities;

(10) "Unlicensed person," when working in occupational therapy, means a person, often referred to as an aide, technician, transporter, or support staff, who performs specific supportive tasks related to occupational therapy practice under the direct supervision of an occupational therapist or an occupational therapy assistant and whose activities do not require professional or advanced training in the basic anatomical, biological, psychological, and social sciences involved in the provision of occupational therapy services; and

(11) "Upper limb" means the hand, wrist, elbow, and shoulder girdle.

(b) Occupational therapy services must be provided for the purpose of promoting health and wellness to those clients who have, or are at risk of developing, illness, injury, disease, disorder, impairment, disability, activity limitation, or participation restriction and may include:

(1) Training in the use of prosthetic devices;

(2) Assessment, design, development, fabrication, adaptation, application, fitting, and training in the use of assistive technology and adaptive and selective orthotic devices;

(3) Application of physical agent modalities with proper training and certification;

(4) Assessment and application of ergonomic principles;

(5) Adaptation or modification of environments, at home, work, school, or community, and use of a range of therapeutic procedures, such as wound care management, techniques to enhance sensory, perceptual, and cognitive processing and manual therapy techniques, to enhance performance skills, occupational performance, or the promotion of health and wellness; and

(6) Practice of dry needling of the upper limb, with proper training and certification.

(c) Occupational therapy practice may occur in a variety of settings, including, but not limited to:

(1) Institutional inpatient settings, such as acute rehabilitation facilities, psychiatric hospitals, community and specialty hospitals, nursing facilities, and prisons;

(2) Outpatient settings, such as clinics, medical offices, and therapist offices;

(3) Home and community settings, such as homes, group homes, assisted living facilities, schools, early intervention centers, daycare centers, industrial and business facilities, hospices, sheltered workshops, wellness and fitness centers, and community mental health facilities;

(4) Research facilities;

(5) Educational institutions; and

(6) Telehealth, telemedicine, or provider-based telemedicine, as authorized by § 63-1-155.

63-33-1703. Applicant requirements – Examinations.

In addition to the requirements of § 63-33-103, an applicant for licensure as an occupational therapist or as an occupational therapy assistant must:

(1) Have successfully completed the academic requirements of an educational program accredited by ACOTE or its predecessor organization. The accredited program must be a program for occupational therapists or a program for occupational therapy assistants, depending upon the category of licensure for which the applicant is applying;

(2) Have successfully completed the period of supervised fieldwork experience required by ACOTE; and

(3) Have made an acceptable score on a written or computerized examination designed to test the applicant's knowledge of the basic and clinical services related to occupational therapy, occupational therapy techniques and methods, and other subjects that may help to determine an applicant's fitness to practice. The commissioner shall approve an examination for occupational therapists and an examination for occupational therapy assistants and establish standards for acceptable performance on each examination. The commissioner may use the entry level national examinations prepared and administered by NBCOT as the examinations used to test applicants for licensure as occupational therapists or occupational therapy assistants and may use the standards of NBCOT in determining an acceptable score on each examination.

63-33-1704. Limited permit.

(a) A limited permit may be issued by the commissioner to an applicant who has applied for a license under this part and part 1 of this chapter, has successfully completed the educational and field experience requirements of § 63-33-1703(1) and (2), and is scheduled to take the examination required by § 63-33-1703(3).

(b) An applicant who has received a limited permit shall take the examination within ninety (90) days of the date the applicant received the limited permit. If the

applicant does not take the examination within that ninety-day period, then the limited permit expires at the end of the ninety-day period.

(c) If an applicant fails the examination, then the applicant's limited permit expires upon the division's receipt of notice that the applicant failed the examination.

(d) If an applicant passes the examination, then the applicant's limited permit remains effective until the commissioner grants or denies a license to the applicant.

(e) An applicant may obtain a limited permit only once.

(f) A limited permit allows an applicant to engage in occupational therapy practice under the supervision of a licensed occupational therapist.

(g) The commissioner shall adopt rules in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5, governing the supervision of persons to whom a limited permit has been issued. The rules must address, at a minimum, initial and periodic inspections, written evaluations, written treatment plans, patient notes, and periodic evaluation of performance.

63-33-1705. Supervision of licensed occupation therapy assistants required.

(a) A licensed occupational therapy assistant shall practice under the supervision of an occupational therapist who is licensed in this state.

(b) The supervising occupational therapist is responsible for all services provided by the occupational therapy assistant, including, but not limited to, the formulation and implementation of a plan of occupational therapy services for each client, and has a continuing responsibility to follow the progress of each client and to ensure the effective and appropriate supervision of the occupational therapy assistant according to the needs of the client.

(c) The supervising occupational therapist shall assign to the occupational therapy assistant only those duties and responsibilities that the occupational therapy assistant is qualified to perform.

(d) The commissioner shall adopt rules in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5, governing the supervision of occupational therapy assistants by occupational therapists. Those rules may address the following:

(1) The manner in which the supervising occupational therapist oversees the work of the occupational therapy assistant;

(2) The ratio of occupational therapists to occupational therapy assistants required under different conditions and in different practice settings; and

(3) The documentation of supervision contacts between the supervising occupational therapist and the occupational therapy assistant.

(e) The rules adopted by the commissioner must recognize that the frequency, methods, and content of supervision of occupational therapy assistants by occupational therapists may vary by practice setting and are dependent upon the following factors, among others:

(1) Complexity of the client's needs;

(2) Number and diversity of clients;

(3) Skills of the occupational therapy assistant and the supervising occupational therapist;

(4) Type of practice setting; and

(5) Requirements of the practice setting.

63-33-1706. Authorized duties of unlicensed persons.

(a) A licensed physician, occupational therapist, or licensed occupational therapy assistant may delegate to an unlicensed person specific routine tasks associated with nontreatment aspects of occupational therapy practice that are not evaluative, assessive, or task selective in nature and do not require making recommendations, making decisions, or making assessment or treatment entries in official patient records, if the following conditions are met:

(1) The physician, occupational therapist, or occupational therapy assistant accepts professional responsibility for the performance of that duty by the person to whom it is delegated. In the case of duties delegated by an occupational therapy assistant, the occupational therapy assistant, the physician, and the occupational therapist who supervises the occupational therapy assistant are responsible;

(2) The unlicensed person does not perform duties that require licensure under this part and part 1 of this chapter; and

(3) The physician, occupational therapist, or occupational therapy assistant ensures that the unlicensed person has been appropriately trained for the performance of the tasks.

(b) Tasks that may be delegated include:

(1) Transporting of patients;

(2) Preparing or setting up a work area or equipment;

(3) Routine department maintenance or housekeeping activities;

(4) Taking care of a patient's personal needs during treatments; and

(5) Clerical, secretarial, or administrative duties.

63-33-1707. Exempted persons – Dry needling by licensed occupational therapists not deemed acupuncture.

(a) This part and part 1 of this chapter do not prevent or restrict the practice, services, or activities of:

(1) A person licensed under this title who is authorized to provide therapeutic services as listed under this part;

(2) A person employed as an occupational therapist or occupational therapy assistant by an agency of the United States government while providing occupational therapy services in that capacity;

(3) A person pursuing a course of study leading to a degree or certificate in occupational therapy in an educational program accredited or granted developing program status by ACOTE, if:

(A) The activities and services constitute a part of a supervised course of study; and

(B) The person is designated by a title that clearly indicates the person's status as a student;

(4) A person fulfilling the supervised fieldwork experience requirements of § 63-33-1703(2), if the activities and services constitute a part of the experience necessary to meet the requirements of § 63-33-1703(2);

(5) An occupational therapist or occupational therapy assistant who is licensed or certified to practice in another state that has licensure or certification requirements at least as stringent as the requirements of this part while in this state for the purpose of providing educational, consulting, or training services for no more than fourteen (14) days in a calendar year; and

(6) A person who is certified by the National Council for Therapeutic Recreation Certification as a certified therapeutic recreation specialist, while engaged in practicing the profession of recreation therapy.

(b) This part and part 1 of this chapter do not prevent certified orthotists from designing, fabricating, and fitting orthotic devices.

(c) This part and part 1 of this chapter do not prevent a person employed by a physician from performing activities related to casting and splinting or teaching exercises related to specific treatment by the employing physician.

(d) The practice of dry needling of the upper limb by an occupational therapist licensed under this part and part 1 of this chapter does not constitute the practice of acupuncture under part 4 of this chapter.

63-33-1708. Disciplinary grounds.

In addition to the disciplinary grounds listed in § 63-33-106(b), the commissioner may also exercise the powers enumerated in § 63-33-106(a) for the following:

- (1) Making or signing in one's professional capacity a certificate that is known to be false at the time one makes or signs the certificate;
- (2) Soliciting by agents or persons generally known as "cappers" or "steerers" of professional patronage or profiting by the acts of those representing themselves to be agents of the licensee;
- (3) Dividing fees or agreeing to split or divide fees received for professional services with a person for bringing or referring a patient;
- (4) Conducting practice so as to permit, directly or indirectly, an unlicensed person to perform services or work that, under this part, can be done legally only by persons licensed to practice;
- (5) Professionally connecting or associating with a person, firm, or corporation, in any manner, in an effort to avoid and circumvent this part or lending one's name to another for illegal practice;
- (6) Paying or accepting commissions, in any form or manner, on fees for professional services, references, consultations, pathological reports, prescriptions, or other services or articles supplied to patients;
- (7) Giving of testimonials, directly or indirectly, concerning the supposed virtue of secret therapeutic agents or proprietary preparations, such as remedies, or other articles or materials that are offered to the public, claiming radical cure or prevention of diseases by their use; and
- (8) Using or participating in the use of any form of public communication containing a false, fraudulent, misleading, or deceptive statement or claim on behalf of the licensee or the licensee's partner, associate, or any other person affiliated with the licensee or the licensee's facility.

63-33-1709. Violations – Use of titles – Unlicensed practice.

(a) It is unlawful for a person to violate this part.

(b) It is unlawful for a person who is not licensed under this part as an occupational therapist or an occupational therapy assistant or whose license has been suspended or revoked to use, in connection with the person's name or place of business, the words "occupational therapist," "occupational therapist registered," "licensed occupational therapist," "occupational therapy assistant," "certified occupational therapy assistant" or "licensed occupational therapy assistant," the letters "OT," "OTA," "OTR," "OT/L," "OTA/L," "COTA," or any other words, letters, abbreviations, or insignia indicating or implying that the person is an occupational therapist or an occupational therapy assistant or who in any way, orally, in writing, in print or by sign, directly or by implication, claims to be an occupational therapist or an occupational therapy assistant.

(c) A violation of this part constitutes unlicensed practice as set forth in § 63-33-107.

63-33-1710. Requirements of applicants educated outside the United States.

Applicants for licensure who have completed educational programs outside the United States shall furnish proof of good moral character and of completion of an educational program and supervised fieldwork substantially similar to those contained in § 63-33-1703(1) and (2) and satisfy the examination requirements of § 63-33-1703(3).

63-33-1711. Requirements of an occupational therapist for dry needling.

In addition to the powers and duties granted to or imposed by this chapter, the commissioner has the authority to establish minimum competency requirements that an occupational therapist must demonstrate in order to practice dry needling of the upper limb.

SECTION 84. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-1801. Part definitions.

As used in this part, unless the context otherwise requires:

(1) "Advertise" includes the issuing or causing to be distributed a card, sign, or device to a person, or the causing, permitting, or allowing a sign or marking on or in a building or structure, or in a newspaper or magazine, in a directory, or on radio or television or by advertising by any other means designed to secure public attention;

(2) "Appraisal activities":

(A) Means selecting, administering, scoring, and interpreting instruments designed to assess an individual's aptitudes, achievements, or interests, which are used to understand, measure, or facilitate the individual's normal human growth and development, but does not include the use of projective techniques in the assessment of personality, nor the use of psychological or clinical tests designed to identify or classify abnormal or pathological human behavior, nor the use of individually administered intelligence tests; and

(B) Includes with regard to licensed professional counselors, consistent with each counselor's formal education and training, administering and utilizing appropriate assessment instruments that measure or diagnose problems or dysfunctions within the context of human growth and development as part of the counseling process or in the development of a treatment plan;

(3) "Approved supervisor" means a licensed marital and family therapist, psychologist, or psychiatrist who is either an American Association for Marriage and Family Therapy-approved supervisor or a commissioner-approved marriage and family supervisor;

(4) "Commissioner-approved marriage and family supervisor" means a person who gives to the commissioner evidence of:

(A) Five (5) years of full-time experience in marriage and family therapy practice and supervision;

(B) Thirty-six (36) hours of supervision specifically in the skill of providing marriage and family therapy supervision; and

(C) A recommendation for commissioner-approved supervisor status from a supervisor who provided supervision for the supervision referred to in subdivision (a)(4)(B);

(5) "Counseling" means assisting an individual, through the counseling relationship, in a manner intended to facilitate normal human growth and development, using a combination of mental health and human development principles, methods, and techniques, to achieve mental, emotional, physical, social, moral, educational, spiritual, or career development and adjustment throughout the life span;

(6) "Licensed marital and family therapist" means a person to whom a license has been issued pursuant to this part, which license is in force and not suspended or revoked as of the particular time in question;

(7) "Licensed professional counselor" means a person licensed under this part who is professionally trained in counseling and guidance services designed to facilitate normal human growth and development through individual, family, or group counseling, educational procedures, assessment, consultation, and research and who assists individuals by the practice of counseling with their personal, social, career, or educational development as they pass through life stages;

(8) "Marital and family therapy" means the diagnosis and treatment of cognitive, affective, and behavioral problems and dysfunctions within the context of marital and family systems, and involves the professional application of psychotherapeutic family systems theories and techniques in the delivery of

services to individuals in the context of family systems theory and practice, couples, and families;

(9) "Practice of counseling":

(A) Means rendering or offering to render to individuals, groups, organizations, or the general public a service involving the application of principles, techniques, methods, or procedures of the counseling profession, including appraisal activities, counseling, consulting, and referral activities; and

(B) Does not include permitting the treatment of a mental, emotional, or adjustment disorder other than marital problems, parent-child problems, child and adolescent antisocial behavior, adult antisocial behavior, other specified family circumstances, other interpersonal problems, phase of life problems, other life circumstance problems, occupational problems, and uncomplicated bereavement, except as provided in subdivision (10);

(10) "Practice of counseling as a mental health service provider":

(A) Means the application of mental health and human development principles in order to:

(i) Facilitate human development and adjustment throughout the life span;

(ii) Prevent, diagnose, and treat mental, emotional, or behavioral disorders and associated disorders that interfere with mental health;

(iii) Conduct assessments and diagnoses for the purpose of establishing treatment goals and objectives within the limitations prescribed in subdivision (2); and

(iv) Plan, implement, and evaluate treatment plans using counseling treatment interventions. As used in this subdivision

(10)(A)(iv), "counseling treatment interventions" means the application of cognitive, affective, behavioral, and systemic counseling strategies that include principles of development, wellness, and pathology that reflect a pluralistic society; and

(B) Does not include the performance of an act that licensed professional counselors designated as mental health service providers are not educated and trained to perform or the designation of testing reports as "psychological";

(11) "Practice of marital and family therapy" means the rendering of professional marital and family therapy to individuals, couples, and family groups, singly or in groups, whether such services are offered directly to the general public or through organizations, either public or private, for a fee;

(12) "Recognized educational institution" means an educational institution that is recognized by the commissioner and by a nationally or regionally recognized educational or professional accrediting body;

(13) "Supervision" means the direct clinical review, for the purpose of training or teaching, by an approved supervisor of a marriage and family therapist's interaction with clients, where the purpose of supervision is to promote the development of the practitioner's clinical skills; such supervision may include, without being limited to, the review of case presentations, audiotapes, videotapes, and direct observation; and

(14) "Use a title or description of" means to hold oneself out to the public as having a particular status by means of stating such status on signs, mailboxes, address plates, stationery, announcements, business cards, or other instruments of professional identification.

63-33-1802. Reciprocity for marital and family therapist applicants.

(a) The commissioner shall license without examination a marital and family therapist applicant who is licensed to practice independently in another state if the applicant's qualifications meet the licensure requirements under this part and part 1 of this chapter.

(b) The board may enter into a reciprocal agreement with any other state that licenses marital and family therapists.

63-33-1803. Required documentation from professional counselor applicants.

In addition to the requirements of § 63-33-103, an applicant for licensure as a professional counselor must provide documentation in the form required by the commissioner that the applicant:

(1)

(A) Has a conferred graduate degree in counseling or a closely related field;

(B) Has obtained a total of sixty (60) graduate hours in counseling or a closely related field; and

(C) Has completed a supervised field experience as either a practicum or internship;

(2) Has had at least two (2) years of professional experience of a type judged to be acceptable by the commissioner subsequent to being granted a graduate degree and has not violated § 63-33-106 or § 63-33-1805;

(3) Has passed the examination offered by the National Board for Certified Counselors (NBCC) or such other examination approved by the commissioner;

(4) Has passed the jurisprudence exam in this state; and

(5) Has met additional criteria of the commissioner established by rule.

63-33-1804. Required documentation from marital and family therapist applicants.

(a) In addition to the requirements of § 63-33-103, an applicant for licensure as a marital and family therapist must provide documentation in the form required by the commissioner that the applicant:

(1) Has met standards set by the commissioner no less stringent than the American Association for Marriage and Family Therapy's standards for a clinical member so long as such standards specify a minimum of a master's level degree; and

(2) Has passed such examination as may be required by the commissioner.

(b) Applicants shall bear the fee for testing in addition to the review fee and application fee.

63-33-1805. Disciplinary grounds.

In addition to the disciplinary grounds listed in § 63-33-106(b), the commissioner may also exercise the powers enumerated in § 63-33-106(a) for the following:

(1) Using fraud or deception in taking an examination required by this part; or

(2) Violating the rules adopted by the commissioner, except that a violation of A.11.b of the 2014 American Counseling Association Code of Ethics, or any similar or successor provision, is not grounds upon which the commissioner shall exercise the commissioner's powers to discipline a license; however, a violation of A.11.b is a ground for the commissioner to exercise these powers in cases involving an individual seeking or undergoing counseling where the individual was in imminent danger of harming themselves or others.

63-33-1806. Exempted persons.

(a) This part and part 1 of this chapter do not apply to a person if the person is preparing for the practice of marital and family therapy under qualified supervision in a training institution or facility or supervisory arrangement recognized and approved by the

commissioner as long as the person is designated by such titles as "marital therapy intern," "family therapy trainee," or others clearly indicating such training status.

(b) This part and part 1 of this chapter do not prevent:

(1) Qualified members of other professional groups as defined by the commissioner by rule, including, but not limited to, licensed clinical social workers, licensed psychologists, licensed psychological examiners, licensed senior psychological examiners, certified psychological assistants, psychiatric nurses, physicians, attorneys at law, or members of the clergy from doing or advertising that they perform the work of a marital and family therapy nature consistent with the accepted standards of their respective professions; and

(2) Alcohol and drug abuse counselors licensed under chapter 11, part 4 of this title or operating under qualified supervision while seeking such certification from doing counseling consistent with the accepted standards of that profession.

63-33-1807. Confidential relations and communications.

The confidential relations and communications between licensed marital and family therapists, licensed professional counselors, or certified pastoral therapists and clients are placed upon the same basis as those provided by law between attorney and client. This part does not require such privileged communication to be disclosed.

However, this section and part 1 of this chapter do not prevent disclosures of confidential communications in proceedings arising under title 37, chapter 1, part 4 concerning mandatory child abuse reports.

63-33-1808. Prohibited conduct by unlicensed persons – Exemptions.

(a) Except as provided in § 63-33-1806, a person who is not licensed or certified as a marital and family therapist under this part shall not:

(1) Advertise the performance of marital and family therapy or counseling service by the person; or

(2) Use a title or description, including, but not limited to, "licensed," "certified," "marriage," "marital," "therapist," "counselor," "advisor," or "consultant," or any other name, style, or description denoting that the person is a marital and family therapist or practices marital and family therapy.

(b) A person who engages in an unlawful act enumerated in this section commits a Class B misdemeanor.

(c) This part does not permit a person licensed or certified as a marital and family therapist to engage in the practice of "licensed psychological examiner," "licensed senior psychological examiner," "certified psychological assistant," "licensed psychologist," or "licensed social worker," as defined in the laws of this state.

(d) This part, except subsection (a), does not apply to a person regulated by the board of nursing.

(e) This part does not permit a person certified or licensed as a marital and family therapist to perform psychological testing intended to measure or diagnose mental illness. Consistent with each therapist's formal education and training, licensed marital and family therapists may administer and utilize appropriate assessment instruments that measure or diagnose cognitive, affective, and behavioral problems and dysfunctions of individuals in the context of marital and family systems, couples, and families as part of the therapy process or in the development of a treatment plan.

63-33-1809. Reciprocity for professional counselors.

(a)

(1) In accordance with § 63-33-104, the commissioner may issue a license to a professional counselor applicant with a valid, unencumbered license issued by another state, territory, or federal district of the United States at an equivalent level of licensure under the following conditions:

(A) The applicant has a sixty-hour graduate degree in counseling or a related field, meets the requirements of § 63-33-1803(2)-(5), and has

actively practiced without supervision at the level of licensure for which the applicant has applied for at least two (2) of the three (3) most recent years; or

(B) The applicant has a conferred graduate degree consisting of a minimum of fifty-two (52) hours in counseling or a related field from an accredited program that were earned prior to 2016, has practiced as a licensed professional counselor for a minimum of twenty-five (25) years, and meets the requirements of § 63-33-1803(2)-(5).

(2) A professional counselor applicant at the highest level of licensure must meet the requirements of § 63-33-1813, as well as the requirements of either subdivision (a)(1)(A) or (a)(1)(B).

(b) The board may enter into a reciprocal agreement with any other state that licenses, certifies, or registers professional counselors, if the board finds that such state has substantially the same or higher licensure requirements than this state. The agreement must provide that the board shall license a resident of another state who is currently licensed, certified, or registered by that state, if the resident has met the same or higher requirements as provided for in this part.

(c) If the commissioner has entered into a reciprocal agreement with another state in accordance with § 63-33-104(b) on or before January 1, 2024, then the commissioner must grant a license to a person who:

(1) Has a master's degree in counseling or education;

(2) Has a valid, unencumbered license in the state with which this state has a reciprocal agreement;

(3) Has actively practiced for at least the preceding twenty-four (24) months in the state with which this state has such reciprocal agreement; and

(4) Has passed the examination offered by the National Board for Certified Counselors (NBCC) or such other examination approved by the commissioner.

63-33-1810. Unlawful use of titles – Exempt persons.

(a)

(1) Except as specifically provided elsewhere in this part, commencing July 1, 2026, it is an offense for a person to engage in the following acts:

(A) Representing or advertising the person as, or using a title or description such as, "licensed professional counselor" without being duly licensed according to this part;

(B) Making use of a title, words, letters, or abbreviations, or any combination thereof, that may reasonably be confused with licensure provided by this part to denote a standard of professional or occupational competence, without being duly licensed under this part; or

(C) Performing activities that may reasonably be construed to fall within the defined scope of practice of persons licensed under this part who are designated as mental health service providers as defined in § 63-33-1813 without being duly licensed under this part.

(2) A person who engages in an unlawful act enumerated in this section commits a Class B misdemeanor.

(3) In addition to subdivisions (a)(1) and (2), the department of health may institute appropriate proceedings, in law or equity, to enjoin a person from engaging in an unlawful act enumerated in this section, such action or proceeding to be brought in the circuit or chancery court of the county in which the unlawful act occurs or in which the defendant resides.

(b) This part does not permit a person licensed or certified as a professional counselor or designated as a mental health service provider under this part and part 1 of

this chapter to engage in the practice of licensed psychological examiner, licensed senior psychological examiner, certified psychological assistant, licensed psychologist, licensed social worker, psychiatric nurse, law, or medicine.

(c) This part does not apply to a person if the person is preparing for the practice of professional counseling or the practice permitted to a person designated as a mental health service provider pursuant to this chapter under qualified supervision.

(d) This part does not:

(1) Limit the ministry, activities, or services of a rabbi, priest, minister of the gospel, or others authorized by a regularly organized and functioning religious body in performing the ordinary duties or functions of the clergy;

(2) Apply to or limit the activities or services of Christian Science practitioners; or

(3) Limit a rabbi, priest, or minister who offers counseling services, even if fees are charged, as long as they do not hold themselves out as certified or licensed professional counselors.

63-33-1811. Exempted professionals.

(a) Except § 63-33-1810, this part does not apply to:

(1) Persons regulated by the board of nursing;

(2) Persons regulated by the state board of education; and

(3) Professional counselors employed in community or human services agencies and working directly under the supervision of a licensed professional counselor.

(b) This part does not prevent members of other professional groups, including, without limitation, licensed social workers, psychologists, psychiatrists, and other physicians, attorneys, or members of the clergy, from performing or advertising their respective professions as long as such persons do not violate § 63-33-1810.

63-33-1812. Application of chapter 11 of this title.

Notwithstanding this part to the contrary, a person licensed pursuant to this part and chapter 11 of this title is subject to the limitations of chapter 11 of this title.

63-33-1813. Requirements of mental health service providers.

A professional counselor licensed under this part and designated as a mental health service provider must have:

- (1) Completed coursework specifically related to the diagnosis, treatment, appraisal, and assessment of mental disorders; and
- (2) Completed two (2) years of postgraduate supervised experience required for licensure in a clinical setting that provides substantial opportunities to diagnose, treat, appraise, and assess mental disorders.

63-33-1814. Temporary licenses.

(a)

(1) A temporary license may be issued by the commissioner for a licensed professional counselor designated as a mental health service provider applicant who has completed the academic coursework and training required for the license sought and who has successfully passed the examination required by the commissioner.

(2) A temporary license obtained pursuant to this section authorizes an applicant to perform the functions constituting the practice of counseling as a mental health service provider, for which the applicant is seeking licensure under qualified supervision.

(3) In order to receive a temporary license, an applicant must submit to the division a completed application for a temporary license with the applicant's completed application for the desired license and all appropriate fees.

(4) If an applicant is granted a temporary license, then the license must remain valid for up to four (4) years and is eligible for extension at the discretion of the commissioner.

(5) The applicant shall notify the division and present supporting documentation demonstrating the satisfactory completion of the required postgraduate supervised experience in a clinical setting. The commissioner shall then grant or deny the license application based on satisfactory completion of all requirements for licensure.

(b)

(1) A temporary license may be issued by the commissioner to a marital and family therapist applicant who has completed the academic coursework and training required for the license sought. In order to retain the temporary license, the applicant must take the written examination required by the commissioner the first time it is scheduled following issuance of the temporary license. The applicant shall successfully pass the exam within two (2) years following issuance of the temporary license.

(2) A temporary license obtained pursuant to this section authorizes the applicant to engage in the practice of marital and family therapy under the supervision of an approved supervisor.

(3) In order to receive a temporary license, the applicant must submit to the division the following:

(A) A completed application for a temporary license;

(B) Satisfactory evidence of an agreement with an approved supervisor under whose supervision the applicant intends to practice; and

(C) All appropriate fees.

(4) A temporary license is nonrenewable and valid for a period of no more than three (3) years. Within such three-year period, the applicant shall submit to the division an application for the regular license and present supporting documentation demonstrating the satisfactory completion of the required amount of post-master's degree supervised experience in a clinical

setting under an approved supervisor. The commissioner shall then grant or deny the application for the regular license, based on satisfactory completion of all requirements for licensure, including the oral examination. If the commissioner approves or denies the application for the regular license or revokes the temporary license for any reason, then the temporary license ceases to be valid and must be returned to the division.

(5) The holder of a temporary license as a marital and family therapist shall not represent the temporary licensee to be a licensed marital and family therapist. The holder of the temporary license shall only represent the temporary licensee to be a "marital therapy intern," a "family therapy trainee," or such other title designation that clearly reflects trainee status and temporary licensure.

SECTION 85. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-1901. Part definitions.

As used in this part:

(1) "Advertise" includes:

(A) Business solicitations, with or without limiting qualifications, in a card, sign, or device issued to a person, in a sign or marking in or on a building or in a newspaper, magazine, directory, or other printed matter; and

(B) Business solicitations communicated by individual, radio, video or television broadcasting, or other means designed to secure public attention;

(2) "Certified clinical pastoral therapist" means a person who has met the qualifications for certified clinical pastoral therapist and holds a current, unsuspended or unrevoked certificate that has been lawfully issued by the commissioner;

(3) "Clinical pastoral education" means a program of training, typically occurring in general medical, psychiatric, or penal institutions, that is designed to acquaint students of theology and practicing clergy with the clinical method of learning, increase skills in the arts of pastoral care, and facilitate integration of a professional pastoral identity;

(4) "Clinical pastoral therapy" means the diagnosis and treatment, from a clinical pastoral perspective, of the psychodynamics, interpersonal dynamics, and spiritual dynamics of persons experiencing emotional behavioral or relational distress or dysfunction, and involves the integration and professional application of resources and techniques from the religious community's traditions of pastoral care and counsel along with recognized principles, methods, and procedures of the contemporary psychotherapy community in the delivery of counseling and psychotherapeutic services to individuals, couples, families, and groups;

(5) "Licensed clinical pastoral therapist" means a person who has met the qualifications for a licensed clinical pastoral therapist and who holds a current, unsuspended or unrevoked license that has been issued lawfully by the commissioner;

(6) "Practice of clinical pastoral therapy" means the rendering of professional clinical pastoral therapy to individuals, couples, families, or groups, either offered directly to the general public by an individual operating independently of an institution, organization, or agency, through mental health clinics or agencies, whether public or private, or through hospitals, whether public or private, for a fee, excluding volunteer hours;

(7) "Recognized educational institution" means an educational institution that is accredited by a nationally or regionally recognized educational accrediting body;

(8) "Supervision":

(A) Means the direct clinical review, for the purpose of training or teaching, by a commissioner-approved supervisor, of a clinical pastoral therapist's interaction with clients and promoting the development of the practitioner's clinical skills; and

(B) Includes the review of case presentations, audiotapes, videotapes, and direct observation; and

(9) "Use the title" means to hold oneself out to the public as having a particular status by means of stating such status on signs, mailboxes, address plates, stationery, announcements, business cards, or other instruments of professional identification.

63-33-1902. Prohibited conduct by unlicensed persons – Exemptions.

(a) Except as specifically provided in § 63-33-1904, a person who is not licensed as a clinical pastoral therapist under this part shall not:

(1) Advertise that the performance of clinical pastoral therapy services is by a licensed clinical pastoral therapist; or

(2) Use the title "licensed clinical pastoral therapist" to denote that the person is a licensed clinical pastoral therapist.

(b) A person who engages in an unlawful act enumerated in this section commits a Class B misdemeanor.

(c) This section does not permit a person certified as a clinical pastoral therapist to engage in the practice of licensed psychological examiner, licensed senior psychological examiner, certified psychological assistant, licensed psychologist, or licensed social worker, as defined in the laws of this state.

(d) This section does not apply to a person regulated by the board of nursing.

(e) This section does not permit a certified clinical pastoral therapist to prescribe medications or to interpret psychological tests intended to measure or diagnose mental illness.

(f) Certified clinical pastoral therapists shall establish and maintain effective working relationships with an interdisciplinary network of professionals, including at least one (1) psychologically oriented physician, usually a psychiatrist, with an unlimited license to practice the healing arts in this state, in order to make provision for referral for the diagnosis and treatment of medical or mental conditions falling outside the scope of clinical pastoral therapy.

63-33-1903. Documentation required from applicants.

In addition to the requirements of § 63-33-103, an applicant for licensure as a clinical pastoral therapist shall provide documentation in the form required by the commissioner that the applicant:

(1) Has met the educational standards set by the commissioner, which must include:

(A) The completion of a course of studies consisting of a minimum of sixty (60) graduate semester hours in a curriculum approved by the commissioner, of which a minimum of nine (9) graduate semester hours must relate specifically to the diagnosis and treatment of mental disorders, and the awarding of a master's or doctoral degree from a recognized educational institution; and

(B) The completion of a supervised clinical experience within the academic degree program as either a practicum or internship that includes experience in the assessment, diagnosis, and treatment of the psychodynamics, interpersonal dynamics, and spiritual dynamics of persons experiencing emotional, behavioral, relational, or spiritual distress or dysfunction. The experience required under this subdivision (1)(B) must be conducted under the supervision of a commissioner-approved supervisor;

(2) Has provided a minimum of one thousand four hundred (1,400) hours of pastoral therapy with individuals, couples, families, or groups while receiving a minimum of two hundred seventy (270) hours of supervision of such therapy under commissioner-approved supervision; and

(3) Has passed examinations as approved by the commissioner.

63-33-1904. Religious, training, and professions exemptions.

(a) This part and part 18 of this chapter do not:

(1) Limit the ministry, activities, or services of a rabbi, priest, minister of the gospel, or others authorized by a regularly organized and functioning religious body in performing the ordinary duties or functions of the clergy;

(2) Apply to or limit the activities or services of Christian Science practitioners; and

(3) Limit a rabbi, priest, or minister who offers counseling services, even if fees are charged, as long as they do not hold themselves out as licensed pastoral therapists or licensed pastoral counselors and do not purport to provide the integration and professional application of resources and techniques from the religious community's traditions of pastoral care and counsel along with recognized principles, methods, and procedures of clinical psychotherapy.

(b) This part does not apply to a person if the person is preparing for the practice of clinical pastoral therapy under qualified supervision in a training institution or facility or supervisory arrangement recognized and approved by the commissioner as long as the person is designated by such title as "pastoral therapy intern," "pastoral therapy trainee," or others clearly indicating such training status.

(c) This part does not prevent:

(1) Qualified members of other professional groups as defined by the commissioner by rule, including, but not limited to, licensed clinical social workers, licensed psychologists, licensed psychological examiners, licensed

senior psychological examiners, certified psychological assistants, psychiatric nurses, physicians, or attorneys at law, from performing or advertising that they provide or offer counseling services consistent with the accepted standards of their respective professions; and

(2) Alcohol and drug abuse counselors licensed under chapter 11, part 4 of this title or operating under qualified supervision while seeking such licensure from doing counseling consistent with the accepted standards of such profession.

63-33-1905. Reciprocity – License by endorsement.

(a) The board may enter into a reciprocal agreement with any other state that licenses, certifies, or registers clinical pastoral therapists, if the board finds that such state has substantially the same or higher licensure requirements than this state. The agreement must provide that the board may license without examination a resident of another state who is currently licensed, certified, or registered by the state, if that resident has met the same or higher requirements as provided in this part.

(b) The commissioner may license by endorsement a clinical pastoral therapist applicant who is a fellow or diplomate of the American Association of Pastoral Counselors if that person otherwise meets the requirements of this chapter.

(c) The commissioner may also license by endorsement a clinical pastoral therapist applicant who is:

(1) Certified as a clinical member of the American Association of Pastoral Counselors;

(2) Can document five (5) years of full-time practice in pastoral therapy subsequent to AAPC certification; and

(3) Otherwise meets the requirements of this part.

63-33-1906. License for persons certified on or before June 3, 2003.

A person who was certified as a certified clinical pastoral therapist on or before June 3, 2003, must be issued a license as a licensed clinical pastoral therapist when the

person's certification is renewed or upon request of the person before the expiration of the person's current certification.

63-33-1907. Temporary license.

(a) The commissioner may issue a temporary license to a clinical pastoral therapist applicant who has completed the academic coursework and supervised clinical experience for the license sought. In order to retain the temporary license, the applicant must take the written examination required by the commissioner the first time it is scheduled following issuance of the temporary license unless granted an extension by the commissioner after submitting a written request to the division and making a showing of good cause as determined by the commissioner. The applicant shall successfully pass the exam within two (2) years following issuance of the temporary license unless the applicant receives a good cause extension from the commissioner. An applicant must only receive one (1) good cause extension from the commissioner.

(b) A temporary license obtained pursuant to this section authorizes the applicant to engage in the practice of clinical pastoral therapy under the supervision of a commissioner-approved supervisor.

(c) In order to receive a temporary license, the applicant must submit to the division the following:

- (1) A completed application for a temporary license;
- (2) Satisfactory evidence of an agreement with a commissioner-approved supervisor; and
- (3) All required fees.

(d) A temporary license is nonrenewable and valid for a period of not more than three (3) years. A temporary license may be extended beyond the three-year period at the discretion of the commissioner if an applicant shows good cause for an extension as determined by the commissioner. Only one (1) extension may be granted by the commissioner for an applicant to extend the length of the applicant's temporary license.

Within the period of temporary licensure, the applicant shall submit to the division an application for the regular license and present supporting documentation demonstrating the satisfactory completion of the required supervised clinical experience. The commissioner shall then grant or deny the application for the regular license, based on satisfactory completion of all requirements for licensure. If the commissioner approves or denies the application for the regular license or revokes the temporary license for any reason, then the temporary license ceases to be valid and must be returned to the division.

(e) The holder of a temporary license as a clinical pastoral therapist shall not represent the holder to be a licensed clinical pastoral therapist. The holder of a temporary license as a clinical pastoral therapist shall only represent the holder to be a "pastoral therapy intern," a "pastoral therapy trainee," or such other title designation that clearly reflects trainee status and temporary licensure.

63-33-1908. Confidential relations and communications.

The confidential relations and communications between certified clinical pastoral therapists and clients are placed upon the same basis as those provided by law between attorney and client. This part does not require such privileged communication to be disclosed. However, this section and part 1 of this chapter do not prevent disclosures of confidential communications in proceedings arising under title 37, chapter 1, part 4, concerning mandatory child abuse reports.

SECTION 86. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-2001. Part definitions.

As used in this part, "counseling or therapy services" means assisting an individual, who is seeking or engaged in the counseling relationship in a private practice setting, in a manner intended to facilitate normal human growth and development, using a combination of mental health and human development principles, methods, and

techniques, to achieve mental, emotional, physical, social, moral, educational, spiritual, or career development and adjustment throughout the individual's life span.

63-33-2002. Client with goals, outcomes, or behaviors in conflict with sincerely held principles of counselor or therapist – Refusal to provide counseling or therapy services.

(a) A counselor or therapist providing counseling or therapy services is not required to counsel or serve a client as to goals, outcomes, or behaviors that conflict with the sincerely held principles of the counselor or therapist if the counselor or therapist coordinates a referral of the client to another counselor or therapist who will provide the counseling or therapy.

(b) The refusal to provide counseling or therapy services as described in subsection (a) is not a basis for:

- (1) A civil cause of action; or
- (2) Criminal prosecution.

(c) Subsections (a) and (b) do not apply to a counselor or therapist when the individual seeking or undergoing counseling is in imminent danger of harming themselves or others.

SECTION 87. Tennessee Code Annotated, Section 68-1-101(a)(8), is amended by deleting the subdivision and substituting:

(8) The division of health related boards for all administrative, fiscal, inspectional, and secretarial functions of the following boards, agencies, and commissions:

- (A) Board of chiropractic examiners;
- (B) Board of communication disorders and sciences;
- (C) Board of dentistry;
- (D) Board of examiners in psychology;
- (E) Board of medical examiners;

- (F) Board of physician assistants;
- (G) Board of nursing;
- (H) Board of optometry;
- (I) Board of osteopathic examination;
- (J) Board of osteopathic examination's council of certified professional midwifery;
- (K) Board of pharmacy;
- (L) Board of social worker licensure;
- (M) Tennessee medical laboratory board;
- (N) Board of veterinary medical examiners; and
- (O) Agency of commissioner issued licenses.

SECTION 88. Tennessee Code Annotated, Section 68-1-120, is amended by deleting the section and substituting the following:

By December 31, 2026, and by each December 31 thereafter, the department of health shall publish an annual report stating the total number of attempted suicides and completed suicides that have occurred in this state using the most recent year of final data available. The department of health shall submit the report electronically to all members of the general assembly within seven (7) calendar days of the publication.

SECTION 89. Tennessee Code Annotated, Section 68-1-125(d), is amended by deleting the subsection and substituting:

(d) The department of health, with ongoing consultation of appropriate experts and representatives of relevant providers to provide such consultation, shall determine which of its current programs are evidence-based, research-based, and theory-based. The department of health shall make the measurements of the individual programs, as set forth in subsection (c), available for review on its website.

SECTION 90. Tennessee Code Annotated, Section 68-1-128(h)(4), is amended by deleting the subdivision.

SECTION 91. Tennessee Code Annotated, Section 68-1-128, is amended by adding the following as a new subsection:

(j) The commissioner shall annually report to the chairs of the health and welfare committee of the senate and the committee of the house of representatives having jurisdiction over prescribers of controlled substances a summary of the data concerning prescribers identified pursuant to this section, disciplinary actions taken against those prescribers or taken related to prescription drug abuse or pain management clinics, distribution and abuse of controlled substances, and access to the controlled substance database.

SECTION 92. Tennessee Code Annotated, Section 68-1-147(a)(4), is amended by deleting the subdivision and substituting:

(4) "Palliative care" means specialized care focused on providing relief from physical, psychological, and spiritual suffering, in order to maximize quality of life for both the patient and family, that is provided for people facing a health condition that carries a high risk of mortality, and:

- (A) Negatively impacts a person's daily function;
- (B) Negatively impacts a person's quality of life; or
- (C) Excessively strains the person's caretaker.

SECTION 93. Tennessee Code Annotated, Title 68, Chapter 1, Part 1, is amended by adding the following as a new section:

68-1-150. Rules.

(a) When legislation is enacted by the general assembly that requires a board administratively attached to the department to promulgate rules in a prescribed period of time, the board retains the exclusive authority to promulgate the rules until sixty (60) days before the effective date of the legislation.

(b) If the board has not promulgated emergency rules and scheduled the rulemaking hearing on permanent rules at least sixty (60) days before the effective date

of the legislation, the commissioner becomes the exclusive authority to promulgate the rules as described in subsection (a). Upon the receipt of such authority, the commissioner shall, in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5:

(1) Promulgate emergency rules, in accordance with § 4-5-208, within the time required by the legislation; and

(2) Promulgate permanent rules within one hundred eighty (180) days of the effective date of the emergency rules.

(c) Upon the completion of the requirements of subsection (b), the exclusive authority to promulgate rules reverts to the board. The board may, at that time, amend the permanent rules in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

(d) The rules promulgated by the commissioner in accordance with subsection (b) remain in full force and effect, the same as if promulgated by the board, unless and until the rules are amended by the board.

SECTION 94. Tennessee Code Annotated, Section 68-1-2408, is amended by deleting the section and substituting the following:

(a) Entities receiving grants shall report to the health care and services facility as often as the department determines to be beneficial to program oversight.

(b) The health care and services facility shall report to the department as often as the department determines to be beneficial to program oversight, but at least annually.

(c) The department shall publish annually a summary of program participation and outcome measures.

SECTION 95. Tennessee Code Annotated, Section 68-11-201(25), is amended by:

(1) Deleting "chapter 3" in subdivision (G)(ii) and substituting "chapter 33, part 14";

(2) Deleting "title 63, chapter 13 or 17" in subdivision (J) and substituting "title 63, chapter 17; title 63, chapter 33, part 16; or title 63, chapter 33, part 17";

(3) Deleting "title 63, chapter 13 or 17" in subdivision (K)(i)(a) and substituting "title 63, chapter 17; title 63, chapter 33, part 16; or title 63, chapter 33, part 17"; and

(4) Deleting "title 63, chapter 13 or 17" in subdivision (L) and substituting "title 63, chapter 17; title 63, chapter 33, part 16; or title 63, chapter 33, part 17".

SECTION 96. Tennessee Code Annotated, Section 68-11-203, is amended by deleting "fifteen (15)" in subdivision (b)(1) and substituting "sixteen (16)"; and by adding the following as a new subdivision (b)(1)(C) and redesignating the existing subdivision (b)(1)(C) and subsequent subdivisions accordingly:

(C) The commissioner of health, or an employee of the department of health, designated by the commissioner;

SECTION 97. Tennessee Code Annotated, Section 68-11-209(d)(1), is amended by deleting "chapter 16" and substituting "chapter 33, part 13".

SECTION 98. Tennessee Code Annotated, Section 68-11-210(c)(4), is amended by deleting "chapter 16" and substituting "chapter 33, part 13".

SECTION 99. Tennessee Code Annotated, Section 68-11-216(a)(3)(B), is amended by deleting "title 63, chapter 13 or 17" and substituting "title 63, chapter 17; title 63, chapter 33, part 16; or title 63, chapter 33, part 17".

SECTION 100.

(a) Tennessee Code Annotated, Section 68-11-222(b)(6), is amended by deleting the subdivision and substituting:

(6) The commissioner of health with regard to licensure under the Occupational Therapy Practice Act, compiled in title 63, chapter 33, part 17;

(b) Tennessee Code Annotated, Section 68-11-222(b)(9), is amended by deleting the subdivision and substituting:

(9) The commissioner of health with regard to licensure under the Physical Therapy Practice Act, compiled in title 63, chapter 33, part 16;

(c) Tennessee Code Annotated, Section 68-11-222(b)(10), is amended by deleting the subdivision.

SECTION 101. Tennessee Code Annotated, Title 68, Chapter 24, Part 6, is amended by deleting the part.

SECTION 102. Tennessee Code Annotated, Section 68-33-109, is amended by deleting "chapter 22" and substituting "chapter 33".

SECTION 103. Tennessee Code Annotated, Section 68-55-102(b), is amended by deleting the subsection and substituting:

(b) The members are appointed to serve three-year or four-year terms. The successor appointee for the half of the members with a service expiration closest in time after July 1, 2026, shall serve three-year terms, while the other half of the members shall serve four-year terms. Appointment of successors must match the three-year or four-year term of the member they are appointed to succeed.

SECTION 104. The following rules are repealed on July 1, 2026:

- (1) Tenn. Comp. R. & Regs. 0880-11;
- (2) Tenn. Comp. R. & Regs. 0480-01;
- (3) Tenn. Comp. R. & Regs. 0540-01, 0540-02, and 0540-03; and
- (4) Tenn. Comp. R. & Regs. 1200-10-02.

SECTION 105.

(a) The headings in this act are for reference purposes only and do not constitute a part of the law enacted by this act. However, the Tennessee Code Commission is requested to include the headings in any compilation or publication containing this act.

(b) The use of "[former]" and "[repealed]" in this act are for notation purposes only and do not constitute a part of the law enacted by this act. However, the

Tennessee Code Commission is requested to include such notations in any compilation or publication containing this act.

(c) The Tennessee Code Commission is requested to reserve parts 10 and 11 in title 63, chapter 33.

SECTION 106. This act takes effect on July 1, 2026, the public welfare requiring it.